CLINICAL PATIENT JOURNEY

Patient ID: 10015860

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PATIENT INFORMATION - Subject ID: 10015860

A **53** year old **m** patient had a total of **21** hospital admission(s) during the recorded period.

Emergency Department Visit #1

Admission Time: September 15, 2186 at 12:56 PM

This is the patient's **1st** Emergency Department visit. The patient arrived via **AMBULANCE**. The patient departed at **2186-09-15 17:15:00**, staying for a duration of **0 days 4 hours 19 minutes**.

Initial Assessment:

Chief Complaint: R FOOT ULCER/CELLULITIS, Right foot infection

Blood Pressure	158.0/83.0 mmHg
Heart Rate	120.0 bpm
Respiratory Rate	18.0 breaths/min
Oxygen Saturation	96.0%
Temperature	99.6°F
Pain Score	8
Acuity Level	3.0

Previous Medications on Record: September 15, 2186 at 01:28 PM

5 medication(s) documented:

• glyburide - Antihyperglycemic - Sulfonylurea Derivatives

• omeprazole - Gastric Acid Secretion Reducing Agents - Proton Pump Inhibitors (PPIs)

• simvastatin - Antihyperlipidemic - HMG CoA Reductase Inhibitors (statins)

• lisinopril - ACE Inhibitors

• sildenafil - Erectile Dysfunction (ED) Drugs-Sel.cGMP Phosphodiesterase Type5 Inhib

Medications Administered: September 15, 2186 at 01:55 PM

Total of **7** medication(s) administered:

• Vancomycin

• Vancomycin

• MetRONIDAZOLE (Flagyl)

• Ciprofloxacin IV

• Insulin Human Regular

• Insulin Human Regular

• Insulin Human Regular

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*Time gap of **2 hours, 17 minutes** since last event*

Hospital Admission #1

Admission Time: September 15, 2186 at 04:12 PM

This is the patient's **1st** hospital admission. The patient was admitted from **EMERGENCY ROOM** as an **EW EMER.** admission. The admitting provider was **P67G26**. Insurance coverage: **Private**. Chief complaint: **Right foot infection**. The primary service provided was **PODIATRY**.

Race: **WHITE**, Marital Status: **SINGLE**, Language: **English**

APR DRG Code:

Code: 710

Description: INFECTIOUS AND PARASITIC DISEASES INCLUDING HIV WITH O.R. PROCEDURE

Severity: 3.0

Mortality Risk: 2.0

HCFA DRG Code:

Code: 853

Description: INFECTIOUS & PARASITIC DISEASES W O.R. PROCEDURE W MCC

Patient Past History:

Past Medical History: DM Type II, Hypertension, Hypercholesterolemia, Appendectomy

Family History: Father - Type II Mother Cancer - Hyperlipidemia

Social History: N/A

History of Present Illness:

Mr. is a very pleasant year old man with a PMH significant for DMII, HTN, HCol, who presents to his PCP today with swollen R foot, subsequently referred to the ED. He had not seen his PCP years, but came to his PCP this AM because his right foot was hot, red, warm, and painful. He has had pain this foot for the past three weeks; he attributes the onset to the self-removal of a callus on the ball of his foot. He had noted the development of some neuropathy his bilateral lower extremities staring a year ago. There was purulent discharge from his wound. Per PCP records, last A1c% was 8.9 . Baseline labs are notable for HDL 40, LDL 83, microalb 3.4 (elevated), Cr 0.77.

Admission Vital Signs:

General Appearance	AAOx3
Blood Pressure	158/83 mmHg
Heart Rate	120 bpm
Respiratory Rate	18 breaths/min
Temperature	99.6°F
Oxygen Saturation	96%

Admission Laboratory Results:

• WBC: 18.0

Admission Medications:

Total of **6** medications on admission:

• Sildenafil 100 mg PRN

• Lisinopril 5 mg Daily

• Glyburide 10 mg Daily

• Simvastatin 40 mg QHS

• Omeprazole 20 mg Daily

• (per PCP he has not filled nor taken any of these in the past year)

ICU Admission - Medical Intensive Care Unit (MICU)

ICU Admission Time: September 15, 2186 at 05:15 PM

The patient was admitted to the **Intensive Care Unit (Medical Intensive Care Unit (MICU))**. The patient remained in ICU until **2186-09-16 11:17:40**, for a total ICU stay of **0 days 18 hours 2 minutes** (Length of Stay: **0.7518518518518519 days**). During this critical care period, intensive monitoring and interventions were

performed including procedures, laboratory tests, and microbiology events.

Prescriptions Ordered: September 15, 2186 at 06:00 PM

1 medication(s) prescribed:

• Influenza Virus Vaccine 0.5 mL IM

Microbiology Tests Performed: September 15, 2186 at 07:59 PM

Total of **1** microbiology result(s):

MRSA SCREEN: MRSA SCREEN:

• No MRSA isolated.

Prescriptions Ordered: September 15, 2186 at 08:00 PM

10 medication(s) prescribed:

• 5% Dextrose 200 mL IV 1.0x/day

• Docusate Sodium 100 mg PO 2.0x/day

• Influenza Virus Vaccine 0.5 mL IM

• Insulin 0 UNIT SC

• Glucagon 1 mg IM

• Senna 1 TAB PO/NG

• Heparin 5000 UNIT SC 3.0x/day

• D5 1/2NS 1000 mL IV

• Dextrose 50% 12.5 gm IV

• Iso-Osmotic Dextrose 200 mL IV 1.0x/day

Laboratory Tests Performed: September 15, 2186 at 08:00 PM

Total of **31** tests performed, with **15** abnormal result(s).

Chemistry

Test	Value	Reference Range
% Hemoglobin A1c	**13.3%**	4.8-5.9
eAG	**335.0mg/dL**	91.0-123.0
Amylase	42.0IU/L	0.0-100.0

Anion Gap	15.0mEq/L	8.0-20.0
Bicarbonate	23.0mEq/L	22.0-32.0
Calcium, Total	8.4mg/dL	8.4-10.3
Chloride	96.0mEq/L	96.0-108.0
Creatine Kinase (CK)	**26.0IU/L**	47.0-322.0
Creatine Kinase, MB Isoenzyme	1.0ng/mL	0.0-10.0
Creatinine	0.8mg/dL	0.5-1.2
Ferritin	**1075.0ng/mL**	30.0-400.0
Glucose	**249.0mg/dL**	70.0-100.0
Iron	**14.0ug/dL**	45.0-160.0
Iron Binding Capacity, Total	**204.0ug/dL**	260.0-470.0
Lipase	38.0IU/L	0.0-60.0
Magnesium	2.0mg/dL	1.6-2.6
Phosphate	2.7mg/dL	2.7-4.5
Potassium	4.6mEq/L	3.3-5.1
Sodium	**129.0mEq/L**	133.0-145.0
Transferrin	**157.0mg/dL**	200.0-360.0
Troponin T	N/A	0.0-0.01
Urea Nitrogen	16.0mg/dL	6.0-20.0

Hematology

Test	Value	Reference Range
Hematocrit	**29.9%**	40.0-52.0
Hemoglobin	**10.8g/dL**	14.0-18.0
MCH	28.9pg	27.0-32.0
MCHC	**36.0%**	31.0-35.0
MCV	**80.0fL**	82.0-98.0
Platelet Count	339.0K/uL	150.0-440.0
RDW	12.2%	10.5-15.5

Red Blood Cells	**3.73m/uL**	4.6-6.2
White Blood Cells	**18.0K/uL**	4.0-11.0

Laboratory Tests Performed: September 15, 2186 at 08:26 PM

Total of **1** tests performed, with **0** abnormal result(s).

Blood Gas

Test	Value	Reference Range
Lactate	1.1mmol/L	0.5-2.0

Prescriptions Ordered: September 15, 2186 at 09:00 PM

2 medication(s) prescribed:

• NS 100 mL IV 3.0x/day

• 0.9% Sodium Chloride 1000 mL IV

Prescriptions Ordered: September 15, 2186 at 10:00 PM

2 medication(s) prescribed:

• Insulin 0 UNIT SC

• 0.9% Sodium Chloride 1000 mL IV

Prescriptions Ordered: September 15, 2186 at 11:00 PM

1 medication(s) prescribed:

• 0.9% Sodium Chloride (Mini Bag Plus) 100 mL IV 1.0x/day

Prescriptions Ordered: September 16, 2186 at 12:00 AM

2 medication(s) prescribed:

• Morphine Sulfate IR 15-30 mg PO/NG

• Morphine Sulfate 2-4 mg IV

Procedure(s) Performed: September 16, 2186 at 12:00 AM

1 procedure(s) performed (Source: ICD):

• Other incision with drainage of skin and subcutaneous tissue

Prescriptions Ordered: September 16, 2186 at 04:00 AM

1 medication(s) prescribed:

• 0.9% Sodium Chloride (Mini Bag Plus) 100 mL IV 1.0x/day

Laboratory Tests Performed: September 16, 2186 at 04:03 AM

Total of **38** tests performed, with **15** abnormal result(s).

Chemistry

Test	Value	Reference Range
Anion Gap	11.0mEq/L	8.0-20.0
Bicarbonate	**21.0mEq/L**	22.0-32.0
C-Reactive Protein	**264.0mg/L**	0.0-5.0
Calcium, Total	**7.0mg/dL**	8.4-10.3
Chloride	106.0mEq/L	96.0-108.0
Cholesterol Ratio (Total/HDL)	4.4Ratio Blood	N/A
Cholesterol, HDL	26.0mg/dL Blood	N/A
Cholesterol, LDL, Calculated	70.0mg/dL	0.0-129.0
Cholesterol, Total	114.0mg/dL	0.0-199.0
Creatine Kinase (CK)	**21.0IU/L**	47.0-322.0
Creatine Kinase, MB Isoenzyme	1.0ng/mL	0.0-10.0
Creatinine	0.7mg/dL	0.5-1.2
Glucose	**180.0mg/dL**	70.0-100.0
Magnesium	1.7mg/dL	1.6-2.6
Phosphate	**2.6mg/dL**	2.7-4.5
Potassium	4.2mEq/L	3.3-5.1
Sodium	134.0mEq/L	133.0-145.0
Triglycerides	88.0mg/dL	0.0-149.0
Troponin T	N/A	0.0-0.01
Urea Nitrogen	16.0mg/dL	6.0-20.0

Hematology

Test	Value	Reference Range
Basophils	0.3%	0.0-2.0
Eosinophils	0.4%	0.0-4.0
Hematocrit	**26.5%**	40.0-52.0
Hemoglobin	**9.2g/dL**	14.0-18.0
Lymphocytes	**14.1%**	18.0-42.0
MCH	28.5pg	27.0-32.0
MCHC	34.9%	31.0-35.0
MCV	82.0fL	82.0-98.0
Monocytes	5.1%	2.0-11.0
Neutrophils	**80.1%**	50.0-70.0
Platelet Count	291.0K/uL	150.0-440.0
RDW	12.4%	10.5-15.5
Red Blood Cells	**3.24m/uL**	4.6-6.2
Sedimentation Rate	**87.0mm/hr**	0.0-15.0
White Blood Cells	**15.1K/uL**	4.0-11.0
INR(PT)	**1.4**	0.9-1.1
PT	**14.8sec**	9.4-12.5
PTT	26.4sec	25.0-36.5

Microbiology Tests Performed: September 16, 2186 at 04:03 AM

Total of **1** microbiology result(s):

Blood Culture, Routine: BLOOD CULTURE:

• NO GROWTH.

Microbiology Tests Performed: September 16, 2186 at 04:03 AM

Total of **1** microbiology result(s):

Blood Culture, Routine: BLOOD CULTURE:

• NO GROWTH.

Laboratory Tests Performed: September 16, 2186 at 04:14 AM

Total of **1** tests performed, with **0** abnormal result(s).

Blood Gas

Test	Value	Reference Range
Lactate	0.7mmol/L	0.5-2.0

Prescriptions Ordered: September 16, 2186 at 06:00 AM

3 medication(s) prescribed:

• 0.9% Sodium Chloride 1 BAG IV 1.0x/day

• Neutra-Phos 2 PKT PO/NG 1.0x/day

• Bag 50 mL IV 1.0x/day

Prescriptions Ordered: September 16, 2186 at 08:00 AM

2 medication(s) prescribed:

• 5% Dextrose 200 mL IV 2.0x/day

• Iso-Osmotic Dextrose 200 mL IV 2.0x/day

Prescriptions Ordered: September 16, 2186 at 09:00 AM

2 medication(s) prescribed:

• Vancomycin 1000 mg IV

• Iso-Osmotic Dextrose 200 mL IV

Procedure(s) Performed: September 16, 2186 at 09:40 AM

1 procedure(s) performed (Source: ICU_EVENTS):

• Significant Events (Task) - OR Sent with value 1.0

Prescriptions Ordered: September 16, 2186 at 10:00 AM

3 medication(s) prescribed:

• Lisinopril 5 mg PO/NG 1.0x/day

• GlyBURIDE 5 mg PO/NG 1.0x/day

• Simvastatin 40 mg PO/NG 1.0x/day

Microbiology Tests Performed: September 16, 2186 at 10:20 AM

Total of **2** microbiology result(s):

GRAM STAIN: SWAB:

• [redacted]

WOUND CULTURE: SWAB:

• This culture contains mixed bacterial types (>=3) so an abbreviated workup is performed. Any growth of P.aeruginosa, S.aureus and beta hemolytic streptococci will be reported. IF THESE BACTERIA ARE NOT REPORTED, THEY ARE NOT PRESENT in this culture.

Unit Admission - PACU

Admission Time: September 16, 2186 at 11:17 AM

The patient was admitted to **PACU** ward. The patient stayed in this unit until **2186-09-16 14:45:34**, for a total duration of **0 days 3 hours 27 minutes**. During this time, various procedures, laboratory tests, and microbiology events were performed.

Prescriptions Ordered: September 16, 2186 at 01:00 PM

2 medication(s) prescribed:

• MetRONIDAZOLE (FLagyl) 500 mg PO/NG 2.0x/day

• Ciprofloxacin HCl 500 mg PO/NG 2.0x/day

Unit Admission - Vascular

Admission Time: September 16, 2186 at 02:45 PM

The patient was admitted to **Vascular** ward. The patient stayed in this unit until **2186-09-29 18:05:35**, for a total duration of **13 days 3 hours 20 minutes**. During this time, various procedures, laboratory tests, and microbiology events were performed.

Prescriptions Ordered: September 16, 2186 at 03:00 PM

6 medication(s) prescribed:

• OxycoDONE (Immediate Release) 5-10 mg PO/NG

• Sodium Chloride 0.9% Flush 3 mL IV

• Acetaminophen 325-650 mg PO/NG

• Ondansetron 4 mg IV

• Bisacodyl 10 mg PO

• Morphine Sulfate 2-6 mg IV

Prescriptions Ordered: September 16, 2186 at 08:00 PM

1 medication(s) prescribed:

• Omeprazole 20 mg PO 2.0x/day

Prescriptions Ordered: September 17, 2186 at 12:00 AM

1 medication(s) prescribed:

• Insulin 0 UNIT SC

Microbiology Tests Performed: September 17, 2186 at 06:16 AM

Total of **1** microbiology result(s):

Blood Culture, Routine: BLOOD CULTURE:

• NO GROWTH.

Microbiology Tests Performed: September 17, 2186 at 09:28 AM

Total of **1** microbiology result(s):

Blood Culture, Routine: BLOOD CULTURE:

• NO GROWTH.

Prescriptions Ordered: September 17, 2186 at 10:00 AM

1 medication(s) prescribed:

• 0.45% Sodium Chloride 1000 mL IV

Prescriptions Ordered: September 17, 2186 at 02:00 PM

1 medication(s) prescribed:

• Calcium Carbonate 1000 mg PO/NG 1.0x/day

Prescriptions Ordered: September 17, 2186 at 06:00 PM

1 medication(s) prescribed:

• 0.9% Sodium Chloride (Mini Bag Plus) 100 mL IV 2.0x/day

Prescriptions Ordered: September 17, 2186 at 08:00 PM

1 medication(s) prescribed:

• 5% Dextrose 500 mL IV 2.0x/day

Procedure(s) Performed: September 18, 2186 at 12:00 AM

1 procedure(s) performed (Source: ICD):

• Other incision with drainage of skin and subcutaneous tissue

Microbiology Tests Performed: September 18, 2186 at 07:40 AM

Total of **1** microbiology result(s):

Blood Culture, Routine: BLOOD CULTURE:

• NO GROWTH.

Prescriptions Ordered: September 18, 2186 at 08:00 AM

1 medication(s) prescribed:

• Insulin 0 UNIT SC

Microbiology Tests Performed: September 18, 2186 at 09:10 AM

Total of **1** microbiology result(s):

Blood Culture, Routine: BLOOD CULTURE:

• NO GROWTH.

Prescriptions Ordered: September 18, 2186 at 11:00 AM

1 medication(s) prescribed:

• Calcium Carbonate 1000 mg PO/NG 1.0x/day

Microbiology Tests Performed: September 18, 2186 at 03:35 PM

Total of **4** microbiology result(s):

ANAEROBIC CULTURE: SWAB:

• PREVOTELLA SPECIES

• FUSOBACTERIUM SPECIES

GRAM STAIN: SWAB:

• [redacted]

WOUND CULTURE: SWAB:

• This culture contains mixed bacterial types (>=3) so an abbreviated workup is performed. Any growth of P.aeruginosa, S.aureus and beta hemolytic streptococci will be reported. IF THESE BACTERIA ARE NOT REPORTED, THEY ARE NOT PRESENT in this culture.

Microbiology Tests Performed: September 19, 2186 at 07:30 AM

Total of **1** microbiology result(s):

Blood Culture, Routine: BLOOD CULTURE:

• NO GROWTH.

Prescriptions Ordered: September 19, 2186 at 11:00 AM

1 medication(s) prescribed:

• Lorazepam 0.5 mg PO/NG

Microbiology Tests Performed: September 19, 2186 at 11:00 AM

Total of **1** microbiology result(s):

Blood Culture, Routine: BLOOD CULTURE:

• NO GROWTH.

Prescriptions Ordered: September 19, 2186 at 07:00 PM

1 medication(s) prescribed:

• Iso-Osmotic Dextrose 50 mL IV 1.0x/day

Prescriptions Ordered: September 21, 2186 at 11:00 AM

1 medication(s) prescribed:

• 0.9% Sodium Chloride 1 BAG IV 1.0x/day

Prescriptions Ordered: September 21, 2186 at 08:00 PM

1 medication(s) prescribed:

• Miconazole 2% Cream 1 Appl TP 2.0x/day

Prescriptions Ordered: September 22, 2186 at 01:00 AM

1 medication(s) prescribed:

• Sarna Lotion 1 Appl TP

Prescriptions Ordered: September 22, 2186 at 04:00 PM

1 medication(s) prescribed:

• Iso-Osmotic Dextrose 200 mL IV 3.0x/day

Prescriptions Ordered: September 22, 2186 at 06:00 PM

1 medication(s) prescribed:

• Levofloxacin 750 mg PO/NG 1.0x/day

Prescriptions Ordered: September 23, 2186 at 12:00 AM

1 medication(s) prescribed:

• Potassium Chl 20 mEq / 1000 mL NS 1000 mL IV

Procedure(s) Performed: September 23, 2186 at 12:00 AM

2 procedure(s) performed (Source: ICD):

• Other skin graft to other sites

• Other fasciectomy

Prescriptions Ordered: September 23, 2186 at 11:00 AM

1 medication(s) prescribed:

• MetRONIDAZOLE (FLagyl) 500 mg PO/NG 3.0x/day

Prescriptions Ordered: September 23, 2186 at 05:00 PM

1 medication(s) prescribed:

• Sodium Chloride 0.9% Flush 3 mL IV

Prescriptions Ordered: September 23, 2186 at 08:00 PM

2 medication(s) prescribed:

• Miconazole 2% Cream 1 Appl TP 2.0x/day

• Triamcinolone Acetonide 0.1% Cream 1 Appl TP 2.0x/day

Procedure(s) Performed: September 25, 2186 at 12:00 AM

1 procedure(s) performed (Source: ICD):

• Central venous catheter placement with guidance

Prescriptions Ordered: September 25, 2186 at 02:00 PM

1 medication(s) prescribed:

• DiphenhydrAMINE 25 mg IV

Prescriptions Ordered: September 25, 2186 at 03:00 PM

1 medication(s) prescribed:

• Heparin Flush (10 units/ml) 2 mL IV

Prescriptions Ordered: September 26, 2186 at 03:00 PM

1 medication(s) prescribed:

• 5% Dextrose 500 mL IV 1.0x/day

Discharge Summary

Discharge Time: September 29, 2186 at 06:05 PM

The patient was discharged from **Vascular** with disposition to **Extended Care**.

Major Procedure(s) During Admission: ['Right foot I&D;', 'Right foot I&D;', 'Right foot I&D;, skin graft, wound vac placement']

Known Allergies: Penicillins

Detailed Allergy List:

• Penicillins

Discharge Diagnoses:

Primary Diagnoses:

• Right foot infection

Secondary Diagnoses:

• Sepsis

• Hyperglycemia/DM

• ?Osteomyelitis

• Anemia

• Hyponatremia

• Anion Gap Acidosis

• Depression

Complete Diagnosis List:

• Streptococcal septicemia

• Diabetes with ketoacidosis, type II or unspecified type, uncontrolled

• Cellulitis and abscess of foot, except toes

• Cellulitis and abscess of leg, except foot

• Hyposmolality and/or hyponatremia

• Ulcer of other part of foot

• Diabetes with neurological manifestations, type II or unspecified type, uncontrolled

• Sepsis

• Anemia of other chronic disease

• Polyneuropathy in diabetes

• Depressive disorder, not elsewhere classified

• Unspecified essential hypertension

• Pure hypercholesterolemia

• Candidiasis of skin and nails

• Contact dermatitis and other eczema, unspecified cause

• Examination of participant in clinical trial

Hospital Course:

Mr. is a very pleasant year old man with a PMH significant for DMII, HTN, Hyperlipidemia, who presents to his PCP today with severe right leg cellulitis and a septic picture. On , he was brought to the operating room; please see the operative report for full details. He tolerated the procedure well and was transferred to the PACU with vitals stable and vascular status intact. On admission he was admitted to the MICU but following the procedure and with stabilization of the pt overnight, the pt was transferred to the podiatry service. blood cultures from grew back GNR and wcx showed preliminary GPB. An ID consult was placed and recommended an echo as well as daily blood cultures. The pt continued to have low grade fevers as well as an elevated wbc. There was no decrease in erythema to his foot with malodor, tenderness, and purulent bleeding. The pt was brought back to the operating room on for a further I&D; please see the operative report for full details. A rash to his PIV and perineal region was noted over the weekend which ID thought may be due to a drug reaction or to a fungal

infection. Topical antifungals did not improve the affected sites and changing antibiotics did not aid the area as well. ID recommended a dermatology consult at this point. Dermatology recommended miconazole 2% cream BID to affected groing area, Triamcinolone 0.1% ointment to wrist & back BID, sarna lotion and benadryl for symptomatic relief. On the floors, daily wet to dry dressings were changed during evaluation of the RLE. He remained afebrile at this time and was able to tolerate po nutrition. Physical therapy evaluated the pt and stated the pt would be best served at a rehabilitation facility. The pt was brought back to the operating room for a right foot debridement, skin graft, and wound vac placement; please see the operative report for full details. The wound vac remained on while the pt was with a wound vac change on the day. The STSG was healing well to the plantar aspect of his foot, and all sutures remained intact with nice closure of wound edges. His thigh STSG donor site was notable to be heeling well, and was changed every second day with a non-adherent bandage. His wbc slowly began to trend downward. He remained hemodynamically stable. On discharge, his VSS and neurovascular status was intact to his RLE. He was discharged to rehab with strict NWB to RLE, non-adherent dry dressing changes to affected areas qdaily, and followup with Dr. and ID. # Sepsis: He met SIRS criteria with fevers, tachycardia, elevated WBC count, and an infectious source on the form of a diabetic foot ulcer and resultant cellulitis. He was placed on broad spectrum antibiotics and cultures will need to be followed. . # Hyperglycemia/DM: Likely secondary to acute infection. Does not appear to have DKA/HONK given modestly elevated BS, normal lactate, and AG of around 18. Insulin drip was started in the ICU but quickly weaned off to insulin sliding scale. Upon transfer to the floor he remained on ISS which normalized his BS. . # ?Osteomyelitis: Per Podiatry wound does not probe to bone, but ESR and CRP are markedly elevated. Continue broad spectrum antibioitcs coverage with vanc/cipro/flagyl pending tissue culture. Consider MRI lower extremity to assess for any enhancement suggestive of osteomyelitis. . # Anemia: HCT was 36.5 with mild microcytosis. Iron studies consistent with anemia of chronic inflammation. . # Hyponatremia: Corrected sodium is 133 on admission. He was likely volume deplete in the setting of infection. . # Anion Gap Acidosis: Likely in the setting of elevated blood sugars and lactate. Resolved after IVF resuscitation and brief stint on an insulin drip in the ICU. . # Depression: Patient endorses having difficulty with his home medication regimen and may have some underlying depression making coping difficult. Therefore, a Social Work consult was placed. . # Communication: Patient, HCP/brother is . # Code Status: Full Code

Microbiology Findings:

FOOT CULTURE: WOUND CULTURE (Final): STREPTOCOCCUS ANGINOSUS () GROUP. MODERATE GROWTH. SWAB: GRAM STAIN (Final): 1+ (<1 per 1000X FIELD): POLYMORPHONUCLEAR LEUKOCYTES. 4+ (>10 per 1000X FIELD): GRAM POSITIVE COCCI. IN PAIRS AND IN SHORT CHAINS. 3+ (per 1000X FIELD): GRAM NEGATIVE ROD(S). SMEAR REVIEWED; RESULTS CONFIRMED. WOUND CULTURE (Final): This culture contains mixed bacterial types (>=3) so an abbreviated workup is performed. Any growth of P.aeruginosa, S.aureus and beta hemolytic streptococci will be reported. IF THESE BACTERIA ARE NOT REPORTED BELOW, THEY ARE NOT PRESENT IN this culture.. SWAB: GRAM STAIN (Final): 1+ (<1 per 1000X FIELD): POLYMORPHONUCLEAR LEUKOCYTES. 1+ (<1 per 1000X FIELD): GRAM POSITIVE COCCI IN PAIRS. WOUND CULTURE (Final): This culture contains mixed bacterial types (>=3) so an abbreviated workup is performed. Any growth of P.aeruginosa, S.aureus and beta hemolytic streptococci will be reported. IF THESE BACTERIA ARE NOT REPORTED BELOW, THEY ARE NOT PRESENT IN this culture.. ANAEROBIC CULTURE (Preliminary): ANAEROBIC GRAM NEGATIVE ROD(S). RARE GROWTH. BLOOD CULTURES: BLOOD CULTURE: Blood Culture, Routine (Final): STREPTOCOCCUS ANGINOSUS () GROUP. FINAL SENSITIVITIES. Sensitivity testing performed by Sensititre. SENSITIVE TO CLINDAMYCIN MIC <= 0.12 MCG/ML. SENSITIVITIES: MIC expressed in MCG/ML STREPTOCOCCUS ANGINOSUS () GROUP | CLINDAMYCIN---------- S ERYTHROMYCIN----------<=0.25 S PENICILLIN G----------<=0.06 S

VANCOMYCIN---------- <=1 S. Anaerobic Bottle Gram Stain (Final): Reported to and read back by @ 3:10 ON . GRAM POSITIVE COCCI IN CHAINS. BLOOD CULTURE: Blood Culture, Routine (Final): STREPTOCOCCUS ANGINOSUS () GROUP. IDENTIFICATION AND SENSITIVITIES PERFORMED ON CULTURE # . FUSOBACTERIUM SPECIES. BETA LACTAMASE NEGATIVE. Anaerobic Bottle Gram Stain (Final): Reported to and read back by PACU OVERRIDE LOCATION @ 12:42 ON . GRAM NEGATIVE ROD(S). Aerobic Bottle Gram Stain (Final): GRAM POSITIVE COCCI IN CHAINS. BLOOD CULTURES from are still pending.

Antibiotic Plan:

Continue levofloxacin, metronidazole, and vancomycin as directed per ID recommendations.

Discharge Status:

Activity Status	Ambulatory - requires assistance or aid (walker or cane).
Level of Consciousness	Alert and interactive.
Mental Status	Clear and coherent.
Code Status	Full Code

Discharge Instructions:

You were discharged with new medications. Please take as directed. You may resume your normal home medications unless otherwise directed. You are to remain NONWEIGHT BEARING to your RIGHT FOOT at all times in a surgical shoe. Physical therapy will continue to work with you on your strength & balance. Nurses will perform all your dressing changes. Keep your dressings clean, dry, and intact. Avoid getting your dressings wet. You may resume your normal home diet. If you develop any of the symptoms listed below or anything else concerning, please see your PCP or go to your nearest emergency room. Please keep all follow up appointments.

New Medications Started:

• oxycodone 5 mg

• levofloxacin 750 mg

• metronidazole 500 mg

• vancomycin 1500 mg total

• heparin 5

• 000 unit/mL

• senna 8.6 mg

• docusate sodium 100 mg

• lorazepam 0.5 mg

- diphenhydramine HCl 50 mg/mL
- triamcinolone acetonide 0.1 % Cream
- miconazole nitrate 2 % Cream
- INSULIN

Medications Stopped:

- Sildenafil 100 mg

*Time gap of **350 days, 20 hours, 8 minutes** since last event*

Emergency Department Visit #2

Admission Time: September 15, 2187 at 02:14 PM

This is the patient's **2nd** Emergency Department visit. The patient arrived via **WALK IN**. The patient departed at **2187-09-15 20:49:00**, staying for a duration of **0 days 6 hours 35 minutes**.

Initial Assessment:

Chief Complaint: R FOOT PAIN, right foot ulcer

Blood Pressure	178.0/105.0 mmHg
Heart Rate	115.0 bpm
Respiratory Rate	18.0 breaths/min
Oxygen Saturation	96.0%
Temperature	98.3°F
Pain Score	5
Acuity Level	3.0

Medications Administered: September 15, 2187 at 04:07 PM

Total of **4** medication(s) administered:

• MetRONIDAZOLE (Flagyl)

• Levofloxacin

• Vancomycin

• Vancomycin

*Time gap of **2 hours, 42 minutes** since last event*

Hospital Admission #2

Admission Time: September 15, 2187 at 06:49 PM

This is the patient's **2nd** hospital admission. The patient was admitted from **EMERGENCY ROOM** as an **EW EMER.** admission. The admitting provider was **P86TXL**. Insurance coverage: **Private**. Chief complaint: **right foot ulcer**. The primary service provided was **PODIATRY**.

Race: **WHITE**, Marital Status: **SINGLE**, Language: **English**

APR DRG Code:

Code: 380

Description: SKIN ULCERS

Severity: 2.0

Mortality Risk: 1.0

HCFA DRG Code:

Code: 571

Description: SKIN DEBRIDEMENT W CC

Patient Past History:

Past Medical History: DM Type II, Hypertension, Hypercholesterolemia, PSH: Appendectomy

Family History: Father - Type II Mother Cancer - Hyperlipidemia

Social History: N/A

History of Present Illness:

y/o DM M with a hx of presents hypertension and hypercholesteremia, who is well known to the podiatry service presents with a right plantar forefoot ulcer. He is a patient of Dr. originally missed his appointment today and wanted to have his foot evaluated before the infection worsened. He presented today in athletic running shoes.

Admission Vital Signs:

General Appearance	NAD
Blood Pressure	N/A mmHg
Heart Rate	N/A bpm
Respiratory Rate	N/A breaths/min
Temperature	N/A°F
Oxygen Saturation	N/A

Admission Laboratory Results:

• WBC: 10.2

• Glucose: 120*

Admission Medications:

Total of **5** medications on admission:

• omeprazole ec 20 mg

• simvastatin 40 mg qhs

• glyburide 5 mg

• lisinopril 5 mg

• sildenafil 100 mg prn

Previous Medications on Record: September 15, 2187 at 06:55 PM

4 medication(s) documented:

• insulin regular human - Human Insulins - Short Acting

• Lantus Solostar - Insulin Analogs - Long Acting

• simvastatin - Antihyperlipidemic - HMG CoA Reductase Inhibitors (statins)

• atenolol - Beta Blockers Cardiac Selective

Prescriptions Ordered: September 15, 2187 at 08:00 PM

5 medication(s) prescribed:

• MetRONIDAZOLE (FLagyl) 500 mg PO/NG 3.0x/day

• Omeprazole 20 mg PO 2.0x/day

• Heparin 5000 UNIT SC 3.0x/day

• Iso-Osmotic Dextrose 200 mL IV 2.0x/day

• Docusate Sodium 100 mg PO/NG 2.0x/day

Unit Admission - Vascular

Admission Time: September 15, 2187 at 08:49 PM

The patient was admitted to **Vascular** ward. The patient stayed in this unit until **2187-09-19 14:55:19**, for a total duration of **3 days 18 hours 6 minutes**. During this time, various procedures, laboratory tests, and microbiology events were performed.

Prescriptions Ordered: September 15, 2187 at 10:00 PM

11 medication(s) prescribed:

• Influenza Virus Vaccine 0.5 mL IM

• OxycoDONE (Immediate Release) 5-10 mg PO/NG

• Dextrose 50% 12.5 gm IV

• Pneumococcal Vac Polyvalent 0.5 mL IM

• Sodium Chloride 0.9% Flush 3 mL IV

• Ciprofloxacin HCl 750 mg PO/NG 2.0x/day

• Zolpidem Tartrate 5 mg PO 1.0x/day

• Glucagon 1 mg IM

• Senna 1 TAB PO/NG

• Ondansetron 8 mg PO/NG

• Glucose Gel 15 g PO

Prescriptions Ordered: September 15, 2187 at 11:00 PM

1 medication(s) prescribed:

• Insulin 0 UNIT SC

Prescriptions Ordered: September 18, 2187 at 09:00 PM

1 medication(s) prescribed:

• Iso-Osmotic Dextrose 200 mL IV 1.0x/day

Discharge Summary

Discharge Time: September 19, 2187 at 02:55 PM

The patient was discharged from **Vascular** with disposition to **Home With Service**.

Known Allergies: Penicillins

Detailed Allergy List:

• Penicillins

Discharge Diagnoses:

Primary Diagnoses:

• right foot ulcer

Secondary Diagnoses:

• DM Type II

• Hypertension

• Hypercholesterolemia

Complete Diagnosis List:

• Ulcer of other part of foot

• Cellulitis and abscess of foot, except toes

• Diabetes mellitus without mention of complication, type II or unspecified type, not stated as uncontrolled

• Unspecified essential hypertension

• Pure hypercholesterolemia

• Personal history of tobacco use

Hospital Course:

Mr. presented to the Emergency Department at after missing a scheduled appointment with Dr. concern that his infection was worsening. He was admitted on for a right foot infection. During his stay, he received IV antibiotics to fight the cellulitis and xrays were obtained and showed no osteomyelitis. The wound was lightly debrided at the bedside during his stay and he was fitted for a bivalve cast by an orthotech. He was given strict instructions on touch down weight bearing to the heel using a walker or crutches. Physical therapy worked with him while in the hospital and cleared him for home with such. Prior to discharge his vital signs were stable and neurovascular status intact. He understood all of his discharge instructions and is to follow up with Dr. in approximately 1 week.

Antibiotic Plan:

Clindamycin 150 mg PO Q6H

Discharge Status:

Activity Status	Ambulatory - requires assistance or aid (walker or cane).
Level of Consciousness	Alert and interactive.
Mental Status	Clear and coherent.
Code Status	N/A

Discharge Instructions:

Please follow these guidelines unless your physician has specifically instructed you otherwise. Please call our office nurse if you have any questions. Dial 911 if you have any medical emergency. ACTIVITY: There are

restrictions on activity. On your right side you are TOUCH DOWN WEIGHT BEARING TO THE HEEL IN A BIVALVE CAST AND CRUTCHES/WALKER for weeks. You should keep this site elevated when ever possible (above the level of the heart!) Physical therapy worked with you in the hospital and gave instructions on weight bearing: please follow these accordingly. No driving until cleared by your Surgeon. PLEASE CALL US IMMEDIATELY FOR ANY OF THE FOLLOWING PROBLEMS: Redness in or drainage from your leg wound(s). New pain, numbness or discoloration of your foot or toes. Watch for signs and symptoms of infection. These are: a fever greater than 101 degrees, chills, increased redness, or pus draining from the incision site. If you experience any of these or bleeding at the incision site, CALL THE DOCTOR. Exercise: Limit strenuous activity for 6 weeks. No heavy lifting greater than 20 pounds for the next days. Try to keep leg elevated when able. BATHING/SHOWERING: You may shower immediately upon coming home, but you must keep your dressing CLEAN, DRY and INTACT. You can use a shower bag taped around your ankle/leg or hang your foot/leg outside of the bathtub. WOUND CARE: You will be getting every other day dressing changes by a visiting nurse with betadine paint to the ulceration and a dry sterile dressing. You may cleanse the foot with peroxide. Once the dressing is in place, avoid getting it wet. MEDICATIONS: Unless told otherwise you should resume taking all of the medications you were taking before surgery. You will be given a new prescription for infection which will be taken every 6 hours. CAUTIONS: NO SMOKING! We know you've heard this before, but it really is an important step to your recovery. Smoking causes narrowing of your blood vessels which in turn decreases circulation. If you smoke you will need to stop as soon as possible. Ask your nurse or doctor for information on smoking cessation. If a follow up appointment was not made prior to your discharge, please call the office on the first working day after your discharge from the hospital to schedule a follow-up visit. This should be scheduled on the calendar for seven to fourteen days after discharge. Normal office hours are through . PLEASE FEEL FREE TO CALL THE OFFICE WITH ANY OTHER CONCERNS OR QUESTIONS THAT MIGHT ARISE.

New Medications Started:

- Clindamycin 150 mg PO Q6H
- Aspirin 81 mg PO DAILY
- Atenolol 25 mg PO DAILY
- Glargine 40 Units Breakfast
- Insulin SC Sliding Scale using REG Insulin
- Omeprazole 20 mg PO BID

Medications Stopped:

- glyburide 5 mg
- sildenafil 100 mg prn
- omeprazole ec 20 mg

*Time gap of **85 days, 21 hours, 53 minutes** since last event*

Emergency Department Visit #3

Admission Time: December 14, 2187 at 12:49 PM

This is the patient's **3rd** Emergency Department visit. The patient arrived via **AMBULANCE**. The patient departed at **2187-12-14 17:48:00**, staying for a duration of **0 days 4 hours 59 minutes**.

Initial Assessment:

Chief Complaint: R LEG CELLULITIS

Blood Pressure	180.0/96.0 mmHg
Heart Rate	97.0 bpm
Respiratory Rate	16.0 breaths/min
Oxygen Saturation	98.0%
Temperature	98.2°F
Pain Score	9
Acuity Level	3.0

Medications Administered: December 14, 2187 at 01:59 PM

Total of **3** medication(s) administered:

• Vancomycin

• Vancomycin

• Insulin Human Regular

Previous Medications on Record: December 14, 2187 at 02:33 PM

8 medication(s) documented:

• aspirin - Salicylate Analgesics

• aspirin - Platelet Aggregation Inhibitors - Salicylates

• atenolol - Beta Blockers Cardiac Selective

• Lantus Solostar - Insulin Analogs - Long Acting

• lisinopril - ACE Inhibitors

• clindamycin HCl - Lincosamide Antibiotics

• simvastatin - Antihyperlipidemic - HMG CoA Reductase Inhibitors (statins)

• Tylenol Ex Str Rapid Release - Analgesic or Antipyretic Non-Opioid

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*Time gap of **1 hour, 15 minutes** since last event*

Hospital Admission #3

Admission Time: December 14, 2187 at 03:48 PM

This is the patient's **3rd** hospital admission. The patient was admitted from **EMERGENCY ROOM** as an **EU OBSERVATION** admission. The admitting provider was **P85WIX**. Insurance coverage: **Private**. Chief complaint: **Cellulitis**. The primary service provided was **MEDICINE**.

Race: **WHITE**, Marital Status: **SINGLE**, Language: **English**

Patient Past History:

Past Medical History: DM Type II with neuropathy, Hypertension, Hypercholesterolemia, Obesity, Appendectomy

Family History: Father - Type II Mother Cancer - Hyperlipidemia

Social History: N/A

History of Present Illness:

y/o M with hx of poorly-controlled DMII presents for eval from for worsening LLE cellulitis x5 days. Endorses feeling "blah and out of it" last week, subsequently seen in f/u by Podiatry for right foot ulcer x2, started on po clindamycin at that time. Patient then noted left lower extremity redness and tenderness over next couple of days, seen in f/u on and continued on clinda for presumed cellulitis. Seen once again today with persistence of LLE redness, warmth, and tenderness, along with glucose elevated to 500, admitted for IV antibiotics. Of note, he reports that he was mitsakenly only taking 1 pill of the clinda TID though it is written for 3 pills TID (for 450mg po TID), and he also reports drinking a large amount of gatorade prior to his admission. Patient denies any fevers, chills, CP, palpitations, SOB, cough, abdominal pain, nausea, vomiting, diarrhea, or any other complaints. In the ED, initial vitals 98.2 97 180/96 16 98%. Received dose of vancomycin and imipenem empirically along with 10 units regular insulin IV for critical FSBG to 500. On arrival to the floor, VS 97.7 150/89 94 16 100%RA Patient comfortable and denies any new complaints. REVIEW OF SYSTEMS: Denies fever, chills, night sweats, headache, vision changes, rhinorrhea, congestion, sore throat, cough, shortness of breath, chest pain, abdominal pain, nausea, vomiting, diarrhea, constipation, BRBPR, melena, hematochezia, dysuria, hematuria. All other 10-system review negative in detail.

Admission Vital Signs:

General Appearance	well-developed
Blood Pressure	150/89 mmHg
Heart Rate	94 bpm
Respiratory Rate	16 breaths/min
Temperature	97.7°F
Oxygen Saturation	100%RA

Admission Laboratory Results:

• WBC: 12.7

• Glucose: 512

• Lactate: 2.4

Admission Medications:

Total of **10** medications on admission:

• Glargine 40 Units Breakfast

• Simvastatin 40 mg PO DAILY

• Clindamycin 150 mg PO Q8H

• Lisinopril 5 mg PO DAILY hold for SBP<100

• Atenolol 25 mg PO DAILY hold for SBP<100 or HR<60

• Sildenafil 100 mg PO 1 HOUR BEFORE SEXUAL ACTIVITY

• Aspirin 81 mg PO DAILY

• Fish Oil (Omega 3) 1000 mg PO BID

• Vitamin E 400 UNIT PO DAILY

• Vitamin D 1000 UNIT PO DAILY

Unit Admission - Medicine

Admission Time: December 14, 2187 at 05:48 PM

The patient was admitted to **Medicine** ward. The patient stayed in this unit until **2187-12-16 14:38:30**, for a total duration of **1 days 20 hours 50 minutes**. During this time, various procedures, laboratory tests, and microbiology events were performed.

Prescriptions Ordered: December 14, 2187 at 07:00 PM

2 medication(s) prescribed:

• Iso-Osmotic Dextrose 200 mL IV 1.0x/day

• Sodium Chloride 0.9% Flush 3 mL IV 3.0x/day

Prescriptions Ordered: December 14, 2187 at 08:00 PM

7 medication(s) prescribed:

• Acetaminophen 1000 mg PO/NG

• Heparin 5000 UNIT SC 3.0x/day

• Dextrose 50% 12.5 gm IV

• Iso-Osmotic Dextrose 200 mL IV 2.0x/day

• Docusate Sodium 100 mg PO/NG 2.0x/day

• Glucagon 1 mg IM

• Glucose Gel 15 g PO

Prescriptions Ordered: December 14, 2187 at 10:00 PM

1 medication(s) prescribed:

• Insulin 40 UNIT SC 1.0x/day

Prescriptions Ordered: December 15, 2187 at 08:00 AM

2 medication(s) prescribed:

• Doxycycline Hyclate 100 mg PO 2.0x/day

• Insulin 40 UNIT SC 1.0x/day

Discharge Summary

Discharge Time: December 16, 2187 at 02:38 PM

The patient was discharged from **Medicine** with disposition to **Home With Service**.

Known Allergies: Penicillins

Detailed Allergy List:

• Penicillins

Discharge Diagnoses:

Primary Diagnoses:

• Cellulitis

• left lower extremity

Secondary Diagnoses:

• Hyperglycemia >500

• no anion gap

Complete Diagnosis List:

• Cellulitis and abscess of foot, except toes

• Ulcer of other part of foot

• Hyposmolality and/or hyponatremia

• Hypertensive chronic kidney disease, unspecified, with chronic kidney disease stage I through stage IV, or unspecified

• Chronic kidney disease, unspecified

• Diabetes with neurological manifestations, type II or unspecified type, uncontrolled

• Polyneuropathy in diabetes

• Pure hypercholesterolemia

• Obesity, unspecified

• Alcohol abuse, in remission

• Personal history of noncompliance with medical treatment, presenting hazards to health

• Long-term (current) use of aspirin

• Long-term (current) use of insulin

• Examination of participant in clinical trial

Hospital Course:

y/o M with hx of poorly-controlled DMII and right foot ulcers presenting with LLE cellulitis persisting after underdosing his oral clindamycin. ACTIVE ISSUES: # Cellulitis: Improved on IV vancomycin. Cellulitis was likely undertreated on his oral clindamycin therapy given that he was only taking of the prescribed dose mistakenly. Given his clinical stability and improvement in LLE cellulitis, he was transitioned to po doxycycline for course of additional days with continued improvement of cellulitis prior to discharge. # Right foot ulcers: Stable based on exam and recent podiatry note. Maintained wrapped in gauze and with pressure-relieving pad in place on foot. No change in ulcer that would necessitate inpatient podiatry consult. Follow-up as outpatient with podiatry. # Hyponatremia: Falsely decreased due to marked hyperglycemia, corrected to 135 when hyperglycemic and normalized when normoglycemic. Asymptomatic. CHRONIC ISSUES: # CKD: Mild creatinine elevation on admission returned to baseline in mid-1s, likely was related to both hyperglycemia with osmotic diuresis and pre-renal etiology along with concurrent infection. # DMII, poorly-controlled: Presentation

of FSBG >500 likely secondary to both underlying cellulitis along with dietary non-compliance given his significant gatorade consumption. Did not have widened anion gap. Continued on his home Lantus 40 units and covered with HISS while in-house. # HLD: Continude home simvastatin. # HTN: Continued home lisinopril and atenolol.

Microbiology Findings:

Blood Culture, Routine (Final): NO GROWTH

Antibiotic Plan:

Transitioned to po doxycycline for course of additional days with continued improvement of cellulitis prior to discharge.

Discharge Status:

Activity Status	Ambulatory - requires assistance or aid (walker or cane).
Level of Consciousness	Alert and interactive.
Mental Status	Clear and coherent.
Code Status	N/A

Discharge Instructions:

Dear Mr. , It was a pleasure caring for you during your hospitalization. You were admitted on for an infection of your left leg called cellulitis and very high blood sugar levels. You were given IV antibiotics initially (vancomycin) and then changed to an oral antibiotic called doxycycline. You were also given some extra insulin in the Emergency Department and started back on your home regimen. Your cellulitis has been significantly improving, and you are now safe to be discharged home. Please be sure to take all of the antibiotic medicine and to follow-up with your doctors as .

New Medications Started:

• Doxycycline Hyclate 100 mg PO Q12H

Medications Stopped:

• Clindamycin 150 mg PO Q8H

*Time gap of **233 days, 5 hours, 57 minutes** since last event*

Emergency Department Visit #4

Admission Time: August 05, 2188 at 08:36 PM

This is the patient's **4th** Emergency Department visit. The patient arrived via **WALK IN**. The patient departed at **2188-08-06 03:24:00**, staying for a duration of **0 days 6 hours 48 minutes**.

Initial Assessment:

Chief Complaint: RIGHT FOOT INFECTION

Blood Pressure	107.0/73.0 mmHg
Heart Rate	119.0 bpm
Respiratory Rate	16.0 breaths/min
Oxygen Saturation	99.0%
Temperature	100.0°F
Pain Score	7
Acuity Level	3.0

Laboratory Tests Performed: August 05, 2188 at 08:50 PM

Total of **25** tests performed, with **12** abnormal result(s).

Chemistry

Test	Value	Reference Range
Light Green Top Hold	N/A Blood	N/A
Blue Top Hold	N/A Blood	N/A
Anion Gap	17.0mEq/L	8.0-20.0
Bicarbonate	23.0mEq/L	22.0-32.0
Chloride	**91.0mEq/L**	96.0-108.0
Creatinine	**2.1mg/dL**	0.5-1.2
Estimated GFR (MDRD equation)	N/A Blood	N/A

Glucose	**523.0mg/dL**	70.0-100.0
Potassium	5.1mEq/L	3.3-5.1
Sodium	**126.0mEq/L**	133.0-145.0
Urea Nitrogen	**32.0mg/dL**	6.0-20.0

Hematology

Test	Value	Reference Range
Basophils	0.3%	0.0-2.0
Eosinophils	0.2%	0.0-4.0
Hematocrit	**30.9%**	40.0-52.0
Hemoglobin	**10.6g/dL**	14.0-18.0
Lymphocytes	**4.9%**	18.0-42.0
MCH	28.3pg	27.0-32.0
MCHC	34.5%	31.0-35.0
MCV	82.0fL	82.0-98.0
Monocytes	4.6%	2.0-11.0
Neutrophils	**90.0%**	50.0-70.0
Platelet Count	**513.0K/uL**	150.0-440.0
RDW	13.2%	10.5-15.5
Red Blood Cells	**3.76m/uL**	4.6-6.2
White Blood Cells	**17.7K/uL**	4.0-11.0

Laboratory Tests Performed: August 05, 2188 at 09:00 PM

Total of **1** tests performed, with **0** abnormal result(s).

Blood Gas

Test	Value	Reference Range
Lactate	1.4mmol/L	0.5-2.0

Previous Medications on Record: August 05, 2188 at 10:13 PM

3 medication(s) documented:

• clindamycin HCl - Lincosamide Antibiotics

• oxycodone-acetaminophen - Analgesic Opioid Oxycodone Combinations

• oxycodone-acetaminophen - Analgesic Opioid Oxycodone and Non-Salicylate Combinations

Medications Administered: August 05, 2188 at 10:22 PM

Total of **8** medication(s) administered:

• OxyCODONE (Immediate Release)

• Acetaminophen

• Ciprofloxacin IV

• MetRONIDAZOLE (Flagyl)

• Insulin Glargine

• Insulin Glargine

• Vancomycin

• Vancomycin

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*Time gap of **2 hours, 27 minutes** since last event*

Hospital Admission #5

Admission Time: August 06, 2188 at 12:49 AM

This is the patient's **5th** hospital admission. The patient was admitted from **EMERGENCY ROOM** as an **EW EMER.** admission. The admitting provider was **P87IIY**. Insurance coverage: **Private**. Chief complaint: **Right foot infection**. The primary service provided was **MEDICINE**.

Race: **WHITE**, Marital Status: **SINGLE**, Language: **English**

APR DRG Code:

Code: 320

Description: OTHER MUSCULOSKELETAL SYSTEM AND CONNECTIVE TISSUE PROCEDURES

Severity: 3.0

Mortality Risk: 3.0

HCFA DRG Code:

Code: 628

Description: OTHER ENDOCRINE, NUTRIT & METAB O.R. PROC W MCC

Patient Past History:

Past Medical History: DM Type II with neuropathy, Hypertension, Hypercholesterolemia, Obesity, CKD, stage , s/p Appendectomy

Family History: Father with diabetes -Mother with colon cancer

Social History: N/A

History of Present Illness:

yo man with hx HTN and DM who presents with a diabetic foot ulcer. For the past week, he has been maintained on clindamycin for a R foot ulcer but the ulcer has worsened and become more painful and his blood glucose has ranged above 400. He has been fatigued and has noted chills. He was seen clinic today and was found to have a WBC of 15.6 and a HR of 130. He was referred to the ED for evaluation for sepsis. On the floor, he has no further complaints.

Admission Vital Signs:

General Appearance	comfortable NAD
Blood Pressure	112/66 mmHg
Heart Rate	88 bpm
Respiratory Rate	18 breaths/min
Temperature	98.0°F
Oxygen Saturation	100% RA

Admission Laboratory Results:

• WBC: 17.7

• Glucose: 523

• Lactate: 1.4

Admission Medications:

Total of **9** medications on admission:

• Omeprazole 20 mg PO DAILY:PRN gerd

• Simvastatin 40 mg PO QPM

• Lisinopril 5 mg PO DAILY

• Sildenafil 100 mg PO DAILY:PRN sexual activity

• Clindamycin 150 mg PO Q6H

• Oxycodone-Acetaminophen (5mg-325mg) TAB PO Q4H:PRN pain

• Atenolol 25 mg PO DAILY

• Glargine 40 Units Breakfast

• Aspirin 81 mg PO DAILY

Unit Admission - Medicine

Admission Time: August 06, 2188 at 03:24 AM

The patient was admitted to **Medicine** ward. The patient stayed in this unit until **2188-08-10 16:33:00**, for a total duration of **4 days 13 hours 9 minutes**. During this time, various procedures, laboratory tests, and microbiology events were performed.

Prescriptions Ordered: August 06, 2188 at 04:00 AM

1 medication(s) prescribed:

• Influenza Virus Vaccine 0.5 mL IM

Prescriptions Ordered: August 06, 2188 at 05:00 AM

8 medication(s) prescribed:

• NS 100 mL IV 3.0x/day

• Acetaminophen 325-650 mg PO/NG

• Senna 8.6 mg PO/NG

• Heparin 5000 UNIT SC 3.0x/day

• 0.9% Sodium Chloride (Mini Bag Plus) 100 mL IV 1.0x/day

• Docusate Sodium 100 mg PO/NG 2.0x/day

• Sodium Chloride 0.9% Flush 3 mL IV 3.0x/day

• 0.9% Sodium Chloride 1000 mL IV

Prescriptions Ordered: August 06, 2188 at 06:00 AM

6 medication(s) prescribed:

• Insulin 0 UNIT SC

• Glucagon 1 mg IM

• OxycoDONE (Immediate Release) 5-10 mg PO/NG

• Glucose Gel 15 g PO

• Dextrose 50% 12.5 gm IV

• Simvastatin 40 mg PO/NG 1.0x/day

Prescriptions Ordered: August 06, 2188 at 08:00 AM

2 medication(s) prescribed:

• Aspirin 81 mg PO/NG 1.0x/day

• Iso-Osmotic Dextrose 200 mL IV 1.0x/day

Prescriptions Ordered: August 06, 2188 at 01:00 PM

1 medication(s) prescribed:

• Insulin 0 UNIT SC

Prescriptions Ordered: August 06, 2188 at 02:00 PM

1 medication(s) prescribed:

• Ciprofloxacin HCl 500 mg PO/NG 2.0x/day

Microbiology Tests Performed: August 06, 2188 at 04:55 PM

Total of **1** microbiology result(s):

Blood Culture, Routine: BLOOD CULTURE:

• NO GROWTH.

Microbiology Tests Performed: August 06, 2188 at 07:10 PM

Total of **1** microbiology result(s):

Blood Culture, Routine: BLOOD CULTURE:

• NO GROWTH.

Prescriptions Ordered: August 06, 2188 at 10:00 PM

1 medication(s) prescribed:

• Insulin 40 UNIT SC 1.0x/day

Prescriptions Ordered: August 06, 2188 at 11:00 PM

1 medication(s) prescribed:

• Insulin 10 UNIT SC 1.0x/day

Procedure(s) Performed: August 07, 2188 at 12:00 AM

1 procedure(s) performed (Source: ICD):

• Local excision of lesion or tissue of bone, other bones

Microbiology Tests Performed: August 07, 2188 at 06:29 AM

Total of **1** microbiology result(s):

Blood Culture, Routine: BLOOD CULTURE:

• NO GROWTH.

Microbiology Tests Performed: August 07, 2188 at 07:55 AM

Total of **5** microbiology result(s):

TISSUE: TISSUE:

• BETA STREPTOCOCCUS GROUP B

• BETA STREPTOCOCCUS GROUP G

• MIXED BACTERIAL FLORA

GRAM STAIN: TISSUE:

• [redacted]

ANAEROBIC CULTURE: TISSUE:

• NO ANAEROBES ISOLATED.

Microbiology Tests Performed: August 07, 2188 at 10:12 AM

Total of **1** microbiology result(s):

Blood Culture, Routine: BLOOD CULTURE:

• NO GROWTH.

Prescriptions Ordered: August 07, 2188 at 12:00 PM

1 medication(s) prescribed:

• Insulin 0 UNIT SC

Prescriptions Ordered: August 07, 2188 at 04:00 PM

1 medication(s) prescribed:

• Lisinopril 5 mg PO/NG 1.0x/day

Microbiology Tests Performed: August 08, 2188 at 06:10 AM

Total of **1** microbiology result(s):

Blood Culture, Routine: BLOOD CULTURE:

• NO GROWTH.

Prescriptions Ordered: August 08, 2188 at 10:00 AM

3 medication(s) prescribed:

• Sodium Chloride 0.9% Flush 20 mL IV

• Lidocaine 1% 5 mL SC

• Heparin Flush (10 units/ml) 2 mL IV 1.0x/day

Procedure(s) Performed: August 09, 2188 at 12:00 AM

1 procedure(s) performed (Source: ICD):

• Local excision of lesion or tissue of bone, other bones

Prescriptions Ordered: August 09, 2188 at 08:00 AM

1 medication(s) prescribed:

• Lisinopril 10 mg PO/NG 1.0x/day

Microbiology Tests Performed: August 09, 2188 at 01:00 PM

Total of **1** microbiology result(s):

Blood Culture, Routine: BLOOD CULTURE:

• NO GROWTH.

Prescriptions Ordered: August 09, 2188 at 08:00 PM

1 medication(s) prescribed:

• 5% Dextrose 250 mL IV 2.0x/day

Prescriptions Ordered: August 10, 2188 at 08:00 AM

2 medication(s) prescribed:

• Insulin 0 UNIT SC

• Insulin 40 UNIT SC 1.0x/day

Unit Admission - Medicine

Admission Time: August 10, 2188 at 04:33 PM

The patient was admitted to **Medicine** ward. The patient stayed in this unit until **2188-08-12 17:49:20**, for a total duration of **2 days 1 hours 16 minutes**. During this time, various procedures, laboratory tests, and microbiology events were performed.

Prescriptions Ordered: August 10, 2188 at 06:00 PM

3 medication(s) prescribed:

• Lidocaine 1% 5 mL SC

• Heparin Flush (10 units/ml) 2 mL IV 1.0x/day

• Sodium Chloride 0.9% Flush 20 mL IV

Prescriptions Ordered: August 11, 2188 at 08:00 AM

3 medication(s) prescribed:

• Amlodipine 5 mg PO/NG 1.0x/day

• Insulin 40 UNIT SC 1.0x/day

• Atorvastatin 20 mg PO/NG 1.0x/day

Procedure(s) Performed: August 12, 2188 at 12:00 AM

1 procedure(s) performed (Source: ICD):

• Venous catheterization, not elsewhere classified

Prescriptions Ordered: August 12, 2188 at 09:00 AM

1 medication(s) prescribed:

• Sodium Chloride 0.9% Flush 10-20 mL IV 1.0x/day

Prescriptions Ordered: August 12, 2188 at 02:00 PM

1 medication(s) prescribed:

• 0.9% Sodium Chloride (Mini Bag Plus) 100 mL IV 1.0x/day

Discharge Summary

Discharge Time: August 12, 2188 at 05:49 PM

The patient was discharged from **Medicine** with disposition to **Home With Service**.

Major Procedure(s) During Admission: ['Incision and drainage ()', 'Right UE PICC line placement ()']

Known Allergies: Penicillins

Detailed Allergy List:

• Penicillins

Discharge Diagnoses:

Primary Diagnoses:

• Right foot diabetic foot ulcer

• Osteomyelitis

Secondary Diagnoses:

• Sepsis

• Type 2 Diabetes

• uncontrolled

• Acute on chronic kidney injury

• Hypertension

• Anemia

Complete Diagnosis List:

• Diabetes with other specified manifestations, type II or unspecified type, uncontrolled

• Streptococcal septicemia

• Sepsis

• Diabetes with neurological manifestations, type II or unspecified type, uncontrolled

• Ulcer of other part of foot

• Unspecified osteomyelitis, ankle and foot

• Other bone involvement in diseases classified elsewhere

• Acute kidney failure, unspecified

• Chronic kidney disease, Stage III (moderate)

• Methicillin susceptible Staphylococcus aureus in conditions classified elsewhere and of unspecified site

• Hypertensive chronic kidney disease, unspecified, with chronic kidney disease stage I through stage IV, or unspecified

• Anemia, unspecified

• Obesity, unspecified

• Pure hypercholesterolemia

• Long-term (current) use of aspirin

• Need for prophylactic vaccination and inoculation against influenza

• Family history of diabetes mellitus

Hospital Course:

PRIMARY REASON FOR HOSPITALIZATION: y/o male with PMHx of poorly controlled DM2 (HbA1c was 13.6), HTN, HLD presented with infected diabetic foot ulcer and sepsis. ACTIVE ISSUES: # Diabetic foot ulcer: Patient presented with a chronic foot ulcer of his right foot. He had been seen clinic earlier that day and labs drawn were significant for a leukocytosis; he was thus sent to the ED for evaluation. the ED initial vitals were: 99%. He spiked a fever to 102.5. Labs were significant for WBC of 17.7 with 90% PMNs, Na of 126, Cr of 2.1, glucose 500s. Lactate 1.4. He was placed on vanc/cipro/flagyl. Podiatry performed bedside I&D; on . Blood cultures grew group B strep, group G strep. These also grew out of wound cultures; MSSA also grew out of the wound. He was taken back to the OR for further I&D; on and . Surface ECHO was performed and negative for any mass or vegetation. WBC trended down, though did have slight uptrend prior to discharge (14.2 on day of discharge). He was hemodynamically stable throughout his stay. He was discharged on ertapenem 1g q24hrs at home, to be taken for another 6-weeks, with stop date of . # Type 2 diabetes: uncontrolled, last HbA1c was 13.6 (). Patient was placed on his home medication which was Lantus 40units qAM. Initially his FSBGs were high, requiring large amounts of sliding scale correction at mealtimes. However as his infection was controlled, his FSBGs were less that 200. Given that not much adjustment was made to his insulin from home, yet his sugars were much better controlled than his HbA1c suggested, it was thought that his dietary discretion at home may be playing a significant role. He was seen by nutrition for diet education. He will follow-up with his PCP for further titration of his insulin regimen. Given drug interaction, his preadmission simvastain was stopped and he was placed on atorvastatin 20mg daily. He was continued on his ASA 81mg daily. # Acute on chronic kidney injury - Cr on admission up to 2.1 from baseline ~1.4-1.6. Likely due to hypovolemia from sepsis as has improved with IVFs and ABx. At discharge his Cr was 1.7. # HTN: Prior to admission the patient was maintained on atenolol and lisinopril. His antihypertensives were intitially held due to severe infection. He was eventually placed back on lisinopril and uptitrated to 10mg daily. Additionally he was started on amlodipine 5mg daily. His atenolol was held given his CKD. His BPs were maintained at SBP 120-130s on this regimen. # Anemia: Review of records show baseline Hbg of around 10. Labs from consistent with chronic inflammation, which is likely still the case given his chronic ulcers and current osteomyelitis. Hbg remained stable at during this hospitalization.

Microbiology Findings:

8:52 pm BLOOD CULTURE #1. **FINAL REPORT Blood Culture, Routine (Final : BETA STREPTOCOCCUS GROUP G. Anaerobic Bottle Gram Stain (Final : GRAM POSITIVE COCCI PAIRS AND CHAINS. Reported to and read back by 1420. 10:00 pm BLOOD CULTURE **FINAL REPORT Blood Culture, Routine (Final : ANAEROBIC GRAM POSITIVE COCCUS(I). (formerly Peptostreptococcus species). Isolated from only one set the previous five days. NO FURTHER WORKUP WILL BE PERFORMED. Anaerobic Bottle Gram Stain (Final : GRAM POSITIVE COCCI PAIRS AND CLUSTERS. Reported to and read back by 1020. 10:30 pm SWAB Source: R foot wcx. **FINAL REPORT GRAM STAIN (Final : 1+ (<1 per 1000X FIELD): POLYMORPHONUCLEAR LEUKOCYTES. 3+ (per 1000X FIELD): GRAM POSITIVE COCCI. PAIRS AND CLUSTERS. 1+ (<1 per 1000X FIELD): GRAM NEGATIVE ROD(S). 1+ (<1 per 1000X FIELD): GRAM POSITIVE ROD(S). WOUND CULTURE (Final : MIXED BACTERIAL FLORA. This culture contains mixed bacterial types (>=3) so an abbreviated workup is performed. Any growth of P.aeruginosa, S.aureus and beta hemolytic streptococci will be reported. IF THESE BACTERIA ARE NOT REPORTED BELOW, THEY ARE NOT PRESENT this culture.. BETA STREPTOCOCCUS GROUP G. MODERATE GROWTH. BETA STREPTOCOCCUS GROUP B. SPARSE GROWTH. STAPH AUREUS COAG +. SPARSE GROWTH. SENSITIVITIES: MIC expressed MCG/ML STAPH AUREUS COAG + | CLINDAMYCIN---------- =>8 R ERYTHROMYCIN---------- =>8 R GENTAMICIN---------- <=0.5 S LEVOFLOXACIN---------- =>8 R OXACILLIN----------<=0.25 S TETRACYCLINE---------- <=1 S TRIMETHOPRIM/SULFA---- <=0.5 S ANAEROBIC CULTURE (Final : NO ANAEROBES ISOLATED. 4:55 pm BLOOD CULTURE **FINAL REPORT Blood Culture, Routine (Final : NO GROWTH. 7:10 pm BLOOD CULTURE **FINAL REPORT Blood Culture, Routine (Final : NO GROWTH. 6:29 am BLOOD CULTURE **FINAL REPORT Blood Culture, Routine (Final : NO GROWTH. 10:12 am BLOOD CULTURE SET#2. **FINAL REPORT Blood Culture, Routine (Final : NO GROWTH. 7:55 am TISSUE Site: BONE RIGHT FOOT BONE. **FINAL REPORT GRAM STAIN (Final : 3+ (per 1000X FIELD): POLYMORPHONUCLEAR LEUKOCYTES. 4+ (>10 per 1000X FIELD): GRAM POSITIVE COCCI. PAIRS AND CHAINS. 2+ (per 1000X FIELD): GRAM POSITIVE ROD(S). 1+ (<1 per 1000X FIELD): GRAM NEGATIVE ROD(S). TISSUE (Final : MIXED BACTERIAL FLORA. This culture contains mixed bacterial types (>=3) so an abbreviated workup is performed. Any growth of P.aeruginosa, S.aureus and beta hemolytic streptococci will be reported. IF THESE BACTERIA ARE NOT REPORTED BELOW, THEY ARE NOT PRESENT this culture.. BETA STREPTOCOCCUS GROUP G. MODERATE GROWTH. IDENTIFICATION PERFORMED ON CULTURE # . BETA STREPTOCOCCUS GROUP B. Isolated from broth media only, INDICATING VERY LOW NUMBERS OF ORGANISMS. ANAEROBIC CULTURE (Final : NO ANAEROBES ISOLATED.

Antibiotic Plan:

Discharged on Ertapenem 1g q24hrs, with stop date of . All questions regarding outpatient parenteral antibiotics should be directed to the Infectious Disease RNs at .

Discharge Status:

Activity Status	Ambulatory - Independent.
Level of Consciousness	Alert and interactive.
Mental Status	Clear and coherent.
Code Status	N/A

Discharge Instructions:

Dear Mr. , It was a pleasure to care for you here at . You were admitted on for a serious right foot infection. Because the infection invloved your bone, you were placed on IV antibiotics and were taken to the operating room to remove infected tissue. You will continue IV antibiotics at home for 6-weeks, with a stop date of . You will continue to be seen as an outpatient by the Infectious Disease Department. Your blood pressure medications were changed. Please see below for medication changes. Controlling your diabetes will be a very important part of your recovery and important means to prevent future foot infections. We urge you to pay close attention to your diet and work with your Primary Care Physician to control your blood sugar. It was great to meet you. We wish you all the best. -Your Team

New Medications Started:

• Amlodipine was started at 5mg daily.

Medications Stopped:

• Atenolol was stopped.

• Simvastatin was changed to atorvastatin 20mg daily (given interaction between simvastatin and amlodipine).

*Time gap of **283 days, 5 hours, 28 minutes** since last event*

Emergency Department Visit #5

Admission Time: May 22, 2189 at 11:18 PM

This is the patient's **5th** Emergency Department visit. The patient arrived via **AMBULANCE**. The patient departed at **2189-05-23 03:38:00**, staying for a duration of **0 days 4 hours 20 minutes**.

Initial Assessment:

Chief Complaint: N/V, Tachycardia

Blood Pressure	139.0/70.0 mmHg
Heart Rate	140.0 bpm
Respiratory Rate	20.0 breaths/min
Oxygen Saturation	95.0%
Temperature	100.3°F
Pain Score	0
Acuity Level	1.0

Laboratory Tests Performed: May 22, 2189 at 11:35 PM

Total of **35** tests performed, with **15** abnormal result(s).

Chemistry

Test	Value	Reference Range
Light Green Top Hold	N/A Blood	N/A
Blue Top Hold	N/A Blood	N/A
Alanine Aminotransferase (ALT)	21.0IU/L	0.0-40.0
Albumin	3.8g/dL	3.5-5.2
Alkaline Phosphatase	123.0IU/L	40.0-130.0
Anion Gap	20.0mEq/L	8.0-20.0
Asparate Aminotransferase (AST)	25.0IU/L	0.0-40.0

Bicarbonate	**21.0mEq/L**	22.0-32.0
Bilirubin, Total	0.6mg/dL	0.0-1.5
C-Reactive Protein	**87.8mg/L**	0.0-5.0
Calcium, Total	9.2mg/dL	8.4-10.3
Chloride	**91.0mEq/L**	96.0-108.0
Creatinine	**2.7mg/dL**	0.5-1.2
Estimated GFR (MDRD equation)	N/A Blood	N/A
Glucose	**314.0mg/dL**	70.0-100.0
Lipase	**171.0IU/L**	0.0-60.0
Magnesium	1.6mg/dL	1.6-2.6
Phosphate	2.8mg/dL	2.7-4.5
Potassium	**5.3mEq/L**	3.3-5.1
Sodium	**127.0mEq/L**	133.0-145.0
Urea Nitrogen	**47.0mg/dL**	6.0-20.0

Hematology

Test	Value	Reference Range
Basophils	0.7%	0.0-2.0
Eosinophils	0.6%	0.0-4.0
Hematocrit	**30.3%**	40.0-52.0
Hemoglobin	**10.5g/dL**	14.0-18.0
Lymphocytes	**3.7%**	18.0-42.0
MCH	28.7pg	27.0-32.0
MCHC	34.7%	31.0-35.0
MCV	83.0fL	82.0-98.0
Monocytes	2.8%	2.0-11.0
Neutrophils	**92.2%**	50.0-70.0
Platelet Count	262.0K/uL	150.0-440.0

RDW	14.0%	10.5-15.5
Red Blood Cells	**3.67m/uL**	4.6-6.2
White Blood Cells	**11.9K/uL**	4.0-11.0

Medications Administered: May 22, 2189 at 11:40 PM

Total of **4** medication(s) administered:

• Ondansetron

• Ondansetron

• Acetaminophen

• CefePIME

Laboratory Tests Performed: May 22, 2189 at 11:40 PM

Total of **7** tests performed, with **4** abnormal result(s).

Blood Gas

Test	Value	Reference Range
Base Excess	0.0mEq/L Blood	N/A
Calculated Total CO2	21.0mEq/L	21.0-30.0
Lactate	**2.4mmol/L**	0.5-2.0
pCO2	**27.0mm Hg**	35.0-45.0
pH	**7.49units**	7.35-7.45
pO2	**51.0mm Hg**	85.0-105.0
Specimen Type	N/A Blood	N/A

Procedure(s) Performed: May 23, 2189 at 12:00 AM

1 procedure(s) performed (Source: ICD):

• Excisional debridement of wound, infection, or burn

Previous Medications on Record: May 23, 2189 at 12:18 AM

10 medication(s) documented:

• amlodipine - Calcium Channel Blockers - Dihydropyridines

• aspirin - Salicylate Analgesics

• aspirin - Platelet Aggregation Inhibitors - Salicylates

• atorvastatin - Antihyperlipidemic - HMG CoA Reductase Inhibitors (statins)

• insulin lispro - Insulin Analogs - Rapid Acting

• lisinopril - ACE Inhibitors

• omeprazole - Gastric Acid Secretion Reducing Agents - Proton Pump Inhibitors (PPIs)

• oxycodone-acetaminophen - Analgesic Opioid Oxycodone Combinations

• oxycodone-acetaminophen - Analgesic Opioid Oxycodone and Non-Salicylate Combinations

• sildenafil - Pulmonary Arterial Hypertension Agents-Selective cGMP-PDE5 Inhibitors

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*Time gap of **1 hour, 56 minutes** since last event*

Hospital Admission #6

Admission Time: May 23, 2189 at 02:14 AM

This is the patient's **6th** hospital admission. The patient was admitted from **EMERGENCY ROOM** as an **EW EMER.** admission. The admitting provider was **P647R8**. Insurance coverage: **Private**. Chief complaint: **Fever, n/v**. The primary service provided was **MEDICINE**.

Race: **WHITE**, Marital Status: **SINGLE**, Language: **English**

APR DRG Code:

Code: 720

Description: SEPTICEMIA AND DISSEMINATED INFECTIONS

Severity: 3.0

Mortality Risk: 2.0

HCFA DRG Code:

Code: 854

Description: INFECTIOUS & PARASITIC DISEASES W O.R. PROCEDURE W CC

Patient Past History:

Past Medical History: DM2 on insulin, HbA1c 14.7%, uncontrolled, with diabetic neuropathy, CKD stage 3, Right diabetic foot ulcer with history of sepsis, Anemia, Hypertension, Hypercholesterolemia, s/p

Appendectomy

Family History: Mother with colon cancer and hyperlipidemia. Father with type 2 DM.

Social History: N/A

History of Present Illness:

with uncontrolled DM2 on insulin, CKD, presents with worsening diabetic foot ulcer pain, swelling, with fever and tachycardia. He was seen by his primary care PA today for routine followup. He was tachycardic there, but declined ECG as he had to go to work. His foot ulcer was not examined. He had routine labs drawn. He admits to one week of worsening foot ulcer pain, tenderness, and swelling without clear inciting factors. He is able to bear weight and walk on it. He presented to ED tonight because of nausea, vomiting, and low grade fever, all of which developed hours prior. He denies any abdominal pain or diarrhea. His FSBS have been elevated, 190 this morning. Recent labs from at show HgA1c 14.7%, Na 132, K 4.4, Cl 97, CO2 25, BUN 33, Cr 2.0 (eGFR 36), Glu 396. He is followed by Podiatry for his chronic diabetic foot ulcer. Of note, he was admitted from to for sepsis due to diabetic foot ulcer and bacteremia requiring OPAT ertapenem for 6 weeks. In the ED, initial vitals were T100.3 (Tm101.7) HR140 BP139/70 RR20 95%RA. ECG with sinus tachycardia. Labs notable for WBC 11.9 (92% PMN), H/H 10.5/30.3 (baseline), Plt 262. Chem-7 with Na 127, K 5.3, Cl 91, CO2 21, BUN 47, Cr 2.7. Glu 314, AG 15. LFTs normal, lipase 171 (ULN 60). VBG 7.49, pCO2 27. Lactate 2.4. UA with few bacteria, 1 WBC, 1000 glucose, neg leuk, neg nitrite. He was given 2L NS, cefepime 2g, ondansetron, and acetaminophen prior to his transfer. On the floor, patient has no acute complaints.

Admission Vital Signs:

General Appearance	Pleasant
Blood Pressure	107/88 mmHg
Heart Rate	93 bpm
Respiratory Rate	19 breaths/min
Temperature	98.8°F
Oxygen Saturation	100RA

Admission Laboratory Results:

• WBC: 11.9

• Creatinine: 2.7

• Glucose: 314

• Lactate: 2.4

• Sodium: 127

• Potassium: 5.3

Admission Medications:

Total of **5** medications on admission:

• Aspirin 81 mg PO DAILY

• Amlodipine 10 mg PO DAILY

• Atorvastatin 40 mg PO DAILY

• Lisinopril 10 mg PO DAILY

• Glargine 55 Units Breakfast

Unit Admission - Cardiac Surgery

Admission Time: May 23, 2189 at 03:38 AM

The patient was admitted to **Cardiac Surgery** ward. The patient stayed in this unit until **2189-05-25 14:49:44**, for a total duration of **2 days 11 hours 11 minutes**. During this time, various procedures, laboratory tests, and microbiology events were performed.

Prescriptions Ordered: May 23, 2189 at 04:00 AM

1 medication(s) prescribed:

• Ibuprofen 600 mg PO 1.0x/day

Microbiology Tests Performed: May 23, 2189 at 04:13 AM

Total of **2** microbiology result(s):

WOUND CULTURE: FOOT CULTURE:

• BETA STREPTOCOCCUS GROUP B

• MIXED BACTERIAL FLORA

Microbiology Tests Performed: May 23, 2189 at 04:40 AM

Total of **1** microbiology result(s):

Blood Culture, Routine: BLOOD CULTURE:

• NO GROWTH.

Prescriptions Ordered: May 23, 2189 at 05:00 AM

14 medication(s) prescribed:

• Influenza Vaccine Quadrivalent 0.5 mL IM

• Sodium Chloride 0.9% Flush 3 mL IV 3.0x/day

• Acetaminophen 650 mg PO/NG

• Ondansetron 4 mg PO/NG

• 0.9% Sodium Chloride (Mini Bag Plus) 100 mL IV 1.0x/day

• Atorvastatin 40 mg PO/NG 1.0x/day

• Sodium Chloride 0.9% 1000 mL IV

• Dextrose 50% 12.5 gm IV

• Insulin 0 UNIT SC

• Aspirin 81 mg PO/NG 1.0x/day

• Senna 8.6 mg PO/NG

• Glucagon 1 mg IM

• Heparin 5000 UNIT SC 3.0x/day

• Glucose Gel 15 g PO

Prescriptions Ordered: May 23, 2189 at 08:00 AM

3 medication(s) prescribed:

• Docusate Sodium 100 mg PO/NG 2.0x/day

• Insulin 55 UNIT SC 1.0x/day

• Iso-Osmotic Dextrose 200 mL IV 1.0x/day

Microbiology Tests Performed: May 23, 2189 at 08:40 AM

Total of **4** microbiology result(s):

WOUND CULTURE: SWAB:

• BETA STREPTOCOCCUS GROUP B

• MIXED BACTERIAL FLORA

ANAEROBIC CULTURE: SWAB:

• MIXED BACTERIAL FLORA

GRAM STAIN: SWAB:

• [redacted]

Prescriptions Ordered: May 23, 2189 at 09:00 AM

2 medication(s) prescribed:

• 0.9% Sodium Chloride (Mini Bag Plus) 100 mL IV 3.0x/day

• Insulin 0 UNIT SC

Microbiology Tests Performed: May 23, 2189 at 09:15 AM

Total of **1** microbiology result(s):

Blood Culture, Routine: BLOOD CULTURE:

• NO GROWTH.

Prescriptions Ordered: May 23, 2189 at 11:00 AM

2 medication(s) prescribed:

• Insulin 15 UNIT SC 1.0x/day

• Sodium Chloride 0.9% 1000 mL IV

Prescriptions Ordered: May 23, 2189 at 05:00 PM

1 medication(s) prescribed:

• Sodium Chloride 0.9% 1000 mL IV

Prescriptions Ordered: May 24, 2189 at 05:00 PM

1 medication(s) prescribed:

• 0.9% Sodium Chloride (Mini Bag Plus) 100 mL IV 1.0x/day

Prescriptions Ordered: May 24, 2189 at 08:00 PM

1 medication(s) prescribed:

• Aquaphor Ointment 1 Appl TP 2.0x/day

Prescriptions Ordered: May 25, 2189 at 08:00 AM

1 medication(s) prescribed:

• Amlodipine 10 mg PO/NG 1.0x/day

Discharge Summary

Discharge Time: May 25, 2189 at 02:49 PM

The patient was discharged from **Cardiac Surgery** with disposition to **Home With Service**.

Major Procedure(s) During Admission: Bedside debridement of R plantar ulcer

Known Allergies: Penicillins

Detailed Allergy List:

- **Penicillins**

Discharge Diagnoses:

Primary Diagnoses:

- sepsis
- infected diabetic foot ulcer
- acute on chronic renal disease

Secondary Diagnoses:

- insulin-dependent diabetes

Complete Diagnosis List:

- Unspecified septicemia
- Acute kidney failure, unspecified
- Diabetes with other specified manifestations, type II or unspecified type, uncontrolled
- Diabetes with neurological manifestations, type II or unspecified type, uncontrolled
- Chronic kidney disease, Stage III (moderate)
- Hyposmolality and/or hyponatremia
- Need for prophylactic vaccination and inoculation against influenza
- Ulcer of other part of foot
- Long-term (current) use of insulin
- Polyneuropathy in diabetes
- Hypertensive chronic kidney disease, unspecified, with chronic kidney disease stage I through stage IV, or unspecified
- Pure hypercholesterolemia
- Sepsis
- Other toe(s) amputation status
- Long-term (current) use of aspirin
- Nausea with vomiting

• Other specified cardiac dysrhythmias

• Hyperpotassemia

Hospital Course:

with h/o poorly controlled IDDM, CKD, chronic R diabetic foot ulcer c/b osteo presents with drainage from foot ulcer, fever and n/v. >> Active issues: # Sepsis diabetic foot ulcer: Pt tachycardic with leukocytosis and fever on presentation. SBPs soft in 90-100s. Only notable source is R foot ulcer which had purulent drainage. Pt given 3.5L IVF initially for resuscitation. He was put on empiric . Podiatry consulted who did a bedside debridement and reports the ulcer did not probe to bone. Given sepsis and concern for underlying osteo given pt history, ID was consulted who recommended 72hrs of IV antibiotics and then reasonable to complete 2wks of oral ABX. Pt completed 72hrs of IV therapy on day of discharge (CTX and on D3, and for days . Pt will complete course with Levoflox on discharge for total of 2wks course. # : Cr 2.9 from baseline around 1.5-1.7. Likely prerenal in the setting of sepsis and vomiting. No documented hypotension but ATN also possible. Lisinopril held. Cr was 2.2 on discharge. Held lisinopril until repeat lytes could be checked by PCP at . # IDDM: Poorly controlled, HbA1c 14.7%. Complicated by diabetic neuropathy and foot ulcers. Continued home lantus 55u QAM. HSSI # Hyperkalemia: Likely due to acute renal failure. No ECG changes. # Hyponatremia: Likely due to hypovolemia given sepsis. Improved with IVF resuscitation # Hypertension: Antihypertensives held and restarted as needed. # Hypercholesterolemia: Stable. Continued atorava

Microbiology Findings:

tissue R foot (debrided) SOURCE:R FOOT ULCER PLEASE SEE NOTE WITH CHANGE TO ,"RIGHT FOOT.". GRAM STAIN (Final : NO POLYMORPHONUCLEAR LEUKOCYTES SEEN. 3+ per 1000X FIELD): GRAM POSITIVE COCCI IN PAIRS. 1+ (<1 per 1000X FIELD): GRAM POSITIVE ROD(S). WOUND CULTURE (Preliminary): ANAEROBIC CULTURE (Preliminary): . bl cx pending. ucx < 10K org.

Antibiotic Plan:

ID evaluated pt and recommended 2wk course of ABX (PO levoflox after 72hrs of IV antibiotics during admission) to end .

Discharge Status:

Activity Status	Ambulatory - Independent.
Level of Consciousness	Alert and interactive.
Mental Status	Clear and coherent.
Code Status	Full, discussed with patient

Discharge Instructions:

Dear Mr. , It was a pleasure taking care of you in the hospital. You came in with fevers and drainage from your right foot ulcer that seemed infected. You were put on antibiotics and the podiatrists debrided your ulcer at the

bedside. You completed 72hrs of IV antibiotics and will need to complete a total 2wk course of oral antibiotics at home starting tomorrow. Please at the appointments listed below. Please see the attached list for updates to your home medications. Please note the following medication changes: - START Levofloxacin 750mg daily on until - STOP Lisinopril until you see your PCP next week and get repeat labs to check your kidney function You should do wet-to-dry dressings daily to your right foot ulcer. A visiting nurse help with this. You should also try to put your weight on the heel of your right foot per podiatry recommendations until your appointment next week.

New Medications Started:

• Levofloxacin 750mg daily on until

Medications Stopped:

• Lisinopril until you see your PCP next week and get repeat labs to check your kidney function

*Time gap of **10 days, 22 hours, 35 minutes** since last event*

Emergency Department Visit #6

Admission Time: June 05, 2189 at 01:25 PM

This is the patient's **6th** Emergency Department visit. The patient arrived via **AMBULANCE**. The patient departed at **2189-06-05 18:06:00**, staying for a duration of **0 days 4 hours 41 minutes**.

ED Disposition: HOME - Patient was discharged directly from Emergency Department without hospital admission.

Initial Assessment:

Chief Complaint: Dizziness

Blood Pressure	94.0/62.0 mmHg
Heart Rate	101.0 bpm
Respiratory Rate	18.0 breaths/min
Oxygen Saturation	99.0%
Temperature	97.2°F
Pain Score	0
Acuity Level	1.0

Emergency Department Discharge Summary

Treatment: The patient received evaluation and treatment in the Emergency Department. Medical staff determined the patient was stable for discharge to **HOME**.

Follow-up Instructions: Patient advised to follow up with primary care physician or return to ED if symptoms worsen.

Previous Medications on Record: June 05, 2189 at 02:32 PM

11 medication(s) documented:

• amlodipine - Calcium Channel Blockers - Dihydropyridines

• aspirin - Salicylate Analgesics

• aspirin - Platelet Aggregation Inhibitors - Salicylates

• atorvastatin - Antihyperlipidemic - HMG CoA Reductase Inhibitors (statins)

• insulin lispro - Insulin Analogs - Rapid Acting

• lisinopril - ACE Inhibitors

• omeprazole - Gastric Acid Secretion Reducing Agents - Proton Pump Inhibitors (PPIs)

• oxycodone-acetaminophen - Analgesic Opioid Oxycodone Combinations

• oxycodone-acetaminophen - Analgesic Opioid Oxycodone and Non-Salicylate Combinations

• sildenafil - Pulmonary Arterial Hypertension Agents-Selective cGMP-PDE5 Inhibitors

• levofloxacin - Fluoroquinolone Antibiotics

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*Time gap of **115 days, 24 minutes** since last event*

Emergency Department Visit #7

Admission Time: September 28, 2189 at 02:56 PM

This is the patient's **7th** Emergency Department visit. The patient arrived via **WALK IN**. The patient departed at **2189-09-28 17:49:00**, staying for a duration of **0 days 2 hours 53 minutes**.

ED Disposition: HOME - Patient was discharged directly from Emergency Department without hospital admission.

Initial Assessment:

Chief Complaint: Foot laceration

Blood Pressure	136.0/81.0 mmHg
Heart Rate	105.0 bpm
Respiratory Rate	16.0 breaths/min
Oxygen Saturation	100.0%
Temperature	98.0°F
Pain Score	0
Acuity Level	3.0

Emergency Department Discharge Summary

Treatment: The patient received evaluation and treatment in the Emergency Department. Medical staff determined the patient was stable for discharge to **HOME**.

Follow-up Instructions: Patient advised to follow up with primary care physician or return to ED if symptoms worsen.

Previous Medications on Record: September 28, 2189 at 03:40 PM

9 medication(s) documented:

• amlodipine - Calcium Channel Blockers - Dihydropyridines

• aspirin - Salicylate Analgesics

• aspirin - Platelet Aggregation Inhibitors - Salicylates

• atorvastatin - Antihyperlipidemic - HMG CoA Reductase Inhibitors (statins)

• insulin lispro - Insulin Analogs - Rapid Acting

• lisinopril - ACE Inhibitors

• omeprazole - Gastric Acid Secretion Reducing Agents - Proton Pump Inhibitors (PPIs)

• sildenafil - Pulmonary Arterial Hypertension Agents-Selective cGMP-PDE5 Inhibitors

• Lantus - Insulin Analogs - Long Acting

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*Time gap of **228 days, 1 hour, 31 minutes** since last event*

Emergency Department Visit #8

Admission Time: May 14, 2190 at 05:11 PM

This is the patient's **8th** Emergency Department visit. The patient arrived via **AMBULANCE**. The patient departed at **2190-05-15 10:00:00**, staying for a duration of **0 days 16 hours 49 minutes**.

Initial Assessment:

Chief Complaint: Abnormal labs, Hyperglycemia

Blood Pressure	179.0/108.0 mmHg
Heart Rate	110.0 bpm
Respiratory Rate	18.0 breaths/min
Oxygen Saturation	97.0%

Temperature	98.0°F
Pain Score	0
Acuity Level	2.0

Laboratory Tests Performed: May 14, 2190 at 06:00 PM

Total of **34** tests performed, with **14** abnormal result(s).

Chemistry

Test	Value	Reference Range
Green Top Hold (plasma)	N/A Blood	N/A
Anion Gap	16.0mEq/L	8.0-20.0
Bicarbonate	24.0mEq/L	22.0-32.0
Chloride	**92.0mEq/L**	96.0-108.0
Creatinine	**3.1mg/dL**	0.5-1.2
Estimated GFR (MDRD equation)	N/A Blood	N/A
Glucose	**473.0mg/dL**	70.0-100.0
Potassium	4.0mEq/L	3.3-5.1
Sodium	**128.0mEq/L**	133.0-145.0
Urea Nitrogen	**42.0mg/dL**	6.0-20.0

Hematology

Test	Value	Reference Range
Absolute Lymphocyte Count	1.72K/uL	1.2-3.7
Basophils	0.9%	0.0-1.0
Eosinophils	3.3%	1.0-7.0
Hematocrit	**30.6%**	40.0-51.0
Hemoglobin	**10.4g/dL**	13.7-17.5
Lymphocytes	**14.2%**	19.0-53.0
MCH	27.2pg	26.0-32.0

MCHC	34.0g/dL	32.0-37.0
MCV	**80.0fL**	82.0-98.0
Monocytes	6.5%	5.0-13.0
Neutrophils	**74.3%**	34.0-71.0
Platelet Count	321.0K/uL	150.0-400.0
RDW	13.3%	10.5-15.5
Red Blood Cells	**3.83m/uL**	4.6-6.1
White Blood Cells	**12.1K/uL**	4.0-10.0
Absolute Basophil Count	**0.11K/uL**	0.01-0.08
Absolute Eosinophil Count	0.4K/uL	0.04-0.54
Absolute Monocyte Count	0.79K/uL	0.2-0.8
Absolute Neutrophil Count	**8.96K/uL**	1.6-6.1
Immature Granulocytes	0.8% Blood	N/A
RDW-SD	38.2fL	35.1-46.3
INR(PT)	1.0	0.9-1.1
PT	11.1sec	9.4-12.5
PTT	30.3sec	25.0-36.5

Previous Medications on Record: May 14, 2190 at 07:48 PM

7 medication(s) documented:

• amlodipine - Calcium Channel Blockers - Dihydropyridines

• aspirin - Salicylate Analgesics

• aspirin - Platelet Aggregation Inhibitors - Salicylates

• atorvastatin - Antihyperlipidemic - HMG CoA Reductase Inhibitors (statins)

• lisinopril - ACE Inhibitors

• sildenafil - Pulmonary Arterial Hypertension Agents-Selective cGMP-PDE5 Inhibitors

• Lantus - Insulin Analogs - Long Acting

Laboratory Tests Performed: May 14, 2190 at 10:15 PM

Total of **8** tests performed, with **6** abnormal result(s).

Chemistry

Test	Value	Reference Range
Anion Gap	13.0mEq/L	8.0-20.0
Bicarbonate	26.0mEq/L	22.0-32.0
Chloride	**95.0mEq/L**	96.0-108.0
Creatinine	**3.0mg/dL**	0.5-1.2
Glucose	**462.0mg/dL**	70.0-100.0
Potassium	**5.2mEq/L**	3.3-5.1
Sodium	**129.0mEq/L**	133.0-145.0
Urea Nitrogen	**40.0mg/dL**	6.0-20.0

Medications Administered: May 15, 2190 at 02:31 AM

Total of **4** medication(s) administered:

• Sulfameth/Trimeth DS

• Sulfameth/Trimeth DS

• Cephalexin

• Acetaminophen

Prescriptions Ordered: May 15, 2190 at 08:00 AM

1 medication(s) prescribed:

• Insulin 55 UNIT SC 1.0x/day

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*Time gap of **16 minutes** since last event*

Hospital Admission #7

Admission Time: May 15, 2190 at 08:16 AM

This is the patient's **7th** hospital admission. The patient was admitted from **EMERGENCY ROOM** as an **EU OBSERVATION** admission. The admitting provider was **P045WF**. Insurance coverage: **Private**. Chief complaint: **hyperglycemia**. The primary service provided was **MEDICINE**.

Race: **WHITE**, Marital Status: **SINGLE**, Language: **English**

Patient Past History:

Past Medical History: DM2 on insulin, HbA1c 14.7%, uncontrolled, with diabetic neuropathy, CKD stage 3, Right diabetic foot ulcer with history of sepsis, Anemia, Hypertension, Hypercholesterolemia, s/p Appendectomy

Family History: Mother with colon cancer and hyperlipidemia. Father with type 2 DM.

Social History: N/A

History of Present Illness:

PCP: Name: . Location: Address: , Phone: Fax: yo M with poorly controlled DM2 with nephropathy, neuropathy, CKD III, HTN, who presents with hyperglycemia and R foot ulcer. He had recently fallen and broke bones in his L hand. This was splinted. He also noted a R foot ulcer. He denies any new pain or drainage from the ulcer. He denies f/c, CP, SOB, n/v/d, abd pain. He denies new leg swelling. He presented to this PCP and his were found to be in the 400-500s. He was referred into the ED. 10 point review of systems reviewed, otherwise negative except as listed above

Admission Vital Signs:

General Appearance	well appearing in NAD
Blood Pressure	N/A mmHg
Heart Rate	N/A bpm
Respiratory Rate	N/A breaths/min
Temperature	N/A°F
Oxygen Saturation	N/A

Admission Laboratory Results:

• WBC: 12.1*

Admission Medications:

Total of **6** medications on admission:

• Lantus 55 units daily

• Lipitor 40mg daily

• Amlodipine 10mg daily

• Lisinpril 10mg daily

• MVI

• Aspirin 81mg daily

Unit Admission - Hematology/Oncology

Admission Time: May 15, 2190 at 10:00 AM

The patient was admitted to **Hematology/Oncology** ward. The patient stayed in this unit until **2190-05-16 15:45:02**, for a total duration of **1 days 5 hours 45 minutes**. During this time, various procedures, laboratory tests, and microbiology events were performed.

Prescriptions Ordered: May 15, 2190 at 12:00 PM

7 medication(s) prescribed:

• Acetaminophen 325-650 mg PO/NG

• Sodium Chloride 0.9% 1000 mL IV

• Dextrose 50% 12.5 gm IV

• Sodium Chloride 0.9% Flush 3 mL IV 3.0x/day

• Glucose Gel 15 g PO

• Glucagon 1 mg IM

• Influenza Vaccine Quadrivalent 0.5 mL IM

Prescriptions Ordered: May 15, 2190 at 03:00 PM

5 medication(s) prescribed:

• Aspirin 81 mg PO/NG 1.0x/day

• Amlodipine 10 mg PO/NG 1.0x/day

• Atorvastatin 40 mg PO/NG 1.0x/day

• Levofloxacin 500 mg PO/NG

• OxycoDONE (Immediate Release) 5 mg PO/NG

Prescriptions Ordered: May 15, 2190 at 05:00 PM

1 medication(s) prescribed:

• Insulin 0 UNIT SC

Prescriptions Ordered: May 15, 2190 at 08:00 PM

1 medication(s) prescribed:

• Heparin 5000 UNIT SC 2.0x/day

Discharge Summary

Discharge Time: May 16, 2190 at 03:45 PM

The patient was discharged from **Hematology/Oncology** with disposition to **Home With Service**.

Known Allergies: Penicillins

Detailed Allergy List:

- **Penicillins**

Discharge Diagnoses:

Primary Diagnoses:

- Type 2 diabetes mellitus uncontrolled with nephropathy

Secondary Diagnoses:

- Foot ulcer
- CKD stage III

Complete Diagnosis List:

- Type 2 diabetes mellitus with hyperglycemia
- Type 2 diabetes mellitus with diabetic nephropathy
- Type 2 diabetes mellitus with foot ulcer
- Non-pressure chronic ulcer of other part of right foot with unspecified severity
- Type 2 diabetes mellitus with diabetic neuropathy, unspecified
- Long term (current) use of insulin
- Acute kidney failure, unspecified
- Hypertensive chronic kidney disease with stage 1 through stage 4 chronic kidney disease, or unspecified chronic kidney disease
- Chronic kidney disease, stage 3 (moderate)
- Pure hypercholesterolemia
- Personal history of nicotine dependence

Hospital Course:

with poorly controlled DM2, foot ulcer, presents with hyperglycemia and foot ulcer Hyperglycemia/DM2 with neph/neuropathy: Poorly controlled DM2 with past elevated A1C. He may also have acute stress response from infection though there does not appear to be clear infection. Another source of stress may be his recent fall and

fracture. his treatment is limited by his inability to realistically take more than Lantus. was consulted and he was maintained on Lantus with HISS. Aic >13. Social factors limit his ability to take aggressive insulin regimen. However, we convinced him to try HISS at home with a Humalog pen in addition to Lantus. his improved to 100-200s on discharge. referral can be recommended Foot ulcer, diabetic: XRay and podiatry consult reviewed. Hand fracture: Casted. - Decided to continue Levofloxacin previously prescribed to complete full course. Given his CKD his dose was renally adjusted to 500mg q48 ARF on CKD III: Likely pre-renal due to hyperglycemia. Improved with hydration. Essential HTN: Continued home regimen on discharge

Antibiotic Plan:

Continue Levofloxacin 500mg q48h to complete full course, renally adjusted for CKD.

Discharge Status:

Activity Status	Ambulatory - Independent.
Level of Consciousness	Alert and interactive.
Mental Status	Clear and coherent.
Code Status	N/A

Discharge Instructions:

You were admitted with high blood sugars and a right foot ulcer. This was treated by the podiatry team. Please continue daily dressing changes. Please take your antibiotics as prescribed. In addition, we recommend that you begin short acting insulin when you eat depending on the sliding scale. Please follow up closely with your PCP and contact if you wish to be seen there. Please note your Lantus dose has been decreased while you are taking your sliding scale short acting insulin, Please continue your Levofloxacin at 500mg doses every other day because of your kidney function. Please take your 250mg pills

New Medications Started:

• Levofloxacin 500 mg PO Q48H

• HumaLOG KwikPen (insulin lispro) 100 unit/mL subcutaneous QAC

Medications Stopped:

• MVI

*Time gap of **243 days, 5 hours, 32 minutes** since last event*

Emergency Department Visit #9

Admission Time: January 14, 2191 at 09:18 PM

This is the patient's **9th** Emergency Department visit. The patient arrived via **WALK IN**. The patient departed at **2191-01-15 03:20:00**, staying for a duration of **0 days 6 hours 2 minutes**.

ED Disposition: HOME - Patient was discharged directly from Emergency Department without hospital admission.

Initial Assessment:

Chief Complaint: R Foot pain, Right foot pain, ulcer

Blood Pressure	107.0/64.0 mmHg
Heart Rate	117.0 bpm
Respiratory Rate	20.0 breaths/min
Oxygen Saturation	100.0%
Temperature	98.2°F
Pain Score	9
Acuity Level	3.0

Emergency Department Discharge Summary

Treatment: The patient received evaluation and treatment in the Emergency Department. Medical staff determined the patient was stable for discharge to **HOME**.

Follow-up Instructions: Patient advised to follow up with primary care physician or return to ED if symptoms worsen.

Previous Medications on Record: January 14, 2191 at 09:39 PM

12 medication(s) documented:

• amlodipine - Calcium Channel Blockers - Dihydropyridines

• aspirin - Salicylate Analgesics

• aspirin - Platelet Aggregation Inhibitors - Salicylates

• atorvastatin - Antihyperlipidemic - HMG CoA Reductase Inhibitors (statins)

• gabapentin - Anticonvulsant - GABA Analogs

• insulin lispro - Insulin Analogs - Rapid Acting

• lisinopril - ACE Inhibitors

• naproxen - NSAID Analgesics (COX Non-Specific) - Propionic Acid Derivatives

• omeprazole - Gastric Acid Secretion Reducing Agents - Proton Pump Inhibitors (PPIs)

• oxycodone-acetaminophen - Analgesic Opioid Oxycodone Combinations

• oxycodone-acetaminophen - Analgesic Opioid Oxycodone and Non-Salicylate Combinations

• sildenafil - Pulmonary Arterial Hypertension Agents-Selective cGMP-PDE5 Inhibitors

Medications Administered: January 15, 2191 at 12:11 AM

Total of **5** medication(s) administered:

• Oxycodone-Acetaminophen 1TAB TAB

• Ciprofloxa 400mg/200mL 200mL BAG

• MetroNIDAZ 500mg/100mL 100mL BAG

• Vancomyci 1000mg/200mL 200mL BAG

• Vancomyci 1000mg/200mL 200mL BAG

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*Time gap of **1 hour, 44 minutes** since last event*

Hospital Admission #8

Admission Time: January 15, 2191 at 01:55 AM

This is the patient's **8th** hospital admission. The patient was admitted from **EMERGENCY ROOM** as an **OBSERVATION ADMIT** admission. The admitting provider was **P86LZJ**. Insurance coverage: **Private**. Chief complaint: **Right foot pain, ulcer**. The primary service provided was **MEDICINE**.

Race: **WHITE**, Marital Status: **SINGLE**, Language: **English**

APR DRG Code:

Code: 720

Description: SEPTICEMIA AND DISSEMINATED INFECTIONS

Severity: 3.0

Mortality Risk: 3.0

HCFA DRG Code:

Code: 872

Description: SEPTICEMIA OR SEVERE SEPSIS W/O MV >96 HOURS W/O MCC

Patient Past History:

Past Medical History: DM2 on insulin, uncontrolled, with diabetic neuropathy, nephropathy, CKD stage 3, Right diabetic foot ulcer with history of sepsis, Anemia, Hypertension, Hypercholesterolemia, s/p Appendectomy

Family History: Mother with colon cancer and hyperlipidemia. Father with type 2 DM.

Social History: N/A

History of Present Illness:

M h/o poorly controlled DM2 c/b neuropathy, nephropathy, CKDIII, who presents with a persistent R foot ulcer and complaints of radiating pain up the right leg. The patient has had multiple recent visits with his outpatient podiatrist for foot ulcers in the last few months. On , he saw his podiatrist with R foot pain and ulcer, and they debrided to the subcutaneous and found that Xrays were not indicative of any bony involvement. At that time, he was prescribed Naproxen 500 mg bid for the pain, which he has been taking as prescribed since that admission. The patient otherwise denies f/c, chest pain, shortness of breath, n/v/d, abdominal pain. He does endorse a few weeks of loose stools that he relates to starting the naproxen.

Admission Vital Signs:

General Appearance	Alert
Blood Pressure	134/71 mmHg
Heart Rate	108 bpm
Respiratory Rate	18 breaths/min
Temperature	97.6°F
Oxygen Saturation	100%RA

Admission Laboratory Results:

• WBC: 24.0

• Glucose: 225

• Lactate: 1.0

Admission Medications:

Total of **7** medications on admission:

• amLODIPine 5 mg PO DAILY

• Aspirin 81 mg PO DAILY

• Atorvastatin 20 mg PO QPM

• Glargine 46 Units Breakfast Insulin SC Sliding Scale using HUM Insulin

• Naproxen 500 mg PO Q12H

• Sildenafil 20 mg PO DAILY:PRN sexual activity

• Labetalol 100 mg PO BID

Unit Admission - Vascular

Admission Time: January 15, 2191 at 03:20 AM

The patient was admitted to **Vascular** ward. The patient stayed in this unit until **2191-01-30 17:09:18**, for a total duration of **15 days 13 hours 49 minutes**. During this time, various procedures, laboratory tests, and microbiology events were performed.

Prescriptions Ordered: January 15, 2191 at 06:00 AM

14 medication(s) prescribed:

• Heparin 5000 UNIT SC 2.0x/day

• Glucagon 1 mg IM

• OxyCODONE (Immediate Release) 5 mg PO/NG

• Acetaminophen 650 mg PO/NG

• Aspirin 81 mg PO/NG 1.0x/day

• Dextrose 50% 12.5 gm IV

• Glucose Gel 15 g PO

• Insulin 46 UNIT SC 1.0x/day

• Sodium Bicarbonate 1300 mg PO/NG 1.0x/day

• Sodium Chloride 0.9% Flush 3 mL IV 3.0x/day

• 5% Dextrose 200 mL IV 1.0x/day

• NS 100 mL IV 3.0x/day

• Senna 8.6 mg PO/NG

• Atorvastatin 20 mg PO/NG 1.0x/day

Prescriptions Ordered: January 15, 2191 at 07:00 AM

1 medication(s) prescribed:

• 5% Dextrose 1000 mL IV

Prescriptions Ordered: January 15, 2191 at 08:00 AM

1 medication(s) prescribed:

• Docusate Sodium 100 mg PO/NG 2.0x/day

Microbiology Tests Performed: January 15, 2191 at 09:11 AM

Total of **1** microbiology result(s):

URINE CULTURE: URINE:

• <10,000 organisms/ml.

Prescriptions Ordered: January 15, 2191 at 10:00 AM

1 medication(s) prescribed:

• Sodium Chloride 0.9% 1000 mL IV

Prescriptions Ordered: January 15, 2191 at 05:00 PM

1 medication(s) prescribed:

• Sodium Chloride 0.9% 1000 mL IV

Prescriptions Ordered: January 15, 2191 at 08:00 PM

1 medication(s) prescribed:

• Sodium Bicarbonate 650 mg PO/NG 2.0x/day

Prescriptions Ordered: January 15, 2191 at 10:00 PM

1 medication(s) prescribed:

• Bag 1 BAG IV 1.0x/day

Microbiology Tests Performed: January 16, 2191 at 08:10 AM

Total of **1** microbiology result(s):

Blood Culture, Routine: BLOOD CULTURE:

• NO GROWTH.

Microbiology Tests Performed: January 16, 2191 at 08:30 AM

Total of **1** microbiology result(s):

Blood Culture, Routine: BLOOD CULTURE:

• NO GROWTH.

Prescriptions Ordered: January 16, 2191 at 10:00 AM

1 medication(s) prescribed:

• Cepacol (Sore Throat Lozenge) 1 LOZ PO

Prescriptions Ordered: January 16, 2191 at 12:00 PM

2 medication(s) prescribed:

• Insulin 0 UNIT SC

• Magnesium Oxide 400 mg PO/NG 1.0x/day

Prescriptions Ordered: January 16, 2191 at 05:00 PM

1 medication(s) prescribed:

• Acetaminophen 1000 mg PO/NG 3.0x/day

Prescriptions Ordered: January 16, 2191 at 08:00 PM

2 medication(s) prescribed:

• Iso-Osmotic Dextrose 200 mL IV

• Labetalol 100 mg PO/NG 2.0x/day

Microbiology Tests Performed: January 17, 2191 at 02:03 PM

Total of **1** microbiology result(s):

Blood Culture, Routine: BLOOD CULTURE:

• NO GROWTH.

Microbiology Tests Performed: January 17, 2191 at 03:00 PM

Total of **1** microbiology result(s):

C. difficile PCR: STOOL:

• Negative for toxigenic C. difficile by the Illumigene DNA amplification assay. (Reference Range-Negative).

Prescriptions Ordered: January 17, 2191 at 04:00 PM

1 medication(s) prescribed:

• 0.9% Sodium Chloride (Mini Bag Plus) 100 mL IV 1.0x/day

Prescriptions Ordered: January 17, 2191 at 08:00 PM

2 medication(s) prescribed:

• Acetaminophen IV 1000 mg IV 1.0x/day

• DiphenhydrAMINE 25 mg IV 1.0x/day

Prescriptions Ordered: January 17, 2191 at 09:00 PM

1 medication(s) prescribed:

• Sodium Chloride 0.9% 1000 mL IV

Microbiology Tests Performed: January 18, 2191 at 08:15 AM

Total of **1** microbiology result(s):

Blood Culture, Routine: BLOOD CULTURE:

• NO GROWTH.

Prescriptions Ordered: January 18, 2191 at 01:00 PM

1 medication(s) prescribed:

• LOPERamide 2 mg PO/NG

Prescriptions Ordered: January 18, 2191 at 02:00 PM

4 medication(s) prescribed:

• Heparin Flush (10 units/ml) 2 mL IV 1.0x/day

• Sodium Chloride 0.9% Flush 20 mL IV

• Lidocaine 1% (For PICC/Midline Insertions) 5 mL SC

• Readi-Cat 2 (Barium Sulfate 2% Suspension) 900 mL PO/NG 1.0x/day

Prescriptions Ordered: January 18, 2191 at 03:00 PM

3 medication(s) prescribed:

• Heparin Flush (10 units/ml) 2 mL IV

• Lidocaine 1% (For PICC/Midline Insertions) 5 mL SC

• Sodium Chloride 0.9% Flush 20 mL IV

Prescriptions Ordered: January 18, 2191 at 08:00 PM

1 medication(s) prescribed:

• Sodium Bicarbonate 1300 mg PO/NG 2.0x/day

Microbiology Tests Performed: January 19, 2191 at 06:45 AM

Total of **1** microbiology result(s):

Blood Culture, Routine: BLOOD CULTURE:

• NO GROWTH.

Prescriptions Ordered: January 19, 2191 at 01:00 PM

1 medication(s) prescribed:

• Insulin 35 UNIT SC 1.0x/day

Microbiology Tests Performed: January 19, 2191 at 03:55 PM

Total of **1** microbiology result(s):

Blood Culture, Routine: BLOOD CULTURE:

• NO GROWTH.

Microbiology Tests Performed: January 19, 2191 at 05:12 PM

Total of **1** microbiology result(s):

C. difficile PCR: STOOL:

• CANCELLED

Prescriptions Ordered: January 20, 2191 at 08:00 AM

1 medication(s) prescribed:

• Sodium Chloride 0.9% Flush 10-20 mL IV 1.0x/day

Prescriptions Ordered: January 20, 2191 at 10:00 AM

2 medication(s) prescribed:

• Insulin 0 UNIT SC

• Insulin 16 UNIT SC 1.0x/day

Prescriptions Ordered: January 20, 2191 at 02:00 PM

1 medication(s) prescribed:

• LOPERamide 2 mg PO/NG 3.0x/day

Microbiology Tests Performed: January 20, 2191 at 07:00 PM

Total of **8** microbiology result(s):

FECAL CULTURE: STOOL:

• NO ENTERIC GRAM NEGATIVE RODS FOUND. NO SALMONELLA OR SHIGELLA FOUND.

CAMPYLOBACTER CULTURE: STOOL:

• NO CAMPYLOBACTER FOUND.

OVA + PARASITES: STOOL:

• NO OVA AND PARASITES SEEN. This test does not reliably detect Cryptosporidium, Cyclospora or Microsporidium. While most cases of Giardia are detected by routine O+P, the Giardia antigen test may enhance detection when organisms are rare.

FECAL CULTURE - R/O VIBRIO: STOOL:

• NO VIBRIO FOUND.

FECAL CULTURE - R/O YERSINIA: STOOL:

• NO YERSINIA FOUND.

MICROSPORIDIA STAIN: STOOL:

• NO MICROSPORIDIUM SEEN.

CYCLOSPORA STAIN: STOOL:

• NO CYCLOSPORA SEEN.

Cryptosporidium/Giardia (DFA): STOOL:

• NO CRYPTOSPORIDIUM OR GIARDIA SEEN.

Prescriptions Ordered: January 21, 2191 at 08:00 AM

4 medication(s) prescribed:

• Heparin Flush (10 units/ml) 2 mL IV 1.0x/day

• HydrOXYzine 25 mg PO/NG

• Sodium Chloride 0.9% Flush 10 mL IV 1.0x/day

• Terbinafine 1% Cream 1 Appl TP 1.0x/day

Prescriptions Ordered: January 21, 2191 at 09:00 AM

1 medication(s) prescribed:

• Insulin 0 UNIT SC

Microbiology Tests Performed: January 21, 2191 at 08:48 PM

Total of **1** microbiology result(s):

Blood Culture, Routine: BLOOD CULTURE:

• NO GROWTH.

Prescriptions Ordered: January 21, 2191 at 11:00 PM

1 medication(s) prescribed:

• Sodium Chloride 0.9% Flush 3 mL IV 3.0x/day

Procedure(s) Performed: January 22, 2191 at 12:00 AM

1 procedure(s) performed (Source: ICD):

• Drainage of Right Hip Joint, Percutaneous Approach, Diagnostic

Prescriptions Ordered: January 22, 2191 at 10:00 AM

1 medication(s) prescribed:

• Insulin 0 UNIT SC

Microbiology Tests Performed: January 22, 2191 at 10:01 AM

Total of **2** microbiology result(s):

GRAM STAIN: JOINT FLUID:

• 2+ (1-5 per 1000X FIELD): POLYMORPHONUCLEAR LEUKOCYTES. NO MICROORGANISMS SEEN.

FLUID CULTURE: JOINT FLUID:

• NO GROWTH.

Prescriptions Ordered: January 22, 2191 at 12:00 PM

3 medication(s) prescribed:

• Sodium Chloride 0.9% Flush 20 mL IV

• Lidocaine 1% (For PICC/Midline Insertions) 5 mL SC

• Heparin Flush (10 units/ml) 2 mL IV

Prescriptions Ordered: January 22, 2191 at 01:00 PM

1 medication(s) prescribed:

• ALPRAZolam 0.5 mg PO/NG 1.0x/day

Prescriptions Ordered: January 22, 2191 at 02:00 PM

2 medication(s) prescribed:

• Sodium Chloride 0.9% Flush 10 mL IV 1.0x/day

• Heparin Flush (10 units/ml) 2 mL IV 1.0x/day

Prescriptions Ordered: January 24, 2191 at 07:00 PM

1 medication(s) prescribed:

• LOPERamide 2 mg PO/NG

Prescriptions Ordered: January 25, 2191 at 10:00 AM

1 medication(s) prescribed:

• Bag 2 BAG IV 1.0x/day

Prescriptions Ordered: January 25, 2191 at 01:00 PM

2 medication(s) prescribed:

• Acetaminophen 1000 mg PO/NG 1.0x/day

• DiphenhydrAMINE 50 mg PO/NG 1.0x/day

Prescriptions Ordered: January 26, 2191 at 10:00 AM

1 medication(s) prescribed:

• LOPERamide 2 mg PO/NG 4.0x/day

Prescriptions Ordered: January 27, 2191 at 08:00 AM

1 medication(s) prescribed:

• amLODIPine 5 mg PO/NG 1.0x/day

Prescriptions Ordered: January 27, 2191 at 06:00 PM

1 medication(s) prescribed:

• Aluminum-Magnesium Hydrox.-Simethicone 15-30 mL PO/NG

Prescriptions Ordered: January 28, 2191 at 08:00 AM

2 medication(s) prescribed:

• Ferrous Sulfate 325 mg PO/NG 1.0x/day

• Pantoprazole 40 mg PO 1.0x/day

Prescriptions Ordered: January 29, 2191 at 04:00 PM

2 medication(s) prescribed:

• Acetaminophen 650 mg PO/NG 1.0x/day

• DiphenhydrAMINE 50 mg PO/NG 1.0x/day

Discharge Summary

Discharge Time: January 30, 2191 at 05:09 PM

The patient was discharged from **Vascular** with disposition to **Extended Care**.

Major Procedure(s) During Admission: ['Transesophageal Echocardiogram', 'Right Upper Extremity Midline Catheter Placement']

Known Allergies: Penicillins

Detailed Allergy List:

• Penicillins

Discharge Diagnoses:

Primary Diagnoses:

• Sepsis secondary to Streptococcus Viridans Bacteremia

• R septic hip

• Anemia

• Chronic kidney disease

Secondary Diagnoses:

• Diabetes mellitus

Complete Diagnosis List:

• Other streptococcal sepsis

• Severe sepsis without septic shock

• Acidosis

• Acute kidney failure, unspecified

• Chronic kidney disease, stage 3 (moderate)

• Type 2 diabetes mellitus with diabetic chronic kidney disease

• Tinea cruris

• Hypo-osmolality and hyponatremia

• Type 2 diabetes mellitus with foot ulcer

• Non-pressure chronic ulcer of other part of right foot with unspecified severity

• Type 2 diabetes mellitus with hyperglycemia

• Long term (current) use of insulin

• Hypertensive chronic kidney disease with stage 1 through stage 4 chronic kidney disease, or unspecified chronic kidney disease

• Personal history of nicotine dependence

• Type 2 diabetes mellitus with diabetic neuropathy, unspecified

• Hyperkalemia

• Other disorders of phosphorus metabolism

• Anemia, unspecified

• Febrile nonhemolytic transfusion reaction

• Diarrhea, unspecified

Hospital Course:

yo M w/ history of Type 2 DM complicated by nephropathy, neuropathy, CKDIII, and multiple hospitalizations presents to with persistent RLE pain, right foot ulcer, and malaise admitted with significant leukocytosis, tachycardia, AGMA, and acute on chronic renal failure concerning for sepsis secondary to bacteremia on . ACTIVE ISSUES: ========== # Sepsis secondary to Strep Viridans Bacteremia: Patient presented with increased lower extremity pain, leukocytosis to 24,000, tachycardia, malaise, and severe metabolic acidosis. He was treated with Vancomycin 1000mg IV q48hr, MetRONIDAZOLE 500mg IV q8hr, Ciprofloxacin

400mg IV q12hr from to . Blood cultures revealed Strep Viridans Bacteremia, broad spectrum antibiotics were discontinued and patient was started on CefTRIAXone 2 gram IV q24hr. Foot XR did not indicate osteomyelitis secondary to foot ulcer. No evidence of cellulitis on exam. TTE and TEE showed no valvular vegetations as source for bacteremia. CT abdomen/pelvis shows no clear source of infection. Hip XR shows no evidence of osteomyelitis. Right hip joint fluid analysis () demonstrates ~30,000 WBCs, RBCs, 94% PMNs, no bacteria on gram stain, no culture growth. Given that cultures can be negative after antibiotic treatment, hip is likely source of infection. Hip joint washout deferred for 4-week course of ceftriaxone . Symptoms have improved and on discharge WBC 11.9. # on CKD: Patient had severely increased creatinine of 5.7 from baseline of on admission. Etiology undetermined, but there is likely a prerenal component from poor perfusion in the setting of poor PO intake and sepsis. Prolonged poor perfusion could have resulted in ATN. Additionally, the patient was prescribed standing Naproxen 500 mg BID for the two weeks prior to admission, which may have caused AIN. Patient was given 2L normal saline for fluid resuscitation, home Labetalol 100 mg PO BID was restarted to restore renal perfusion, Naproxen was held during the hospital stay, and patient received sodium bicarb with improvements in creatinine. He was placed on a low K low phosphate diet. Kidney function returned to baseline on discharge. # Diarrhea: Patient began to experience several watery bowel movements daily while hospitalized. Patient had been on Senna/Colace PRN and Oxycodone for pain. He had also been on broad spectrum antibiotics for 3 days. C diff negative. CT showed right-sided inguinal and pelvic lymphadenopathy of unknown etiology. Last colonoscopy years ago, normal. Stool cultures for various infectious causes were unrevealing. Cause of diarrhea is likely broad spectrum antibiotics and ceftriaxone. Diarrhea resolved with Loperamide 2 mg PO TID. # Right hip pain: On admission, patient describes muscular pain around his right groin, as if he pulled a muscle, that did not improve substantially despite treatment with Oxycodone and Tylenol. The pain is worse when he moves the RLE, and consequently prevents him from ambulating. Hip XR demonstrates OA changes without evidence of osteomyelitis. CT abdomen/pelvis demonstrates inguinal hernia and lymphadenopathy. Joint hip aspiration demonstrates large WBC with PMN predominance, likely infectious. Less likely inflammatory given monoarticular joint pain and lack of associated disease in medical history. Treatment as per sepsis above. # Anemia: Patient has a history of anemia. Iron studies demonstrated low iron with elevated ferritin and low transferrin concerning for anemia of chronic disease or acute inflammatory suppression. CKD also contributing. Hgb dropped below 7.0 several times, blood transfusion given and prematurely discontinued due to reaction during first transfusion. During subsequent transfusions, patient was given Benadryl prior and experienced no complications. Current Hgb 7.6. # Tranfusion Reaction: Patient had a transfusion reaction related to blood transfusion for anemia on . He received most of his transfusion prior to the reaction. Tranfusion discontinued and patient was treated with Benadryl. Likely febrile nonhemolytic reaction, given symptoms of tremulousness, fever, tachycardia, and hypertension with lack of hematuria, back pain, and otherwise negative ROS to support hemolytic reaction. Hemolysis labs did not show evidence of hemolysis. Patient received Benadryl prior to subsequent transfusions and was asymptomatic. # Anion gap metabolic acidosis: Patient had an anion gap of 23 despite a normal lactate on admission with delta/delta 1.2 indicating pure anion gap metabolic acidosis. Cause is likely multifactorial given history of CKD, on admission, and diarrhea while hospitalized. He was treated with Sodium bicarb. Metabolic acidosis resolved by discharge. # Tachycardia: Initially presented with tachycardia, likely in setting of ongoing pain, sepsis, and antihypertensive home medication hold (amlodipine, Labetalol) for suspected pre-renal . S1Q3T3 pattern seen on EKG, however this pattern is consistent with prior EKGs. Old EKG's show patient is typically in sinus tachycardia. We restarted home Labetalol and amlodipine with resolution of his tachycardia. # Right leg pain/swelling: Patient complains of RLE pain on admission partially reminiscent of diabetic neuropathic foot pain, but also with new "muscular" groin pain. US w/ Doppler shows no evidence of DVT or arterial stenosis/hypoperfusion. Pain likely due to right foot ulcer or DM neuropathy. We treated him with Oxycodone mg q4hr prn and Tylenol mg po q8hr for pain with improvement in his pain from admission. He was resistant to a trial of Gabapentin. # Hyponatremia: Patient was

hyponatremic on admission. Etiology is likely multifactorial. Extra-renal causes are possible in the setting of poor PO intake. Urine lytes indicate inadequate reabsorption of sodium given his hyponatremic state possibly due to advanced renal failure and inability of the urine to appropriately re-uptake sodium. Pain causing SIADH may have contributed, though this is less likely given his improved pain control. Hyponatremia has resolved on discharge. # Type 2 DM c/b neuropathy and nephropathy: Patient was hyperglycemic on presentation. He was treated with Glargine and Humalog. Continued home aspirin and atorvastatin while hospitalized. # Hypertension: Home amlodipine and Labetalol were held on admission with concerns for secondary to hypoperfusion. Patient was tachycardic on presentation, and remained tachycardic despite treatment of sepsis. We restarted home Labetalol and amlodipine with resolution of his tachycardia.

Microbiology Findings:

{'Blood Culture': 'VIRIDANS STREPTOCOCCI. Sensitivities: MIC expressed in MCG/ML CLINDAMYCIN---------- R ERYTHROMYCIN---------- 2 R PENICILLIN G----------<=0.06 S VANCOMYCIN---------- 0.5 S', 'Urine Culture': '< 10,000 organisms/mL', 'C Difficile PCR Amplification': 'negative', 'Joint Fluid': 'Source: R hip. GRAM STAIN (Final): 2+ POLYMORPHONUCLEAR LEUKOCYTES. NO MICROORGANISMS SEEN. FLUID CULTURE (Preliminary): NO GROWTH.'}

Antibiotic Plan:

Continue ceftriaxone 2g q24

Discharge Status:

Activity Status	Ambulatory - requires assistance or aid (walker or cane).
Level of Consciousness	Alert and interactive.
Mental Status	Clear and coherent.
Code Status	Full

Discharge Instructions:

Dear Mr. , It was a pleasure taking care of you during your time at . Why was I here? - You had a whole body infection in your bloodstream (sepsis) - You had damage to your kidneys possibly caused by a few things, including the infection, the Naproxen, and not eating/drinking enough What happened while I was here? - We treated you with antibiotics for your infection - We searched for an infection source by examining your heart with ultrasound, your foot with an X-ray, your abdomen and pelvis with CT scan, and your hip with fluid aspiration. We think the hip was the source of your infection - We gave you fluid and medications to to help your kidneys. We also stopped your Naproxen and changed you to Tylenol to help your kidneys. Your kidneys are back to your normal baseline - We gave you blood transfusions since your blood had low red blood cells - We gave you oxycodone and Tylenol for your foot pain What should I do? - It is VERY IMPORTANT that you FINISH your course of antibiotics. If not, the infection could come back and put you in the hospital again - Stay well hydrated and control your blood sugars with an appropriate diet - Continue to take your medications as prescribed - Follow up with your outpatient providers wish you good health! Your Medicine Team

New Medications Started:

• Consider starting lisinopril and vitamin D by outpatient nephrologist for BP control and CKD optimization

Medications Stopped:

• Naproxen

*Time gap of **466 days, 23 hours, 16 minutes** since last event*

Emergency Department Visit #10

Admission Time: May 11, 2192 at 04:26 PM

This is the patient's **10th** Emergency Department visit. The patient arrived via **AMBULANCE**. The patient departed at **2192-05-12 09:31:00**, staying for a duration of **0 days 17 hours 5 minutes**.

Initial Assessment:

Chief Complaint: Coffee ground emesis, Nausea, vomiting

Blood Pressure	151.0/73.0 mmHg
Heart Rate	128.0 bpm
Respiratory Rate	20.0 breaths/min
Oxygen Saturation	99.0%
Temperature	100.1°F
Pain Score	0
Acuity Level	2.0

Laboratory Tests Performed: May 11, 2192 at 04:45 PM

Total of **46** tests performed, with **23** abnormal result(s).

Chemistry

Test	Value	Reference Range
Light Green Top Hold	N/A Blood	N/A
Alanine Aminotransferase (ALT)	12.0IU/L	0.0-40.0
Albumin	3.5g/dL	3.5-5.2
Alkaline Phosphatase	103.0IU/L	40.0-130.0
Anion Gap	**28.0mEq/L**	10.0-18.0
Asparate Aminotransferase (AST)	11.0IU/L	0.0-40.0
Bicarbonate	**11.0mEq/L**	22.0-32.0

Bilirubin, Total	0.3mg/dL	0.0-1.5
Calcium, Total	8.9mg/dL	8.4-10.3
Chloride	**87.0mEq/L**	96.0-108.0
Creatinine	**9.1mg/dL**	0.5-1.2
Estimated GFR (MDRD equation)	N/A Blood	N/A
Glucose	**310.0mg/dL**	70.0-100.0
H	1.0 Blood	N/A
I	0.0 Blood	N/A
Lipase	32.0IU/L	0.0-60.0
Magnesium	**1.4mg/dL**	1.6-2.6
Phosphate	**4.9mg/dL**	2.7-4.5
Potassium	4.2mEq/L	3.3-5.1
Sodium	**126.0mEq/L**	135.0-147.0
Urea Nitrogen	**82.0mg/dL**	6.0-20.0
L	4.0 Blood	N/A

Hematology

Test	Value	Reference Range
INR(PT)	**1.3**	0.9-1.1
PT	**13.7sec**	9.4-12.5
PTT	27.1sec	25.0-36.5
Absolute Lymphocyte Count	**0.46K/uL**	1.2-3.7
Basophils	0.2%	0.0-1.0
Eosinophils	**0.0%**	1.0-7.0
Hematocrit	**26.4%**	40.0-51.0
Hemoglobin	**8.9g/dL**	13.7-17.5
Lymphocytes	**1.7%**	19.0-53.0
MCH	28.3pg	26.0-32.0

MCHC	33.7g/dL	32.0-37.0
MCV	84.0fL	82.0-98.0
Monocytes	6.4%	5.0-13.0
Neutrophils	**89.8%**	34.0-71.0
Platelet Count	360.0K/uL	150.0-400.0
RDW	14.0%	10.5-15.5
Red Blood Cells	**3.14m/uL**	4.6-6.1
White Blood Cells	**26.4K/uL**	4.0-10.0
Absolute Basophil Count	0.04K/uL	0.01-0.08
Absolute Eosinophil Count	**0.01K/uL**	0.04-0.54
Absolute Monocyte Count	**1.68K/uL**	0.2-0.8
Absolute Neutrophil Count	**23.71K/uL**	1.6-6.1
Immature Granulocytes	**1.9%**	0.0-0.6
RDW-SD	42.8fL	35.1-46.3

Laboratory Tests Performed: May 11, 2192 at 04:50 PM

Total of **2** tests performed, with **0** abnormal result(s).

Blood Gas

Test	Value	Reference Range
Lactate	1.1mmol/L	0.5-2.0
Potassium, Whole Blood	4.0mEq/L	3.3-5.1

Previous Medications on Record: May 11, 2192 at 05:34 PM

17 medication(s) documented:

• aspirin - Salicylate Analgesics

• aspirin - Platelet Aggregation Inhibitors - Salicylates

• atorvastatin - Antihyperlipidemic - HMG CoA Reductase Inhibitors (statins)

• blood-glucose meter [FreeStyle Freedom Lite] - Medical Supplies and DME - Glucose Monitoring Test Supplies

• blood-glucose meter [FreeStyle Freedom Lite] - Medical Supply, FDB Superset

• blood sugar diagnostic [FreeStyle Lite Strips] - Medical Supplies and DME - Blood Glucose Tests

• blood sugar diagnostic [FreeStyle Lite Strips] - Medical Supply, FDB Superset

• calcium carbonate - Minerals and Electrolytes - Calcium Replacement

• cholecalciferol (vitamin D3) - Vitamins - D Derivatives

• epoetin alfa [Epogen] - Erythropoietins

• ferrous sulfate [iron] - Minerals and Electrolytes - Iron

• furosemide - Diuretic - Loop

• insulin glargine [Lantus Solostar] - Insulin Analogs - Long Acting

• lisinopril - ACE Inhibitors

• pen needle, diabetic [BD Insulin Pen Needle UF Mini] - Medical Supplies and DME - Insulin Needles-Syringes and Admin Supplies

• pen needle, diabetic [BD Insulin Pen Needle UF Mini] - Medical Supply, FDB Superset

• sodium bicarbonate - Antacid - Bicarbonate

Medications Administered: May 11, 2192 at 07:34 PM

Total of **5** medication(s) administered:

• Magnesium Sulf 2gm/50mL 50mL BAG

• Ondansetron 4mg/2mL 2mL VIAL

• Ondansetron 4mg/2mL 2mL VIAL

• Atorvastatin 20mg TAB

• CefePIME (Fro 2g/100mL 100mL Bag

Laboratory Tests Performed: May 11, 2192 at 08:30 PM

Total of **10** tests performed, with **1** abnormal result(s).

Chemistry

Test	Value	Reference Range
Length of Urine Collection	N/A Urine	N/A
Urine tube, held	N/A Urine	N/A
Creatinine, Urine	64.0mg/dL Urine	N/A
Length of Urine Collection	N/A Urine	N/A
Osmolality, Urine	335.0mOsm/kg Urine	N/A

Protein/Creatinine Ratio	**5.2mg/mg**	0.0-0.2
Sodium, Urine	48.0mEq/L Urine	N/A
Total Protein, Urine	335.0mg/dL Urine	N/A
Length of Urine Collection	N/A Urine	N/A
Uhold	N/A Urine	N/A

Laboratory Tests Performed: May 11, 2192 at 08:42 PM

Total of **18** tests performed, with **2** abnormal result(s).

Hematology

Test	Value	Reference Range
Bacteria	**N/A Urine**	N/A
Bilirubin	N/A Urine	N/A
Blood	**N/A Urine**	N/A
Epithelial Cells	0.0#/hpf Urine	N/A
Glucose	**300.0mg/dL**	0.0-0.0
Ketone	N/A	0.0-0.0
Leukocytes	N/A Urine	N/A
Nitrite	N/A Urine	N/A
pH	6.0units	5.0-8.0
Protein	**300.0mg/dL**	0.0-0.0
RBC	1.0#/hpf	0.0-2.0
Specific Gravity	1.012	1.001-1.035
Urine Appearance	N/A Urine	N/A
Urine Color	N/A Urine	N/A
Urine Mucous	**N/A Urine**	N/A
Urobilinogen	N/A	0.2-1.0
WBC	1.0#/hpf	0.0-5.0
Yeast	N/A Urine	N/A

Laboratory Tests Performed: May 11, 2192 at 09:15 PM

Total of **12** tests performed, with **7** abnormal result(s).

Chemistry

Test	Value	Reference Range
Anion Gap	**27.0mEq/L**	10.0-18.0
Bicarbonate	**10.0mEq/L**	22.0-32.0
Chloride	**90.0mEq/L**	96.0-108.0
Creatinine	**8.8mg/dL**	0.5-1.2
Glucose	**229.0mg/dL**	70.0-100.0
H	150.0 Blood	N/A
I	0.0 Blood	N/A
Potassium	4.7mEq/L	3.3-5.1
Sodium	**127.0mEq/L**	135.0-147.0
Urea Nitrogen	**83.0mg/dL**	6.0-20.0
L	4.0 Blood	N/A
Green Top Hold (plasma)	N/A Blood	N/A

Laboratory Tests Performed: May 11, 2192 at 09:20 PM

Total of **7** tests performed, with **4** abnormal result(s).

Blood Gas

Test	Value	Reference Range
Base Excess	-11.0mEq/L Blood	N/A
Calculated Total CO2	**13.0mEq/L**	21.0-30.0
Intubated	N/A Blood	N/A
pCO2	**25.0mm Hg**	35.0-45.0
pH	**7.31units**	7.35-7.45
pO2	**43.0mm Hg**	85.0-105.0

Specimen Type	N/A Blood	N/A

Procedure(s) Performed: May 12, 2192 at 12:00 AM

3 procedure(s) performed (Source: ICD):

• Detachment at Right 2nd Toe, Complete, Open Approach

• Excision of Right Foot Subcutaneous Tissue and Fascia, Open Approach

• Excision of Right Metatarsal, Open Approach

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*Time gap of **7 hours, 42 minutes** since last event*

Hospital Admission #9

Admission Time: May 12, 2192 at 07:42 AM

This is the patient's **9th** hospital admission. The patient was admitted from **PHYSICIAN REFERRAL** as an **EW EMER.** admission. The admitting provider was **P61V4U**. Insurance coverage: **Private**. Chief complaint: **Nausea, vomiting**. The primary service provided was **MEDICINE**.

Race: **WHITE**, Marital Status: **SINGLE**, Language: **English**

APR DRG Code:

Code: 710

Description: INFECTIOUS AND PARASITIC DISEASES INCLUDING HIV WITH O.R. PROCEDURE

Severity: 3.0

Mortality Risk: 3.0

HCFA DRG Code:

Code: 853

Description: INFECTIOUS & PARASITIC DISEASES W O.R. PROCEDURE W MCC

Patient Past History:

Past Medical History: ESRD stage 5, DM, HTN, Chronic anemia, History of Septic R Hip, R foot ulcer s/p metatarsal resection (), s/p metatarsal removal ()

Family History: Mother with colon cancer and hyperlipidemia. Father with type 2 DM.

Social History: N/A

History of Present Illness:

year old man with a hx of poorly controlled diabetes, hypertension, stage 5 CKD, neuropathy, chronic R foot wound, anemia (iron deficiency and AoCKD), in the process of transplant evaluation and tentative planning for dialysis, who is presenting with nausea, coughing, and emesis. 2 days ago he started having vomiting. This progressed to two episodes of "brown" emesis, which prompted calling EMS on . He had no episodes of hematemesis in the ED.

Admission Vital Signs:

General Appearance	Alert
Blood Pressure	142/64 mmHg
Heart Rate	124 bpm
Respiratory Rate	16 breaths/min
Temperature	100.1°F
Oxygen Saturation	99% RA

Admission Laboratory Results:

• WBC: 26.4

• Glucose: 310

• Lactate: 1.1

Admission Medications:

Total of **10** medications on admission:

• Calcitriol 0.25 mcg PO DAILY

• Lisinopril 5 mg PO DAILY

• Sodium Bicarbonate 1300 mg PO BID

• Calcium Carbonate 500 mg PO TID

• Epoetin units SC EVERY 14 DAYS

• Furosemide 20 mg PO DAILY

• Glargine 20 Units Bedtime

• Atorvastatin 20 mg PO QPM

• Ferrous Sulfate 325 mg PO DAILY

• Vitamin D UNIT PO DAILY

ICU Admission - Surgical Intensive Care Unit (SICU)

ICU Admission Time: May 12, 2192 at 09:31 AM

The patient was admitted to the **Intensive Care Unit (Surgical Intensive Care Unit (SICU))**. The patient remained in ICU until **2192-05-13 00:55:45**, for a total ICU stay of **0 days 15 hours 24 minutes** (Length of Stay: **0.6421875 days**). During this critical care period, intensive monitoring and interventions were performed including procedures, laboratory tests, and microbiology events.

Prescriptions Ordered: May 12, 2192 at 11:00 AM

16 medication(s) prescribed:

- Vitamin D 2000 UNIT PO/NG 1.0x/day
- Docusate Sodium 100 mg PO 2.0x/day
- Dextrose 50% 12.5 gm IV
- Senna 8.6 mg PO
- Heparin 5000 UNIT SC 2.0x/day
- 0.9% Sodium Chloride 100 mL IV
- Calcitriol 0.25 mcg PO 1.0x/day
- Bag 1 BAG IV
- Sodium Bicarbonate 1300 mg PO/NG 2.0x/day
- Calcium Carbonate 500 mg PO/NG 3.0x/day
- D5 1/2NS 1000 mL IV
- Atorvastatin 20 mg PO/NG 1.0x/day
- Bisacodyl 10 mg PR
- 0.9% Sodium Chloride 100 mL IV DRIP
- Glucose Gel 15 g PO
- Glucagon 1 mg IM

Procedure(s) Performed: May 12, 2192 at 11:30 AM

1 procedure(s) performed (Source: ICU_EVENTS):

- Significant Events (Task) - OR Sent with value 1.0

Microbiology Tests Performed: May 12, 2192 at 12:04 PM

Total of **24** microbiology result(s):

TISSUE: TISSUE:

- STAPH AUREUS COAG + | ERYTHROMYCIN=8.0(R)
- STAPH AUREUS COAG +
- STAPH AUREUS COAG + | TRIMETHOPRIM/SULFA=0.5(S)
- STAPH AUREUS COAG + | TETRACYCLINE=1.0(S)
- STAPH AUREUS COAG + | GENTAMICIN=0.5(S)
- STAPH AUREUS COAG + | OXACILLIN=0.25(S)
- STAPH AUREUS COAG + | LEVOFLOXACIN=0.25(S)
- PSEUDOMONAS AERUGINOSA | GENTAMICIN=1.0(S)
- PSEUDOMONAS AERUGINOSA | TOBRAMYCIN=1.0(S)
- PSEUDOMONAS AERUGINOSA | CEFTAZIDIME=1.0(S)
- PSEUDOMONAS AERUGINOSA | CIPROFLOXACIN=0.25(S)
- PSEUDOMONAS AERUGINOSA | PIPERACILLIN/TAZO=4.0(S)
- PSEUDOMONAS AERUGINOSA | CEFEPIME=1.0(S)
- PSEUDOMONAS AERUGINOSA | MEROPENEM=0.25(S)
- CORYNEBACTERIUM SPECIES (DIPHTHEROIDS)
- STREPTOCOCCUS ANGINOSUS | PENICILLIN G=0.12(S)
- STREPTOCOCCUS ANGINOSUS | ERYTHROMYCIN=8.0(R)
- STREPTOCOCCUS ANGINOSUS
- STREPTOCOCCUS ANGINOSUS | VANCOMYCIN=1.0(S)
- STREPTOCOCCUS ANGINOSUS | CEFTRIAXONE=0.5(S)
- MIXED BACTERIAL FLORA

ANAEROBIC CULTURE: TISSUE:

- BACTEROIDES FRAGILIS GROUP
- MIXED BACTERIAL FLORA

GRAM STAIN: TISSUE:

- [redacted]

Procedure(s) Performed: May 12, 2192 at 12:51 PM

1 procedure(s) performed (Source: ICU_EVENTS):

- Significant Events (Task) - OR Received with value 1.0

Prescriptions Ordered: May 12, 2192 at 02:00 PM

1 medication(s) prescribed:

• Acetaminophen 650 mg PO/NG

Prescriptions Ordered: May 12, 2192 at 04:00 PM

3 medication(s) prescribed:

• Iso-Osmotic Dextrose 50 mL IV 3.0x/day

• Insulin 0 UNIT SC

• Insulin 10 UNIT SC 1.0x/day

Laboratory Tests Performed: May 12, 2192 at 04:25 PM

Total of **24** tests performed, with **11** abnormal result(s).

Hematology

Test	Value	Reference Range
Hematocrit	**24.4%**	40.0-51.0
Hemoglobin	**8.1g/dL**	13.7-17.5
MCH	28.7pg	26.0-32.0
MCHC	33.2g/dL	32.0-37.0
MCV	87.0fL	82.0-98.0
Platelet Count	323.0K/uL	150.0-400.0
RDW	13.8%	10.5-15.5
Red Blood Cells	**2.82m/uL**	4.6-6.1
White Blood Cells	**22.5K/uL**	4.0-10.0
RDW-SD	43.8fL	35.1-46.3

Chemistry

Test	Value	Reference Range
Anion Gap	**24.0mEq/L**	10.0-18.0
Bicarbonate	**9.0mEq/L**	22.0-32.0

Calcium, Total	8.5mg/dL	8.4-10.3
Chloride	99.0mEq/L	96.0-108.0
Creatinine	**8.1mg/dL**	0.5-1.2
Glucose	**152.0mg/dL**	70.0-100.0
H	14.0 Blood	N/A
I	0.0 Blood	N/A
Magnesium	1.6mg/dL	1.6-2.6
Phosphate	**5.7mg/dL**	2.7-4.5
Potassium	4.4mEq/L	3.3-5.1
Sodium	**132.0mEq/L**	135.0-147.0
Urea Nitrogen	**76.0mg/dL**	6.0-20.0
L	7.0 Blood	N/A

Laboratory Tests Performed: May 12, 2192 at 04:33 PM

Total of **8** tests performed, with **5** abnormal result(s).

Blood Gas

Test	Value	Reference Range
Base Excess	-14.0mEq/L Blood	N/A
Calculated Total CO2	**11.0mEq/L**	21.0-30.0
Free Calcium	**1.1mmol/L**	1.12-1.32
Lactate	0.9mmol/L	0.5-2.0
pCO2	**22.0mm Hg**	35.0-45.0
pH	**7.28units**	7.35-7.45
pO2	**154.0mm Hg**	85.0-105.0
Specimen Type	N/A Blood	N/A

Procedure(s) Performed: May 12, 2192 at 10:00 PM

1 procedure(s) performed (Source: ICU_EVENTS):

• Procedures (Task) - EKG with value 1.0

Prescriptions Ordered: May 12, 2192 at 11:00 PM

2 medication(s) prescribed:

• OxyCODONE (Immediate Release) 2.5 mg PO/NG

• Acetaminophen 650 mg PO/NG 4.0x/day

Unit Admission - Medicine

Admission Time: May 13, 2192 at 12:55 AM

The patient was admitted to **Medicine** ward. The patient stayed in this unit until **2192-05-27 18:51:47**, for a total duration of **14 days 17 hours 56 minutes**. During this time, various procedures, laboratory tests, and microbiology events were performed.

Prescriptions Ordered: May 13, 2192 at 02:00 AM

4 medication(s) prescribed:

• 5% Dextrose 1000 mL IV 1.0x/day

• Sodium Chloride 0.9% Flush 3-10 mL IV 3.0x/day

• Sodium Bicarbonate 1950 mg PO/NG 3.0x/day

• OxyCODONE (Immediate Release) 2.5-5 mg PO/NG

Prescriptions Ordered: May 13, 2192 at 08:00 AM

2 medication(s) prescribed:

• Polyethylene Glycol 17 g PO/NG 1.0x/day

• Vial 1 VIAL IV 1.0x/day

Microbiology Tests Performed: May 13, 2192 at 01:35 PM

Total of **1** microbiology result(s):

Blood Culture, Routine: BLOOD CULTURE:

• NO GROWTH.

Microbiology Tests Performed: May 13, 2192 at 01:45 PM

Total of **1** microbiology result(s):

Blood Culture, Routine: BLOOD CULTURE:

• NO GROWTH.

Prescriptions Ordered: May 13, 2192 at 10:00 PM

1 medication(s) prescribed:

• Insulin 20 UNIT SC 1.0x/day

Prescriptions Ordered: May 14, 2192 at 11:00 AM

1 medication(s) prescribed:

• Bag 1 BAG IV 1.0x/day

Prescriptions Ordered: May 14, 2192 at 01:00 PM

1 medication(s) prescribed:

• DiphenhydrAMINE 25 mg PO 1.0x/day

Prescriptions Ordered: May 14, 2192 at 02:00 PM

1 medication(s) prescribed:

• Calcium Carbonate 1000 mg PO/NG 3.0x/day

Prescriptions Ordered: May 14, 2192 at 04:00 PM

1 medication(s) prescribed:

• Tuberculin Protein 0.1 mL ID 1.0x/day

Prescriptions Ordered: May 15, 2192 at 02:00 PM

1 medication(s) prescribed:

• Iso-Osmotic Dextrose 150 mL IV 1.0x/day

Procedure(s) Performed: May 16, 2192 at 12:00 AM

3 procedure(s) performed (Source: ICD):

• Performance of Urinary Filtration, Intermittent, Less than 6 Hours Per Day

• Insertion of Tunneled Vascular Access Device into Chest Subcutaneous Tissue and Fascia, Percutaneous Approach

• Insertion of Infusion Device into Right Atrium, Percutaneous Approach

Prescriptions Ordered: May 16, 2192 at 09:00 AM

1 medication(s) prescribed:

• Ondansetron 4 mg PO/NG 1.0x/day

Prescriptions Ordered: May 16, 2192 at 11:00 AM

6 medication(s) prescribed:

• Heparin Dwell (1000 Units/mL) 1000-4000 UNIT DWELL

• Iso-Osmotic Dextrose 200 mL IV 1.0x/day

• Acetaminophen 1000 mg PO/NG 4.0x/day

• Neomycin-Polymyxin-Bacitracin 1 Appl TP 1.0x/day

• Sodium Chloride 0.9% 1000 mL IV

• Heparin Dwell (1000 Units/mL) 1000-4000 UNIT DWELL

Prescriptions Ordered: May 17, 2192 at 01:00 PM

2 medication(s) prescribed:

• amLODIPine 5 mg PO/NG 1.0x/day

• Insulin 0 UNIT SC

Prescriptions Ordered: May 17, 2192 at 02:00 PM

1 medication(s) prescribed:

• Bag 1 BAG IV

Prescriptions Ordered: May 17, 2192 at 03:00 PM

1 medication(s) prescribed:

• Tuberculin Protein 0.1 mL ID 1.0x/day

Procedure(s) Performed: May 18, 2192 at 12:00 AM

1 procedure(s) performed (Source: ICD):

• Change Other Device in Trunk Subcutaneous Tissue and Fascia, External Approach

Prescriptions Ordered: May 18, 2192 at 08:00 AM

7 medication(s) prescribed:

• Heparin Dwell (1000 Units/mL) 1000-4000 UNIT DWELL

• Syringe (SW) 1 mL IV 1.0x/day

- Heparin Dwell (1000 Units/mL) 1000-4000 UNIT DWELL
- Neomycin-Polymyxin-Bacitracin 1 Appl TP 1.0x/day
- Syringe (SW) 1 mL IV 1.0x/day
- Iso-Osmotic Dextrose 100 mL IV 1.0x/day
- Sodium Chloride 0.9% 1000 mL IV

Prescriptions Ordered: May 18, 2192 at 09:00 AM

1 medication(s) prescribed:

- Insulin 0 UNIT SC

Prescriptions Ordered: May 18, 2192 at 02:00 PM

1 medication(s) prescribed:

- Calcium Carbonate 1500 mg PO/NG 3.0x/day

Prescriptions Ordered: May 18, 2192 at 03:00 PM

3 medication(s) prescribed:

- amLODIPine 5 mg PO/NG 1.0x/day
- Acetaminophen 1000 mg PO/NG
- MetroNIDAZOLE 500 mg PO/NG 3.0x/day

Prescriptions Ordered: May 18, 2192 at 05:00 PM

1 medication(s) prescribed:

- Hepatitis B Vaccine 1 mL IM 1.0x/day

Prescriptions Ordered: May 18, 2192 at 10:00 PM

1 medication(s) prescribed:

- Insulin 10 UNIT SC 1.0x/day

Procedure(s) Performed: May 19, 2192 at 12:00 AM

1 procedure(s) performed (Source: ICD):

- Excision of Right Metatarsal, Open Approach

Prescriptions Ordered: May 19, 2192 at 09:00 AM

1 medication(s) prescribed:

• Bag 1 BAG IV

Microbiology Tests Performed: May 19, 2192 at 11:25 AM

Total of **13** microbiology result(s):

TISSUE: TISSUE:

Organism: *PSEUDOMONAS AERUGINOSA*

Antibiotic	Sensitivity
GENTAMICIN	1.0(S)
TOBRAMYCIN	1.0(S)
CEFTAZIDIME	32.0(R)
CIPROFLOXACIN	0.25(S)
CEFEPIME	4.0(S)
MEROPENEM	0.25(S)

ANAEROBIC CULTURE: TISSUE:

• BACTEROIDES FRAGILIS GROUP

GRAM STAIN: TISSUE:

• 1+ (<1 per 1000X FIELD): POLYMORPHONUCLEAR LEUKOCYTES. NO MICROORGANISMS SEEN.

ACID FAST SMEAR: TISSUE:

• NO ACID FAST BACILLI SEEN ON DIRECT SMEAR.

ACID FAST CULTURE: TISSUE:

• NO MYCOBACTERIA ISOLATED.

FUNGAL CULTURE: TISSUE:

• NO FUNGUS ISOLATED.

Prescriptions Ordered: May 19, 2192 at 12:00 PM

1 medication(s) prescribed:

• Heparin Flush (1000 units/mL) 4000-11000 UNIT DWELL

Prescriptions Ordered: May 19, 2192 at 02:00 PM

5 medication(s) prescribed:

• Neomycin-Polymyxin-Bacitracin 1 Appl TP 1.0x/day

• Sodium Chloride 0.9% 1000 mL IV

• Iso-Osmotic Dextrose 200 mL IV 1.0x/day

• Heparin Dwell (1000 Units/mL) 1000-4000 UNIT DWELL

• Heparin Dwell (1000 Units/mL) 1000-4000 UNIT DWELL

Prescriptions Ordered: May 19, 2192 at 06:00 PM

1 medication(s) prescribed:

• OxyCODONE (Immediate Release) 5 mg PO/NG 1.0x/day

Prescriptions Ordered: May 19, 2192 at 10:00 PM

1 medication(s) prescribed:

• Insulin 5 UNIT SC 1.0x/day

Prescriptions Ordered: May 20, 2192 at 08:00 AM

1 medication(s) prescribed:

• Sodium Chloride 0.9% Flush 10 mL IV 1.0x/day

Prescriptions Ordered: May 20, 2192 at 10:00 AM

1 medication(s) prescribed:

• Hepatitis B Vaccine 1 mL IM 1.0x/day

Prescriptions Ordered: May 20, 2192 at 05:00 PM

1 medication(s) prescribed:

• Insulin 0 UNIT SC

Microbiology Tests Performed: May 21, 2192 at 01:47 AM

Total of **1** microbiology result(s):

C. difficile PCR: STOOL:

• Negative for toxigenic C. difficile by the Cepheid nucleic acid amplification assay. (Reference Range-Negative).

Prescriptions Ordered: May 21, 2192 at 07:00 AM

5 medication(s) prescribed:

• Neomycin-Polymyxin-Bacitracin 1 Appl TP 1.0x/day

• Heparin Dwell (1000 Units/mL) 1000-4000 UNIT DWELL

• Iso-Osmotic Dextrose 200 mL IV 1.0x/day

• Sodium Chloride 0.9% 1000 mL IV

• Heparin Dwell (1000 Units/mL) 1000-4000 UNIT DWELL

Prescriptions Ordered: May 21, 2192 at 10:00 AM

1 medication(s) prescribed:

• 5% Dextrose 50 mL IV

Prescriptions Ordered: May 22, 2192 at 08:00 AM

1 medication(s) prescribed:

• amLODIPine 10 mg PO/NG 1.0x/day

Prescriptions Ordered: May 22, 2192 at 05:00 PM

1 medication(s) prescribed:

• TraMADol 25 mg PO

Prescriptions Ordered: May 23, 2192 at 07:00 AM

5 medication(s) prescribed:

• Iso-Osmotic Dextrose 100 mL IV 1.0x/day

• Heparin Dwell (1000 Units/mL) 1000-4000 UNIT DWELL

• Sodium Chloride 0.9% 1000 mL IV

• Heparin Dwell (1000 Units/mL) 1000-4000 UNIT DWELL

• Neomycin-Polymyxin-Bacitracin 1 Appl TP 1.0x/day

Prescriptions Ordered: May 23, 2192 at 10:00 AM

1 medication(s) prescribed:

• 5% Dextrose 100 mL IV

Prescriptions Ordered: May 24, 2192 at 11:00 AM

1 medication(s) prescribed:

• Insulin 0 UNIT SC

Prescriptions Ordered: May 25, 2192 at 10:00 AM

5 medication(s) prescribed:

• Iso-Osmotic Dextrose 100 mL IV 1.0x/day

• Sodium Chloride 0.9% 1000 mL IV

• Heparin Dwell (1000 Units/mL) 1000-4000 UNIT DWELL

• Neomycin-Polymyxin-Bacitracin 1 Appl TP 1.0x/day

• Heparin Dwell (1000 Units/mL) 1000-4000 UNIT DWELL

Prescriptions Ordered: May 26, 2192 at 08:00 AM

1 medication(s) prescribed:

• Lisinopril 2.5 mg PO/NG 1.0x/day

Prescriptions Ordered: May 26, 2192 at 01:00 PM

1 medication(s) prescribed:

• Ramelteon 8 mg PO/NG

Prescriptions Ordered: May 27, 2192 at 09:00 AM

5 medication(s) prescribed:

• Neomycin-Polymyxin-Bacitracin 1 Appl TP 1.0x/day

• Sodium Chloride 0.9% 1000 mL IV

• Heparin Dwell (1000 Units/mL) 1000-4000 UNIT DWELL

• Iso-Osmotic Dextrose 100 mL IV 1.0x/day

• Heparin Dwell (1000 Units/mL) 1000-4000 UNIT DWELL

Discharge Summary

Discharge Time: May 27, 2192 at 06:51 PM

The patient was discharged from **Medicine** with disposition to **Extended Care**.

Major Procedure(s) During Admission: OR with podiatry, R toe amputation

Known Allergies: Penicillins / metformin

Detailed Allergy List:

- **Penicillins / metformin**

Discharge Diagnoses:

Primary Diagnoses:

- ESRD with HD initiation
- necrotizing fasciitis with Osteomyelitis

Secondary Diagnoses:

- T2DM
- HTN
- Anemia
- HLD

Complete Diagnosis List:

- Sepsis, unspecified organism
- End stage renal disease
- Necrotizing fasciitis
- Acidosis
- Hypertensive chronic kidney disease with stage 5 chronic kidney disease or end stage renal disease
- Type 2 diabetes mellitus with diabetic peripheral angiopathy with gangrene
- Other acute osteomyelitis, right ankle and foot
- Hypo-osmolality and hyponatremia
- Chronic viral hepatitis B without delta-agent
- Other complication of vascular dialysis catheter, initial encounter
- Severe sepsis without septic shock
- Type 2 diabetes mellitus with diabetic chronic kidney disease
- Type 2 diabetes mellitus with other specified complication
- Other streptococcus as the cause of diseases classified elsewhere
- Other specified bacterial agents as the cause of diseases classified elsewhere
- Methicillin susceptible Staphylococcus aureus infection as the cause of diseases classified elsewhere
- Pseudomonas (aeruginosa) (mallei) (pseudomallei) as the cause of diseases classified elsewhere
- Bacteroides fragilis [B. fragilis] as the cause of diseases classified elsewhere
- Surgical operation with implant of artificial internal device as the cause of abnormal reaction of the patient, or of later complication, without mention of misadventure at the time of the procedure

- Patient room in hospital as the place of occurrence of the external cause
- Type 2 diabetes mellitus with diabetic neuropathy, unspecified
- Type 2 diabetes mellitus with foot ulcer
- Non-pressure chronic ulcer of other part of right foot with necrosis of bone
- Type 2 diabetes mellitus with hyperglycemia
- Iron deficiency anemia, unspecified
- Anemia in chronic kidney disease
- Hyperlipidemia, unspecified
- Benign neoplasm of meninges, unspecified
- Anxiety disorder, unspecified
- Other symbolic dysfunctions
- Long term (current) use of insulin
- Acquired absence of other right toe(s)
- Personal history of nicotine dependence

Hospital Course:

Summary w/pmh poorly controlled diabetes, hypertension, stage 5 CKD, neuropathy, chronic R foot wound who presented with emesis, anion gap metabolic acidodis, and leukocytosis, found to have necrotizing fasciitis of the R foot s/p R toe amputation on . Went back to OR for additional debridement and was found to have osteomyelitis. Was initiated on HD for worsening ESRD and will be treated with course of IV vancomycin and ceftaz, po flagyl. He was discharged to rehab. # R Foot Necrotizing Fasciitis # Diabetic Foot Infection H/o chronic foot infection I/s/o diabetes, but with gas on exam of foot by Podiatry. Podiatry took to OR on , amputation of toe done, with evidence of Nec Fasciitis in OR. Gram stain with polymicrobial: GPC, GPR, GNR. ABI was done and normal bilaterally. Went back to OR for additional debridement with close to complete closure. OPAT Antimicrobial Regimen and Projected Duration: Agent & Dose: 500mg vancomycin qHD, 2g-2g-3g Ceftazidime qHD, and metronidazole 500mg TID PO. Start Date: Projected End Date: ALL LAB RESULTS SHOULD BE SENT TO: ATTN: CLINIC - FAX: PLEASE CHECK WEEKLY: CBC with differential, BUN, Cr, Vancomycin trough, ESR, CRP # ESRD Presented with metabolic acidosis secondary to uremia/ESRD. Labs not consistent with DKA. Had been nearing dialysis as outpatient, with recent vein mapping and discussion of HD and Transplant. Underwent tunneled HD line placement and started HD on . Hepatitis viaral panel with borderline HBsAb. PPD placed and negative this admission. HBV vaccine given for intermediate titer. Will continue HD as outpatient. # Hyperglycemia # IDDM Presented with Glucose >300 and anion gap, though no ketones. Temporarily on insulin gtt in ED, discontinued in ICU. Was discharged on lantus 8u qpm with sliding scale at meals. # Normocytic anemia Close to baseline on presentation, likely related to underlying renal disease. Iron panel consistent with inflammation, B12 wnl. S/p transfusion 1u pRBC with appropriate response. Continue iron infusions with HD. # HTN Continue lisinopril 5mg daily and uptitrate as tolerated. # HLD Continue home atorvastatin.

Antibiotic Plan:

Antibiotic course with vancomycin, ceftazidime and flagyl as below. Please ensure patient is receiving antibiotics with HD sessions. OPAT Antimicrobial Regimen and Projected Duration: Agent & Dose: 500mg vancomycin qHD, 2g-2g-3g Ceftazidime qHD, and metronidazole 500mg TID PO. Start Date: Projected End Date:

Discharge Status:

Activity Status	Out of Bed with assistance to chair or wheelchair.
Level of Consciousness	Alert and interactive.
Mental Status	Clear and coherent.
Code Status	Full

Discharge Instructions:

Dear Mr. , You came to with a foot infection. We have treated you with antibiotics and started you on dialysis. It was a pleasure taking care of you, best of luck. Your medical team

New Medications Started:

- CefTAZidime 2 g IV 2X/WEEK
- CefTAZidime 3 g IV 1X/WEEK
- MetroNIDAZOLE 500 mg PO Q8H
- Vancomycin (MD to order daily dose IV HD PROTOCOL)

Medications Stopped:

- Sodium Bicarbonate 1300 mg PO BID
- Calcium Carbonate 500 mg PO TID
- Furosemide 20 mg PO DAILY
- Ferrous Sulfate 325 mg PO DAILY

*Time gap of **64 days, 16 hours, 8 minutes** since last event*

Emergency Department Visit #11

Admission Time: July 31, 2192 at 11:00 AM

This is the patient's **11th** Emergency Department visit. The patient arrived via **AMBULANCE**. The patient departed at **2192-07-31 17:10:00**, staying for a duration of **0 days 6 hours 10 minutes**.

Initial Assessment:

Chief Complaint: Abnormal labs, Bacteremia

Blood Pressure	138.0/84.0 mmHg
Heart Rate	103.0 bpm
Respiratory Rate	18.0 breaths/min
Oxygen Saturation	97.0%
Temperature	100.0°F
Pain Score	0
Acuity Level	2.0

Previous Medications on Record: July 31, 2192 at 12:47 PM

17 medication(s) documented:

• atorvastatin - Antihyperlipidemic - HMG CoA Reductase Inhibitors (statins)

• calcitriol - Vitamins - D Derivatives

• calcitriol - Hyperparathyroid Treatment Agents - Vitamin D Analog-Type

• cholecalciferol (vitamin D3) - Vitamins - D Derivatives

• epoetin alfa [Epogen] - Erythropoietins

• Florastor - Intestinal Flora Modifiers

• lisinopril - ACE Inhibitors

• Renvela - Phosphate Binders

• tramadol - Analgesic Opioid Agonists

• blood-glucose meter [FreeStyle Freedom Lite] - Medical Supplies and DME - Glucose Monitoring Test Supplies

• blood-glucose meter [FreeStyle Freedom Lite] - Medical Supply, FDB Superset

• blood sugar diagnostic [FreeStyle Lite Strips] - Medical Supplies and DME - Blood Glucose Tests

• blood sugar diagnostic [FreeStyle Lite Strips] - Medical Supply, FDB Superset

• insulin glargine [Lantus Solostar] - Insulin Analogs - Long Acting

• insulin lispro [Humalog] - Insulin Analogs - Rapid Acting

• pen needle, diabetic [BD Insulin Pen Needle UF Mini] - Medical Supplies and DME - Insulin Needles-Syringes and Admin Supplies

• pen needle, diabetic [BD Insulin Pen Needle UF Mini] - Medical Supply, FDB Superset

Medications Administered: July 31, 2192 at 01:56 PM

Total of **2** medication(s) administered:

• Vancomyci 1000mg/200mL 200mL BAG

• Vancomyci 1000mg/200mL 200mL BAG

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*Time gap of **2 hours, 9 minutes** since last event*

Hospital Admission #10

Admission Time: July 31, 2192 at 04:05 PM

This is the patient's **10th** hospital admission. The patient was admitted from **TRANSFER FROM SKILLED NURSING FACILITY** as an **OBSERVATION ADMIT** admission. The admitting provider was **P94BYL**. Insurance coverage: **Private**. Chief complaint: **Bacteremia**. The primary service provided was **MEDICINE**.

Race: **WHITE**, Marital Status: **SINGLE**, Language: **English**

APR DRG Code:

Code: 466

Description: MALFUNCTION, REACTION, COMPLICATION OF GENITOURINARY DEVICE OR PROCEDURE

Severity: 4.0

Mortality Risk: 3.0

HCFA DRG Code:

Code: 314

Description: OTHER CIRCULATORY SYSTEM DIAGNOSES W MCC

Patient Past History:

Past Medical History: ESRD stage 5, DM, HTN, Chronic anemia, History of Septic R Hip, R foot ulcer s/p metatarsal resection, s/p metatarsal removal

Family History: Mother with colon cancer and hyperlipidemia. Father with type 2 DM.

Social History: N/A

History of Present Illness:

w/ PMH DM, HTN, ESRD on dialysis MWF via left IJ tunnel line presents with fever and tachycardia. Patient stated that he did not feel well on , was found to have fever, did not go for HD. had peripheral blood cultures drawn in rehab at that time. H went to HD unit for catch up session yesterday (), was noted to be tachycardia with low grade temp(100.2), was able to complete HD treatment with 1.6L UF. Blood culture turned out positive today, patient was then sent to ED for further management. Per ED, all blood cultures grew MRSA; gram stain with GPCs and GPRs. Patient was seen in ED, feels ok, denied any chills, no cough or shortness of breath, no nausea/vomiting, stated that he still makes a good amount of urine, denied any diarrhea and endorses chronic constipation that is not bothersome. Per chart, he was discharged from the hospital () for right foot necrotizing fasciitis requiring amputation of toe on followed by additional debridement on , initiated HD during that admission on with left IJ tunnel line placement.

Admission Vital Signs:

General Appearance	AOx3
Blood Pressure	181/105 R Lying mmHg
Heart Rate	109 bpm
Respiratory Rate	18 breaths/min
Temperature	97.7 PO°F
Oxygen Saturation	98% RA

Admission Laboratory Results:

- WBC: 11.6
- Creatinine: 5.9
- Glucose: 132
- BUN: 42
- Sodium: 133
- Potassium: 5.8

Admission Medications:

Total of **14** medications on admission:

• Lisinopril 5 mg PO DAILY

• LORazepam 0.5 mg PO PRE DIALYSIS

• Epoetin UNIT IV Q14 DAYS

• Atorvastatin 20 mg PO QPM

• Furosemide 20 mg PO DAILY

• TraMADol 50 mg PO UNKNOWN

• PRN Pain - Moderate

• Polyethylene Glycol 17 g PO Frequency is Unknown

• Calcium Carbonate 500 mg PO TID

• Aspirin 81 mg PO DAILY

• Calcitriol 0.25 mcg PO DAILY

• Vitamin D UNIT PO DAILY

• Glargine 8 Units Bedtime

• Insulin SC Sliding Scale using HUM Insulin

Unit Admission - Medicine

Admission Time: July 31, 2192 at 05:10 PM

The patient was admitted to **Medicine** ward. The patient stayed in this unit until **2192-08-06 16:37:22**, for a total duration of **5 days 23 hours 27 minutes**. During this time, various procedures, laboratory tests, and microbiology events were performed.

Prescriptions Ordered: July 31, 2192 at 06:00 PM

4 medication(s) prescribed:

• Dextrose 50% 12.5 gm IV

• Glucagon 1 mg IM

• Insulin 0 UNIT SC

• Glucose Gel 15 g PO

Prescriptions Ordered: July 31, 2192 at 07:00 PM

3 medication(s) prescribed:

- Heparin 5000 UNIT SC 2.0x/day
- Sodium Chloride 0.9% Flush 3-10 mL IV 3.0x/day
- Influenza Vaccine Quadrivalent 0.5 mL IM

Prescriptions Ordered: July 31, 2192 at 09:00 PM

10 medication(s) prescribed:

- Glucagon 1 mg IM
- Insulin 8 UNIT SC 1.0x/day
- Calcitriol 0.25 mcg PO 1.0x/day
- Aspirin 81 mg PO/NG 1.0x/day
- Furosemide 20 mg PO/NG 1.0x/day
- Vitamin D 2000 UNIT PO/NG 1.0x/day
- Atorvastatin 20 mg PO/NG 1.0x/day
- Calcium Carbonate 500 mg PO/NG 3.0x/day
- Glucose Gel 15 g PO
- Dextrose 50% 12.5 gm IV

Prescriptions Ordered: July 31, 2192 at 10:00 PM

2 medication(s) prescribed:

- LORazepam 0.5 mg PO/NG
- Bag 1 BAG IV

Prescriptions Ordered: August 01, 2192 at 08:00 AM

5 medication(s) prescribed:

- Hydrocerin 1 Appl TP 2.0x/day
- Lisinopril 5 mg PO/NG 1.0x/day
- sevelamer CARBONATE 800 mg PO 3.0x/day
- Polyethylene Glycol 17 g PO/NG 1.0x/day
- Docusate Sodium 100 mg PO/NG 2.0x/day

Prescriptions Ordered: August 01, 2192 at 09:00 AM

1 medication(s) prescribed:

- Sodium Polystyrene Sulfonate 30 gm PO/NG 1.0x/day

Prescriptions Ordered: August 01, 2192 at 06:00 PM

1 medication(s) prescribed:

• Iso-Osmotic Dextrose 200 mL IV 1.0x/day

Microbiology Tests Performed: August 01, 2192 at 06:15 PM

Total of **1** microbiology result(s):

URINE CULTURE: URINE:

• < 10,000 CFU/mL.

Microbiology Tests Performed: August 01, 2192 at 06:38 PM

Total of **1** microbiology result(s):

Blood Culture, Routine: BLOOD CULTURE:

• NO GROWTH.

Microbiology Tests Performed: August 01, 2192 at 06:45 PM

Total of **1** microbiology result(s):

Blood Culture, Routine: BLOOD CULTURE:

• NO GROWTH.

Prescriptions Ordered: August 01, 2192 at 08:00 PM

2 medication(s) prescribed:

• Senna 8.6 mg PO/NG 2.0x/day

• Polyethylene Glycol 17 g PO/NG 2.0x/day

Microbiology Tests Performed: August 02, 2192 at 06:32 AM

Total of **1** microbiology result(s):

Blood Culture, Routine: BLOOD CULTURE:

• NO GROWTH.

Prescriptions Ordered: August 02, 2192 at 08:00 AM

1 medication(s) prescribed:

• Nephrocaps 1 CAP PO/NG 1.0x/day

Prescriptions Ordered: August 02, 2192 at 02:00 PM

2 medication(s) prescribed:

• Dextrose 50% 25 gm IV

• Furosemide 40 mg IV 1.0x/day

Prescriptions Ordered: August 02, 2192 at 03:00 PM

3 medication(s) prescribed:

• 0.9% Sodium Chloride 1 BAG IV 1.0x/day

• Insulin (Regular) for Hyperkalemia 10 UNIT IV 1.0x/day

• Iso-Osmotic Dextrose 200 mL IV

Prescriptions Ordered: August 02, 2192 at 06:00 PM

1 medication(s) prescribed:

• Furosemide 100 mg IV 1.0x/day

Prescriptions Ordered: August 03, 2192 at 12:00 AM

1 medication(s) prescribed:

• Furosemide 100 mg IV 1.0x/day

Prescriptions Ordered: August 03, 2192 at 01:00 AM

2 medication(s) prescribed:

• Insulin (Regular) for Hyperkalemia 10 UNIT IV 1.0x/day

• Dextrose 50% 25 gm IV 1.0x/day

Microbiology Tests Performed: August 03, 2192 at 06:40 AM

Total of **1** microbiology result(s):

Blood Culture, Routine: BLOOD CULTURE:

• NO GROWTH.

Prescriptions Ordered: August 03, 2192 at 10:00 AM

1 medication(s) prescribed:

• Calcitriol 0.25 mcg PO

Procedure(s) Performed: August 04, 2192 at 12:00 AM

2 procedure(s) performed (Source: ICD):

• Insertion of Tunneled Vascular Access Device into Chest Subcutaneous Tissue and Fascia, Percutaneous Approach

• Insertion of Infusion Device into Right Atrium, Percutaneous Approach

Microbiology Tests Performed: August 04, 2192 at 06:44 AM

Total of **1** microbiology result(s):

Blood Culture, Routine: BLOOD CULTURE:

• NO GROWTH.

Procedure(s) Performed: August 05, 2192 at 12:00 AM

1 procedure(s) performed (Source: ICD):

• Performance of Urinary Filtration, Intermittent, Less than 6 Hours Per Day

Prescriptions Ordered: August 05, 2192 at 07:00 AM

4 medication(s) prescribed:

• Heparin Dwell (1000 Units/mL) 1000-4000 UNIT DWELL

• Neomycin-Polymyxin-Bacitracin 1 Appl TP 1.0x/day

• Sodium Chloride 0.9% 1000 mL IV

• Heparin Dwell (1000 Units/mL) 1000-4000 UNIT DWELL

Prescriptions Ordered: August 05, 2192 at 08:00 AM

1 medication(s) prescribed:

• Iso-Osmotic Dextrose 100 mL IV 1.0x/day

Microbiology Tests Performed: August 05, 2192 at 08:10 AM

Total of **1** microbiology result(s):

Blood Culture, Routine: BLOOD CULTURE:

• NO GROWTH.

Prescriptions Ordered: August 05, 2192 at 09:00 AM

1 medication(s) prescribed:

• Acetaminophen 650 mg PO/NG 1.0x/day

Microbiology Tests Performed: August 06, 2192 at 06:25 AM

Total of **1** microbiology result(s):

Blood Culture, Routine: BLOOD CULTURE:

• NO GROWTH.

Prescriptions Ordered: August 06, 2192 at 12:00 PM

1 medication(s) prescribed:

• Lisinopril 5 mg PO/NG 1.0x/day

Prescriptions Ordered: August 06, 2192 at 03:00 PM

1 medication(s) prescribed:

• Iso-Osmotic Dextrose 200 mL IV

Discharge Summary

Discharge Time: August 06, 2192 at 04:37 PM

The patient was discharged from **Medicine** with disposition to **Extended Care**.

Major Procedure(s) During Admission: ['Removal of tunneled HD line', 'Placement of tunneled HD line', 'TEE']

Known Allergies: Penicillins / metformin

Detailed Allergy List:

• Penicillins / metformin

Discharge Diagnoses:

Primary Diagnoses:

• MRSA Bacteremia

Secondary Diagnoses:

• ESRD on HD

• Hyperkalemia

Complete Diagnosis List:

• Infection and inflammatory reaction due to other cardiac and vascular devices, implants and grafts, initial encounter

• Sepsis due to Methicillin resistant Staphylococcus aureus

• Acute and subacute infective endocarditis

• End stage renal disease

• Hypertensive chronic kidney disease with stage 5 chronic kidney disease or end stage renal disease

• Type 2 diabetes mellitus with diabetic chronic kidney disease

• Type 2 diabetes mellitus with diabetic nephropathy

• Hyperkalemia

• Fluid overload, unspecified

• Hyperlipidemia, unspecified

• Constipation, unspecified

• Anemia in chronic kidney disease

• Acquired keratosis [keratoderma] palmaris et plantaris

• Dependence on renal dialysis

• Acquired absence of other right toe(s)

• Long term (current) use of insulin

• Personal history of nicotine dependence

Hospital Course:

is a with DMII c/b ESRD on HD MWF p/w 2 days of fevers and positive blood cultures with MRSA, now s/p HD port removal and identification of aortic valve vegetation. # MRSA BACTEREMIA LIKELY DUE TO LINE INFECTION # MRSA ENDOCARDITIS OF AORTIC VALVE Patient presented from his facility on after episode of fevers, rigors, found to have high grade MRSA bacteremia (positive culture at rehab 1 of 4 cultures on with MRSA). Likely etiology is line sepsis from infected tunneled HD catheter and this was removed on . TTE was done which was suboptimal quality but did not show any vegetations. TEE on ndocarditis with mod vegetation on AV cusp. No paravalvular abscess seen. Repeat surveillance blood cultures were negative. New tunneled hemodialysis line was placed . Plan is to treat with 6 weeks of vancomycin dosed with HD through . Will follow-up in clinic. #ESRD on HD MWF #Hyperkalemia Patient was initiated on HD during his last hospitalization . Renal failure is secondary to diabetic nephropathy. Still makes urine. He was given a line holiday and missed dialysis session on and . CMP checked daily, hyperK+ and hypervolemia treated with insulin/dextrose and 100 mg Lasix and insulin/dextrose PRN. HD tunneled line was replaced on . He was kept on strict low K+ diet, strict mL fluid restriction and continued on home nephro caps, calcitriol, calcium carbonate and Vitamin D. Last dialysis session on . His home Lisinopril was held and then restarted on discharge. #s/p right toe amputation Healing well, no signs of infection. Podiatry curbsided and had a very low suspicion for

infection, but recommened x-ray to ensure no signs of osteo, though patient declined. CRP ~11. #DIABETES MELLITUS II Previously followed by . Most recent A1C 6.6%. Continued home glargine and Humalog SS. #HYPERTENSION SBP 140-150s. Continued on Lisinopril on discharge. #HLD: Continued on atorvastatin 20mg QHS and ASA 81mg daily #ANEMIA Hb 11.3, bl . Unclear why higher than usual, maybe some component of hemoconcentration in setting of fever and bacteremia. Iron studies from suggestive of anemia of chronic inflammation; B12 normal. Anemia likely due to ESRD. EPO 10,000units q14 days per renal recs.

Microbiology Findings:

URINE CULTURE: < 10,000 CFU/mL. BLOOD CULTURE #1: Preliminary: STAPH AUREUS COAG +. Final Sensitivities: Oxacillin RESISTANT. SENSITIVITIES (MIC in MCG/ML) for STAPH AUREUS COAG +: CLINDAMYCIN <=0.25 S, ERYTHROMYCIN 1 I, GENTAMICIN <=0.5 S, LEVOFLOXACIN 4 R, OXACILLIN =>4 R, RIFAMPIN <=0.5 S, TRIMETHOPRIM/SULFA <=0.5 S, VANCOMYCIN <=0.5 S. Anaerobic Bottle Gram Stain: GRAM POSITIVE COCCI IN PAIRS AND CLUSTERS. BLOOD CULTURE #2: Final: NO GROWTH. Other Blood Cultures: 7 pending.

Antibiotic Plan:

Continue vancomycin with hemodialysis sessions (end date for a total of 6 weeks.

Discharge Status:

Activity Status	Ambulatory - Independent.
Level of Consciousness	Alert and interactive.
Mental Status	Clear and coherent.
Code Status	N/A

Discharge Instructions:

Dear Mr. , You were admitted to with fevers. You had bacteria in your blood and were treated with intravenous antibiotics. We suspected the source of this infection to be from your hemodialysis line which was removed. We replaced your hemodialysis line. You underwent echocardiograms of your heart which found a bacteria in one of your heart valves. You will need antibiotics for several weeks with your hemodialysis sessions. It was a pleasure caring for you. Wishing you the best, Your Team

New Medications Started:

- Nephrocaps 1 CAP PO DAILY
- Vancomycin 1000 mg IV WITH HD

Medications Stopped:

- TraMADol 50 mg PO UNKNOWN

- PRN Pain - Moderate

*Time gap of **48 days, 6 hours, 5 minutes** since last event*

Emergency Department Visit #12

Admission Time: September 23, 2192 at 10:43 PM

This is the patient's **12th** Emergency Department visit. The patient arrived via **AMBULANCE**. The patient departed at **2192-09-24 09:38:00**, staying for a duration of **0 days 10 hours 55 minutes**.

Initial Assessment:

Chief Complaint: L Leg pain, s/p Fall, Comminuted left intertrochanteric fracture

Blood Pressure	106.0/64.0 mmHg
Heart Rate	98.0 bpm
Respiratory Rate	18.0 breaths/min
Oxygen Saturation	96.0%
Temperature	98.2°F
Pain Score	7
Acuity Level	2.0

Laboratory Tests Performed: September 23, 2192 at 11:19 PM

Total of **35** tests performed, with **12** abnormal result(s).

Chemistry

Test	Value	Reference Range
Green Top Hold (plasma)	N/A Blood	N/A
Blue Top Hold	N/A Blood	N/A
Anion Gap	13.0mEq/L	10.0-18.0
Bicarbonate	31.0mEq/L	22.0-32.0
Chloride	**93.0mEq/L**	96.0-108.0
Creatinine	**3.8mg/dL**	0.5-1.2
Estimated GFR (MDRD equation)	N/A Blood	N/A

Glucose	**148.0mg/dL**	70.0-100.0
H	4.0 Blood	N/A
I	0.0 Blood	N/A
Potassium	4.3mEq/L	3.5-5.4
Sodium	137.0mEq/L	135.0-147.0
Urea Nitrogen	**27.0mg/dL**	6.0-20.0
L	15.0 Blood	N/A

Hematology

Test	Value	Reference Range
Absolute Lymphocyte Count	**0.87K/uL**	1.2-3.7
Basophils	0.5%	0.0-1.0
Eosinophils	1.8%	1.0-7.0
Hematocrit	**30.2%**	40.0-51.0
Hemoglobin	**10.0g/dL**	13.7-17.5
Lymphocytes	**11.2%**	19.0-53.0
MCH	28.5pg	26.0-32.0
MCHC	33.1g/dL	32.0-37.0
MCV	86.0fL	82.0-98.0
Monocytes	**13.1%**	5.0-13.0
Neutrophils	**72.9%**	34.0-71.0
Platelet Count	261.0K/uL	150.0-400.0
RDW	14.6%	10.5-15.5
Red Blood Cells	**3.51m/uL**	4.6-6.1
White Blood Cells	7.8K/uL	4.0-10.0
Absolute Basophil Count	0.04K/uL	0.01-0.08
Absolute Eosinophil Count	0.14K/uL	0.04-0.54
Absolute Monocyte Count	**1.02K/uL**	0.2-0.8
Absolute Neutrophil Count	5.67K/uL	1.6-6.1

Immature Granulocytes	0.5%	0.0-0.6
RDW-SD	45.1fL	35.1-46.3

Procedure(s) Performed: September 24, 2192 at 12:00 AM

1 procedure(s) performed (Source: ICD):

• Insertion of Intramedullary Internal Fixation Device into Left Upper Femur, Percutaneous Approach

Medications Administered: September 24, 2192 at 12:19 AM

Total of **8** medication(s) administered:

• Acetaminophen 500mg TAB

• Fentanyl 100mcg/2mL 2mL AMP

• Fentanyl 100mcg/2mL 2mL AMP

• Fentanyl 100mcg/2mL 2mL AMP

• Fentanyl 100mcg/2mL 2mL AMP

• Furosemide 20mg TAB

• Fentanyl 100mcg/2mL 2mL AMP

• Fentanyl 100mcg/2mL 2mL AMP

Previous Medications on Record: September 24, 2192 at 12:46 AM

19 medication(s) documented:

• calcium gluconate - Minerals and Electrolytes - Calcium Replacement

• melatonin - Alternative Therapy - Pineal Hormone Agents

• melatonin - Hypnotics - Melatonin - Single Agents

• Renvela - Phosphate Binders

• aspirin - Salicylate Analgesics

• aspirin - Platelet Aggregation Inhibitors - Salicylates

• atorvastatin - Antihyperlipidemic - HMG CoA Reductase Inhibitors (statins)

• furosemide - Diuretic - Loop

• acetaminophen - Analgesic or Antipyretic Non-Opioid

• lisinopril - ACE Inhibitors

• Laxative (bisacodyl) - Laxative - Stimulant

• Fleet Enema - Laxative - Saline/Osmotic Mixtures

• Phillips Milk of Magnesia - Laxative - Saline and Osmotic

• tramadol - Analgesic Opioid Agonists

• Valtrex - Herpes Antiviral Agent - Purine Analogs

• Lyrica - Anticonvulsant - GABA Analogs

• Lyrica - Fibromyalgia Agents - GABA Analogs

• Humalog U-100 Insulin - Insulin Analogs - Rapid Acting

• Lantus U-100 Insulin - Insulin Analogs - Long Acting

Prescriptions Ordered: September 24, 2192 at 08:00 AM

1 medication(s) prescribed:

• Senna 8.6 mg PO 1.0x/day

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*Time gap of **1 hour, 19 minutes** since last event*

Hospital Admission #11

Admission Time: September 24, 2192 at 09:19 AM

This is the patient's **11th** hospital admission. The patient was admitted from **TRANSFER FROM SKILLED NURSING FACILITY** as an **URGENT** admission. The admitting provider was **P57U5Y**. Insurance coverage: **Private**. Chief complaint: **Comminuted left intertrochanteric fracture**. The primary service provided was **ORTHOPAEDICS**.

Race: **WHITE**, Marital Status: **SINGLE**, Language: **English**

APR DRG Code:

Code: 308

Description: HIP AND FEMUR FRACTURE REPAIR

Severity: 2.0

Mortality Risk: 2.0

HCFA DRG Code:

Code: 480

Description: HIP & FEMUR PROCEDURES EXCEPT MAJOR JOINT W MCC

Patient Past History:

Past Medical History: ESRD stage 5, DM, HTN, Chronic anemia, History of Septic R Hip, R foot ulcer s/p metatarsal resection (), s/p metatarsal removal ()

Family History: Mother with colon cancer and hyperlipidemia. Father with type 2 DM.

Social History: N/A

History of Present Illness:

HPI: PMHx ESRD (b/l Cr variable, >5 last admission) on HD MWF w/ access via tunneled Left IJ, DM w/ peripheral neuropathy, HTN, PVD s/p multiple Right toe amputations, and recent admission for MRSA bacteremia/AV endocarditis s/p 6 weeks IV Vanc (completed) presents from rehab s/p fall from standing with Left hip pain. He states he felt a "crunch" when he hit the ground. He endorses head strike, but reports no LOC. He has no new N/T/P in his BLE (baseline peripheral neuropathy). He states that for the past several years he has been ambulating with a walker given his Right foot status (multiple toe amps). His HCP is his brother : . He denies chest pain, SOB, or abdominal pain. He is alert and oriented to situation, place, and date. He states he last ate yesterday (dinner)>

Admission Laboratory Results:

• WBC: 7.8

• Glucose: 148

Admission Medications:

Total of **6** medications on admission:

• Lisinopril 5 mg PO DAILY

• Atorvastatin 20 mg PO QPM

• Aspirin 81 mg PO DAILY

• LORazepam 0.5 mg PO DAILY:PRN anxiety

• Glargine 8 Units Bedtime

• melatonin 5 mg oral QPM

Unit Admission - PACU

Admission Time: September 24, 2192 at 09:38 AM

The patient was admitted to **PACU** ward. The patient stayed in this unit until **2192-09-24 16:53:36**, for a total duration of **0 days 7 hours 15 minutes**. During this time, various procedures, laboratory tests, and microbiology events were performed.

Unit Admission - Surgery/Trauma

Admission Time: September 24, 2192 at 04:53 PM

The patient was admitted to **Surgery/Trauma** ward. The patient stayed in this unit until **2192-09-27 21:11:59**, for a total duration of **3 days 4 hours 18 minutes**. During this time, various procedures, laboratory tests, and microbiology events were performed.

Prescriptions Ordered: September 24, 2192 at 05:00 PM

6 medication(s) prescribed:

- Sodium Chloride 0.9% Flush 3-10 mL IV 3.0x/day
- LORazepam 0.5 mg PO/NG
- Pantoprazole 40 mg PO 1.0x/day
- Acetaminophen 650 mg PO 4.0x/day
- OxyCODONE (Immediate Release) 5-10 mg PO
- Sodium Chloride 0.9% 1000 mL IV

Prescriptions Ordered: September 24, 2192 at 06:00 PM

7 medication(s) prescribed:

- Ondansetron 4 mg IV
- Glucose Gel 15 g PO
- Bisacodyl 10 mg PR
- Dextrose 50% 12.5 gm IV
- Insulin 0 UNIT SC
- Glucagon 1 mg IM
- HYDROmorphone (Dilaudid) 1-2 mg PO/NG

Prescriptions Ordered: September 24, 2192 at 08:00 PM

3 medication(s) prescribed:

- Heparin 5000 UNIT SC 2.0x/day
- Calcium Carbonate 1250 mg PO 3.0x/day
- Docusate Sodium 100 mg PO 2.0x/day

Procedure(s) Performed: September 25, 2192 at 12:00 AM

1 procedure(s) performed (Source: ICD):

- Performance of Urinary Filtration, Intermittent, Less than 6 Hours Per Day

Prescriptions Ordered: September 25, 2192 at 08:00 AM

4 medication(s) prescribed:

• Lisinopril 5 mg PO/NG 1.0x/day

• Aspirin 81 mg PO/NG 1.0x/day

• Vitamin D 800 UNIT PO 1.0x/day

• Nephrocaps 1 CAP PO/NG 1.0x/day

Prescriptions Ordered: September 25, 2192 at 12:00 PM

6 medication(s) prescribed:

• Heparin Dwell (1000 Units/mL) 1000-4000 UNIT DWELL

• Heparin Dwell (1000 Units/mL) 1000-4000 UNIT DWELL

• Sodium Chloride 0.9% 1000 mL IV

• Neomycin-Polymyxin-Bacitracin 1 Appl TP 1.0x/day

• Epoetin Alfa 1000 UNIT IV 1.0x/day

• DiphenhydrAMINE 12.5 mg IV

Prescriptions Ordered: September 25, 2192 at 08:00 PM

1 medication(s) prescribed:

• Atorvastatin 20 mg PO/NG 1.0x/day

Unit Admission - Surgery/Trauma

Admission Time: September 27, 2192 at 09:11 PM

The patient was admitted to **Surgery/Trauma** ward. The patient stayed in this unit until **2192-09-28 19:58:14**, for a total duration of **0 days 22 hours 46 minutes**. During this time, various procedures, laboratory tests, and microbiology events were performed.

Prescriptions Ordered: September 28, 2192 at 10:00 AM

7 medication(s) prescribed:

• Epoetin Alfa 1000 UNIT IV 1.0x/day

• Heparin (Hemodialysis) 4000 UNIT IV 1.0x/day

• Sodium Chloride 0.9% 1000 mL IV

• DiphenhydrAMINE 12.5 mg IV

• Heparin Dwell (1000 Units/mL) 1000-4000 UNIT DWELL

• Neomycin-Polymyxin-Bacitracin 1 Appl TP 1.0x/day

• Heparin Dwell (1000 Units/mL) 1000-4000 UNIT DWELL

Prescriptions Ordered: September 28, 2192 at 03:00 PM

1 medication(s) prescribed:

• HYDROmorphone (Dilaudid) 0.25-0.5 mg IV

Discharge Summary

Discharge Time: September 28, 2192 at 07:58 PM

The patient was discharged from **Surgery/Trauma** with disposition to **Extended Care**.

Major Procedure(s) During Admission: Left short TFN , KRod)

Known Allergies: Penicillins, metformin, Naprosyn

Detailed Allergy List:

• Penicillins
• metformin
• Naprosyn

Discharge Diagnoses:

Primary Diagnoses:

• Comminuted Left intertrochanteric fracture

Complete Diagnosis List:

• Nondisplaced intertrochanteric fracture of left femur, initial encounter for closed fracture

• Fall on same level from slipping, tripping and stumbling without subsequent striking against object, initial encounter

• Bathroom in nursing home as the place of occurrence of the external cause

• Hypertensive chronic kidney disease with stage 5 chronic kidney disease or end stage renal disease

• End stage renal disease

• Type 2 diabetes mellitus with diabetic chronic kidney disease

• Type 2 diabetes mellitus with diabetic polyneuropathy

• Acute posthemorrhagic anemia

• Type 2 diabetes mellitus with diabetic peripheral angiopathy without gangrene

• Anemia in chronic kidney disease

• Pure hypercholesterolemia, unspecified

• Dependence on renal dialysis

• Long term (current) use of insulin

• Acquired absence of other right toe(s)

• Personal history of nicotine dependence

Hospital Course:

The patient presented to the emergency department and was evaluated by the orthopedic surgery team. The patient was found to have fracture of the left hip and was admitted to the orthopedic surgery service. The patient was taken to the operating room on for ORIF with short TFN, which the patient tolerated well. For full details of the procedure please see the separately dictated operative report. After the procedure the patient was taken from the OR to the PACU in stable condition and after satisfactory recovery from anesthesia was transferred to the floor. The patient was initially given IV fluids and IV pain medications, and progressed to a renal diet and oral medications. The patient was given antibiotics (IV Vanc given penicillin allergy) and anticoagulation (5000U BID given ESRD). The patient's home medications were continued throughout this hospitalization. Nephrology was consulted given need for HD. At the time of discharge the patient's pain was well controlled with oral medications, incisions were clean/dry/intact, and the patient was voiding/moving bowels spontaneously. The patient is weight bearing as tolerated in the left lower extremity, and will be discharged on 5000U BID for DVT prophylaxis. The patient will follow up with Dr. routine. A thorough discussion was had with the patient regarding the diagnosis and expected post-discharge course including reasons to call the office or return to the hospital, and all questions were answered. The patient was also given written instructions concerning precautionary instructions and the appropriate follow-up care. The patient expressed readiness for discharge.

Discharge Status:

Activity Status	Ambulatory - requires assistance or aid (walker or cane).
Level of Consciousness	Alert and interactive.
Mental Status	Clear and coherent.
Code Status	N/A

Discharge Instructions:

INSTRUCTIONS AFTER ORTHOPAEDIC SURGERY: - You were in the hospital for orthopedic surgery. It is normal to feel tired or "washed out" after surgery, and this feeling should improve over the first few days to week. - Resume your regular activities as tolerated, but please follow your weight bearing precautions strictly at all times. ACTIVITY AND WEIGHT BEARING: - weight bearing as tolerated in left lower extremity MEDICATIONS: - Please take all medications as prescribed by your physicians at discharge. - Continue all home medications unless specifically instructed to stop by your surgeon. - Do not drink alcohol, drive a motor vehicle, or operate machinery while taking narcotic pain relievers. - Narcotic pain relievers can cause constipation, so you should drink eight 8oz glasses of water daily and take a stool softener (colace) to prevent

this side effect. ANTICOAGULATION: - Please take SQH 5000U twice a day for 4 weeks WOUND CARE: - Your incision is covered with a dry dressing. Please change the dressing daily. If there is no drainage, you may leave your incision open to the air. If you are continuing to have drainage, you may place a dry dressing over the incision as needed. - You may shower. No baths or swimming for at least 4 weeks. - Any stitches or staples that need to be removed will be taken out at your 2-week follow up appointment. THIS PATIENT IS EXPECTED TO REQUIRE <30 DAYS OF REHAB Physical Therapy: weight bearing as tolerated in left lower extremity Treatments Frequency: Any staples or superficial sutures you have are to remain in place for at least 2 weeks postoperatively. Incision may be left open to air unless actively draining. If draining, you may apply a gauze dressing secured with paper tape. You may shower and allow water to run over the wound, but please refrain from bathing for at least 4 weeks postoperatively.

New Medications Started:

• SQH 5000U twice a day for 4 weeks

*Time gap of **7 days, 18 hours, 40 minutes** since last event*

Emergency Department Visit #13

Admission Time: October 06, 2192 at 02:39 PM

This is the patient's **13th** Emergency Department visit. The patient arrived via **AMBULANCE**. The patient departed at **2192-10-06 21:15:00**, staying for a duration of **0 days 6 hours 36 minutes**.

ED Disposition: HOME - Patient was discharged directly from Emergency Department without hospital admission.

Initial Assessment:

Chief Complaint: Diarrhea, Hypotension

Blood Pressure	134.0/70.0 mmHg
Heart Rate	96.0 bpm
Respiratory Rate	18.0 breaths/min
Oxygen Saturation	97.0%
Temperature	99.8°F
Pain Score	7
Acuity Level	3.0

Emergency Department Discharge Summary

Treatment: The patient received evaluation and treatment in the Emergency Department. Medical staff determined the patient was stable for discharge to **HOME**.

Follow-up Instructions: Patient advised to follow up with primary care physician or return to ED if symptoms worsen.

Microbiology Tests Performed: October 06, 2192 at 03:37 PM

Total of **1** microbiology result(s):

Blood Culture, Routine: BLOOD CULTURE:

• CANCELLED

Microbiology Tests Performed: October 06, 2192 at 04:14 PM

Total of **1** microbiology result(s):

Blood Culture, Routine: BLOOD CULTURE:

• CANCELLED

Microbiology Tests Performed: October 06, 2192 at 04:15 PM

Total of **1** microbiology result(s):

C. difficile PCR: STOOL:

• CANCELLED

Microbiology Tests Performed: October 06, 2192 at 04:15 PM

Total of **2** microbiology result(s):

FECAL CULTURE: STOOL:

• NO SALMONELLA OR SHIGELLA FOUND.

CAMPYLOBACTER CULTURE: STOOL:

• NO CAMPYLOBACTER FOUND.

Medications Administered: October 06, 2192 at 04:55 PM

Total of **2** medication(s) administered:

• HYDROmorphone (Dilaudid) 2mg TAB

• Acetaminophen 500mg TAB

Previous Medications on Record: October 06, 2192 at 05:07 PM

23 medication(s) documented:

• aspirin - Salicylate Analgesics

• aspirin - Platelet Aggregation Inhibitors - Salicylates

• atorvastatin - Antihyperlipidemic - HMG CoA Reductase Inhibitors (statins)

• B complex with C#20-folic acid [Nephrocaps] - B-Complex Vitamin Combinations

• blood-glucose meter [FreeStyle Freedom Lite] - Medical Supplies and DME - Glucose Monitoring Test Supplies

• blood-glucose meter [FreeStyle Freedom Lite] - Medical Supply, FDB Superset

• blood sugar diagnostic [FreeStyle Lite Strips] - Medical Supplies and DME - Blood Glucose Tests

• blood sugar diagnostic [FreeStyle Lite Strips] - Medical Supply, FDB Superset

• calcitriol - Vitamins - D Derivatives

• calcitriol - Hyperparathyroid Treatment Agents - Vitamin D Analog-Type

• calcium carbonate - Minerals and Electrolytes - Calcium Replacement

• cholecalciferol (vitamin D3) - Vitamins - D Derivatives

• furosemide - Diuretic - Loop

• insulin glargine [Lantus Solostar] - Insulin Analogs - Long Acting

• insulin lispro [Humalog] - Insulin Analogs - Rapid Acting

• lisinopril - ACE Inhibitors

• lorazepam - Antianxiety Agent - Benzodiazepines

• lorazepam - Benzodiazepines

• melatonin - Hypnotics - Melatonin - Single Agents

• Miralax - Laxative - Saline and Osmotic

• pen needle, diabetic [BD Insulin Pen Needle UF Mini] - Medical Supplies and DME - Insulin Needles-Syringes and Admin Supplies

• pen needle, diabetic [BD Insulin Pen Needle UF Mini] - Medical Supply, FDB Superset

• heparin (porcine) - Heparins

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*Time gap of **209 days, 2 hours, 5 minutes** since last event*

Emergency Department Visit #14

Admission Time: May 03, 2193 at 07:12 PM

This is the patient's **14th** Emergency Department visit. The patient arrived via **AMBULANCE**. The patient departed at **2193-05-05 02:13:00**, staying for a duration of **1 days 7 hours 1 minutes**.

ED Disposition: HOME - Patient was discharged directly from Emergency Department without hospital admission.

Initial Assessment:

Chief Complaint: L Arm numbness, L Arm swelling

Blood Pressure	119.0/64.0 mmHg

Heart Rate	100.0 bpm
Respiratory Rate	16.0 breaths/min
Oxygen Saturation	100.0%
Temperature	98.1°F
Pain Score	0
Acuity Level	2.0

Emergency Department Discharge Summary

Treatment: The patient received evaluation and treatment in the Emergency Department. Medical staff determined the patient was stable for discharge to **HOME**.

Follow-up Instructions: Patient advised to follow up with primary care physician or return to ED if symptoms worsen.

Previous Medications on Record: May 03, 2193 at 07:50 PM

23 medication(s) documented:

• acetaminophen - Analgesic or Antipyretic Non-Opioid

• aspirin - Salicylate Analgesics

• aspirin - Platelet Aggregation Inhibitors - Salicylates

• atorvastatin - Antihyperlipidemic - HMG CoA Reductase Inhibitors (statins)

• bisacodyl - Laxative - Stimulant

• calcium acetate - Phosphate Binders

• calcium acetate - Phosphate Binders - Calcium-based

• calcium carbonate - Antacid - Calcium

• calcium carbonate - Minerals and Electrolytes - Calcium Replacement

• cholecalciferol (vitamin D3) - Vitamins - D Derivatives

• fluoxetine - Antidepressant - Selective Serotonin Reuptake Inhibitors (SSRIs)

• furosemide - Diuretic - Loop

• glucagon (human recombinant) [Glucagon Emergency Kit (human)] - Agents to treat Hypoglycemia (Hyperglycemics)

• insulin glargine [Lantus Solostar U-100 Insulin] - Insulin Analogs - Long Acting

• lisinopril - ACE Inhibitors

• lorazepam - Antianxiety Agent - Benzodiazepines

• lorazepam - Benzodiazepines

• melatonin - Hypnotics - Melatonin - Single Agents

• pantoprazole - Gastric Acid Secretion Reducing Agents - Proton Pump Inhibitors (PPIs)

• polyethylene glycol 3350 - Laxative - Saline and Osmotic

• sennosides [senna] - Laxative - Stimulant

• sevelamer carbonate - Phosphate Binders

• sodium phosphates [Fleet Enema] - Laxative - Saline/Osmotic Mixtures

Medications Administered: May 03, 2193 at 10:38 PM

Total of **8** medication(s) administered:

• Aspirin 81mg TAB

• Lisinopril 5mg TAB

• Pantoprazole 40mg TAB

• Atorvastatin 20mg TAB

• Aspirin 81mg TAB

• Lisinopril 5mg TAB

• Pantoprazole 40mg TAB

• Atorvastatin 20mg TAB

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*Time gap of **7 minutes** since last event*

Hospital Admission #12

Admission Time: May 03, 2193 at 10:45 PM

This is the patient's **12th** hospital admission. The patient was admitted from **TRANSFER FROM HOSPITAL** as an **EU OBSERVATION** admission. The admitting provider was **P67JZU**. Insurance coverage: **Private**. Chief complaint: **N/A**. The primary service provided was **N/A**.

Race: **WHITE**, Marital Status: **SINGLE**, Language: **English**

Unit Admission - Emergency Department Observation

Admission Time: May 05, 2193 at 02:13 AM

The patient was admitted to **Emergency Department Observation** ward. The patient stayed in this unit until **2193-05-05 02:47:38**, for a total duration of **0 days 0 hours 34 minutes**. During this time, various procedures, laboratory tests, and microbiology events were performed.

Discharge Summary

Discharge Time: May 05, 2193 at 02:47 AM

The patient was discharged from **Emergency Department Observation** with disposition to **N/A**.

Known Allergies: None

Discharge Diagnoses:

Complete Diagnosis List:

- Localized swelling, mass and lump, left upper limb
- Anesthesia of skin
- Type 2 diabetes mellitus with diabetic chronic kidney disease
- Hypertensive chronic kidney disease with stage 5 chronic kidney disease or end stage renal disease
- End stage renal disease
- Dependence on renal dialysis
- Type 2 diabetes mellitus with diabetic polyneuropathy
- Type 2 diabetes mellitus with diabetic peripheral angiopathy without gangrene
- Hyperlipidemia, unspecified
- Anemia, unspecified
- Personal history of nicotine dependence
- Long term (current) use of insulin
- Acquired absence of other right toe(s)

*Time gap of **202 days, 9 hours, 50 minutes** since last event*

Emergency Department Visit #15

Admission Time: November 23, 2193 at 12:38 PM

This is the patient's **15th** Emergency Department visit. The patient arrived via **WALK IN**. The patient departed at **2193-11-23 19:16:36**, staying for a duration of **0 days 6 hours 38 minutes**.

Initial Assessment:

Chief Complaint: L Arm pain, L Arm swelling, ? cellulitis

Blood Pressure	154.0/76.0 mmHg
Heart Rate	77.0 bpm
Respiratory Rate	18.0 breaths/min
Oxygen Saturation	99.0%
Temperature	97.1°F
Pain Score	5
Acuity Level	3.0

Previous Medications on Record: November 23, 2193 at 05:05 PM

23 medication(s) documented:

• acetaminophen - Analgesic or Antipyretic Non-Opioid

• aspirin - Salicylate Analgesics

• aspirin - Platelet Aggregation Inhibitors - Salicylates

• atorvastatin - Antihyperlipidemic - HMG CoA Reductase Inhibitors (statins)

• B complex with C#20-folic acid [Nephrocaps] - B-Complex Vitamin Combinations

• calcium acetate - Phosphate Binders

• calcium acetate - Phosphate Binders - Calcium-based

• calcium carbonate - Antacid - Calcium

• calcium carbonate - Minerals and Electrolytes - Calcium Replacement

• cholecalciferol (vitamin D3) - Vitamins - D Derivatives

• docusate sodium [Stool Softener] - Laxative - Surfactant

• fluoxetine - Antidepressant - Selective Serotonin Reuptake Inhibitors (SSRIs)

• furosemide - Diuretic - Loop

• insulin glargine [Lantus Solostar U-100 Insulin] - Insulin Analogs - Long Acting

• lisinopril - ACE Inhibitors

• lorazepam - Antianxiety Agent - Benzodiazepines

• lorazepam - Benzodiazepines

• melatonin - Hypnotics - Melatonin - Single Agents

• metoprolol tartrate [Lopressor] - Beta Blockers Cardiac Selective

• pantoprazole - Gastric Acid Secretion Reducing Agents - Proton Pump Inhibitors (PPIs)

• polyethylene glycol 3350 - Laxative - Saline and Osmotic

• sennosides [senna] - Laxative - Stimulant

• sevelamer carbonate - Phosphate Binders

Medications Administered: November 23, 2193 at 05:33 PM

Total of **2** medication(s) administered:

• Vancomycin

• Vancomycin

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*Time gap of **1 hour, 42 minutes** since last event*

Hospital Admission #13

Admission Time: November 23, 2193 at 07:15 PM

This is the patient's **13th** hospital admission. The patient was admitted from **WALK-IN/SELF REFERRAL** as an **OBSERVATION ADMIT** admission. The admitting provider was **P13QBO**. Insurance coverage: **Private**. Chief complaint: **? cellulitis**. The primary service provided was **MEDICINE**.

Race: **WHITE**, Marital Status: **SINGLE**, Language: **English**

APR DRG Code:

Code: 182

Description: OTHER PERIPHERAL VASCULAR PROCEDURES

Severity: 2.0

Mortality Risk: 2.0

HCFA DRG Code:

Code: 252

Description: OTHER VASCULAR PROCEDURES W MCC

Patient Past History:

Past Medical History: ESRD stage 5 on HD with L AV fistula, DM, HTN, Chronic anemia, History of Septic R Hip, R foot ulcer s/p metatarsal resection (), s/p metatarsal removal (), L brachiocephalic vein occlusion and stenting

Family History: Mother with colon cancer and hyperlipidemia. Father with type 2 DM.

Social History: N/A

History of Present Illness:

yo male with ESRD on HD, IDDM and history of left brachiocephalic vein occlusion s/p stenting who presents from Presentation Rehab on recommendation from his outpatient nephrologist with left arm redness and swelling of entire left arm c/f restenosis of brachiocephalic vein and overlying cellulitis. His left arm swelling and redness has been worsening over past 2 weeks. He has noted the arm has become more painful over the last days. He is unable to provide a history of the pattern of spread for the erythema. He was evaluated by transplant nephrology in clinic today who noted c/f restenosis of left brachiocephalic vein stent. They were also concerned for cellulitis and therefore deferred fistulogram and angioplasty. Dr. a low suspicion for graft infection and no systematic signs of infection. They recommended arm elevation, exercise, antibiotic treatment and then proceeding with a fistulogram/angioplasty prior to discharge if edema does not resolve with treatment of cellulitis. Regarding his current AV graft, it was placed in left upper extremity on on . He has required 2 procedures since graft creation for arm swelling. His first procedure was in and at that time a significant stenosis at the venous anastomosis was also angioplastied and stented. The second procedure was in , and involved recanalization of a brachiocephalic occlusion and stenting of the brachiocephalic.

Admission Vital Signs:

General Appearance	Alert and interactive. In no acute distress.
Blood Pressure	184 / 10 mmHg
Heart Rate	84 bpm
Respiratory Rate	18 breaths/min
Temperature	97.6°F
Oxygen Saturation	96Ra

Admission Laboratory Results:

• WBC: 7.7

Admission Medications:

Total of **20** medications on admission:

• Lisinopril 5 mg PO DAILY

• LORazepam 0.5 mg PO THREE TIMES WEEKLY

• Metoprolol Tartrate 50 mg PO BID

• Atorvastatin 20 mg PO QPM

• Calcium Acetate 1334 mg PO TID W/MEALS

• FLUoxetine 20 mg PO DAILY

• Glargine 8 Units Bedtime

• Pantoprazole 40 mg PO Q24H

• sevelamer CARBONATE 2400 mg PO TID W/MEALS

• Polyethylene Glycol 17 g PO DAILY:PRN Constipation - Third Line

• Nepro Carb Steady (nut.tx.imp.renal fxn

• lac-reduc) 0.08 gram-1.8 kcal/mL oral BID

• Glucagon Emergency Kit (human) (glucagon (human recombinant)) 1 mg injection ONCE:PRN hypoglycemia

• Aspirin 81 mg PO DAILY

• Nephrocaps 1 CAP PO DAILY

• Acetaminophen 650 mg PO Q6H:PRN Pain - Mild/Fever

• Vitamin D UNIT PO DAILY

• melatonin 5 mg oral DAILY

• Senna 8.6 mg PO QHS

• ProMod Protein (protein supplement) 30 ml oral BID

Unit Admission - Medicine

Admission Time: November 23, 2193 at 07:16 PM

The patient was admitted to **Medicine** ward. Service provided: **MED**. The patient stayed in this unit until **2193-11-23 20:12:47**, for a total duration of **0 days 0 hours 56 minutes**. During this time, various procedures, laboratory tests, and microbiology events were performed.

Unit Admission - Medicine

Admission Time: November 23, 2193 at 08:12 PM

The patient was admitted to **Medicine** ward. The patient stayed in this unit until **2193-11-27 22:00:02**, for a total duration of **4 days 1 hours 47 minutes**. During this time, various procedures, laboratory tests, and microbiology events were performed.

Prescriptions Ordered: November 23, 2193 at 10:00 PM

21 medication(s) prescribed:

- Calcium Acetate 1334 mg PO/NG 3.0x/day
- Metoprolol Tartrate 50 mg PO/NG 2.0x/day
- FLUoxetine 20 mg PO/NG 1.0x/day
- Pantoprazole 40 mg PO 1.0x/day
- LORazepam 0.5 mg PO/NG
- Insulin 8 UNIT SC 1.0x/day
- Vitamin D 2000 UNIT PO/NG 1.0x/day
- sevelamer CARBONATE 2400 mg PO 3.0x/day
- Aspirin 81 mg PO/NG 1.0x/day
- Lisinopril 5 mg PO/NG 1.0x/day
- Sodium Chloride 0.9% Flush 3-10 mL IV 3.0x/day
- Atorvastatin 20 mg PO/NG 1.0x/day
- Heparin 5000 UNIT SC 2.0x/day
- Glucose Gel 15 g PO
- Ramelteon 8 mg PO/NG
- Nephrocaps 1 CAP PO/NG 1.0x/day
- Senna 8.6 mg PO/NG 1.0x/day
- Glucagon 1 mg IM
- Acetaminophen 650 mg PO/NG
- Polyethylene Glycol 17 g PO/NG
- Dextrose 50% 12.5 gm IV

Prescriptions Ordered: November 23, 2193 at 11:00 PM

2 medication(s) prescribed:

- Metoprolol Tartrate 25 mg PO/NG 4.0x/day
- Iso-Osmotic Dextrose 100 mL IV 1.0x/day

Procedure(s) Performed: November 24, 2193 at 12:00 AM

1 procedure(s) performed (Source: ICD):

• Performance of Urinary Filtration, Intermittent, Less than 6 Hours Per Day

Prescriptions Ordered: November 24, 2193 at 08:00 AM

1 medication(s) prescribed:

• Lisinopril 5 mg PO/NG 1.0x/day

Prescriptions Ordered: November 24, 2193 at 12:00 PM

3 medication(s) prescribed:

• Lidocaine 1% 1 mL ID

• Sodium Chloride 0.9% 1000 mL IV

• Heparin (Hemodialysis) 4000 UNIT IV 1.0x/day

Prescriptions Ordered: November 24, 2193 at 05:00 PM

1 medication(s) prescribed:

• Metoprolol Tartrate 25 mg PO/NG 4.0x/day

Prescriptions Ordered: November 24, 2193 at 08:00 PM

1 medication(s) prescribed:

• Metoprolol Tartrate 50 mg PO/NG 2.0x/day

Prescriptions Ordered: November 25, 2193 at 04:00 PM

1 medication(s) prescribed:

• Calcium Acetate 1334 mg PO/NG 3.0x/day

Prescriptions Ordered: November 26, 2193 at 08:00 AM

3 medication(s) prescribed:

• Heparin (Hemodialysis) 4000 UNIT IV 1.0x/day

• Sodium Chloride 0.9% 1000 mL IV

• Lidocaine 1% 1 mL ID

Prescriptions Ordered: November 26, 2193 at 02:00 PM

1 medication(s) prescribed:

• Sarna Lotion 1 Appl TP 3.0x/day

Prescriptions Ordered: November 26, 2193 at 05:00 PM

1 medication(s) prescribed:

• Iso-Osmotic Dextrose 200 mL IV 1.0x/day

Procedure(s) Performed: November 27, 2193 at 12:00 AM

3 procedure(s) performed (Source: ICD):

• Dilation of Left Brachial Vein, Percutaneous Approach

• Dilation of Left Cephalic Vein, Percutaneous Approach

• Fluoroscopy of Dialysis Shunt/Fistula using Low Osmolar Contrast

Prescriptions Ordered: November 27, 2193 at 08:00 AM

1 medication(s) prescribed:

• Lidocaine 1% 1 mL ID

Prescriptions Ordered: November 27, 2193 at 09:00 AM

2 medication(s) prescribed:

• Sodium Chloride 0.9% 1000 mL IV

• Heparin (Hemodialysis) 4000 UNIT IV 1.0x/day

Prescriptions Ordered: November 27, 2193 at 02:00 PM

1 medication(s) prescribed:

• Bag 1 BAG IV

Prescriptions Ordered: November 27, 2193 at 03:00 PM

1 medication(s) prescribed:

• Iso-Osmotic Dextrose 100 mL IV 1.0x/day

Discharge Summary

Discharge Time: November 27, 2193 at 10:00 PM

The patient was discharged from **Medicine** with disposition to **Extended Care**.

Major Procedure(s) During Admission: Fistulogram with on .

Known Allergies: Penicillins, metformin, Naprosyn

Detailed Allergy List:

- **Penicillins**
- **metformin**
- **Naprosyn**

Discharge Diagnoses:

Primary Diagnoses:

- Thrombosis due to vascular prosthetic devices
- implants and grafts

Secondary Diagnoses:

- End Stage Renal Disease
- Type 2 Diabetes

Complete Diagnosis List:

- Thrombosis due to vascular prosthetic devices, implants and grafts, initial encounter
- End stage renal disease
- Hypertensive chronic kidney disease with stage 5 chronic kidney disease or end stage renal disease
- Cellulitis of left upper limb
- Type 2 diabetes mellitus with foot ulcer
- Non-pressure chronic ulcer of other part of right foot limited to breakdown of skin
- Surgical instruments, materials and cardiovascular devices (including sutures) associated with adverse incidents
- Type 2 diabetes mellitus with diabetic chronic kidney disease
- Prurigo nodularis
- Hyperlipidemia, unspecified
- Anxiety disorder, unspecified
- Gastro-esophageal reflux disease without esophagitis
- Dependence on renal dialysis
- Long term (current) use of insulin
- Anemia in chronic kidney disease
- Acquired absence of right foot
- Alcohol dependence, in remission

• Personal history of nicotine dependence

• Peripheral vascular angioplasty status with implants and grafts

• Personal history of other malignant neoplasm of skin

Hospital Course:

Mr. is a with IDDM, ESRD on HD MWF, and and history of left brachiocephalic vein occlusion s/p stenting who presents with weeks of left arm swelling and 1.5 weeks of erythema due to venous congestion/stenosis vs cellulitis. #Left arm edema with concern for Left brachiocephalic vein stent restenosis and cellulitis. Arm swelling is concerning for restenosis of left brachiocephalic vein occlusion which was noted on fistulogram in and s/p stenting. Patient was evaluated by interventional radiology and transplant surgery, underwent fistulogram with on with balloon angioplasty of brachiocephalic vein stent. Given erythema and edema patient was also treated with IV Vancomycin with HD. Plan for a 7 day course (). #ESRD on HD Pt currently with AV fistula for only access. Transplant surgery reports it is ok to use AV fistula as it is functioning now. Continued nephrocaps, sevelemer, Vitamin D, calcium acetate. #Hypertension: Elevated BP on admission. Continued home metoprolol tartrate 50mg BID and lisinopril 5mg #Insulin Dependent Diabetes: Continue lantus 8 units at night and ISS. #Primary Prevention: Continue aspirin 81mg #HLD: Continue home atorvastatin #Anxiety: Continue home fluoxetine and home lorazepam 0.5mg 1 tablet by mouth 3x weekly pre dialysis. #GERD: Continue pantoprazole 40mg

Microbiology Findings:

negative blood culture

Antibiotic Plan:

[]Please continue IV Vancomycin with HD to complete 7 day course, last day of administration on .

Discharge Status:

Activity Status	Ambulatory - Independent.
Level of Consciousness	Alert and interactive.
Mental Status	Clear and coherent.
Code Status	full confirmed

Discharge Instructions:

Dear Mr , It was a privilege caring for you at . WHY WAS I IN THE HOSPITAL? - You were admitted to the hospital because your left arm was very swollen WHAT HAPPENED TO ME IN THE HOSPITAL? - In the hospital you continued your normal dialysis schedule - We gave you IV antibiotics in case your arm was infected - We performed a procedure to allow better blood flow in the veins in your arm WHAT SHOULD I DO AFTER I LEAVE THE HOSPITAL? - Continue to take all your medicines and keep your appointments. We wish

you the best! Sincerely, Your Team

*Time gap of **351 days, 16 hours, 43 minutes** since last event*

Emergency Department Visit #16

Admission Time: November 14, 2194 at 02:44 PM

This is the patient's **16th** Emergency Department visit. The patient arrived via **N/A**. The patient departed at **2194-11-19 15:06:00**, staying for a duration of **5 days 0 hours 22 minutes**.

*Time gap of **5 hours, 8 minutes** since last event*

Hospital Admission #14

Admission Time: November 14, 2194 at 07:52 PM

This is the patient's **14th** hospital admission. The patient was admitted from **TRANSFER FROM SKILLED NURSING FACILITY** as an **EU OBSERVATION** admission. The admitting provider was **P574VN**. Insurance coverage: **Private**. Chief complaint: **N/A**. The primary service provided was **N/A**.

Race: **WHITE**, Marital Status: **SINGLE**, Language: **English**

Unit Admission - Emergency Department Observation

Admission Time: November 19, 2194 at 03:06 PM

The patient was admitted to **Emergency Department Observation** ward. The patient stayed in this unit until **2194-11-19 15:19:31**, for a total duration of **0 days 0 hours 13 minutes**. During this time, various procedures, laboratory tests, and microbiology events were performed.

Discharge Summary

Discharge Time: November 19, 2194 at 03:19 PM

The patient was discharged from **Emergency Department Observation** with disposition to **N/A**.

Known Allergies: None

Discharge Diagnoses:

Complete Diagnosis List:

• Stenosis of other vascular prosthetic devices, implants and grafts, initial encounter

• Surgical operation with anastomosis, bypass or graft as the cause of abnormal reaction of the patient, or of later complication, without mention of misadventure at the time of the procedure

• Hypertensive chronic kidney disease with stage 5 chronic kidney disease or end stage renal disease

• Type 2 diabetes mellitus with diabetic chronic kidney disease

• End stage renal disease

• Type 2 diabetes mellitus with diabetic peripheral angiopathy without gangrene

• Hyperlipidemia, unspecified

• Dependence on renal dialysis

• Long term (current) use of insulin

• Acquired absence of other right toe(s)

• Personal history of nicotine dependence

• Family history of diabetes mellitus

• Personal history of other infectious and parasitic diseases

*Time gap of **9 days, 1 hour, 19 minutes** since last event*

Emergency Department Visit #17

Admission Time: November 28, 2194 at 04:39 PM

This is the patient's **17th** Emergency Department visit. The patient arrived via **N/A**. The patient departed at **2194-11-29 18:11:00**, staying for a duration of **1 days 1 hours 32 minutes**.

*Time gap of **5 hours, 5 minutes** since last event*

Hospital Admission #15

Admission Time: November 28, 2194 at 09:44 PM

This is the patient's **15th** hospital admission. The patient was admitted from **TRANSFER FROM SKILLED NURSING FACILITY** as an **EU OBSERVATION** admission. The admitting provider was **P67JZU**. Insurance coverage: **Private**. Chief complaint: **N/A**. The primary service provided was **N/A**.

Race: **WHITE**, Marital Status: **SINGLE**, Language: **English**

Unit Admission - Emergency Department Observation

Admission Time: November 29, 2194 at 06:11 PM

The patient was admitted to **Emergency Department Observation** ward. The patient stayed in this unit until **2194-11-29 18:12:58**, for a total duration of **0 days 0 hours 1 minutes**. During this time, various procedures, laboratory tests, and microbiology events were performed.

Discharge Summary

Discharge Time: November 29, 2194 at 06:12 PM

The patient was discharged from **Emergency Department Observation** with disposition to **N/A**.

Known Allergies: None

Discharge Diagnoses:

Complete Diagnosis List:

- Unspecified nondisplaced fracture of surgical neck of right humerus, initial encounter for closed fracture
- Dependence on renal dialysis
- Long term (current) use of insulin
- Fall on same level from slipping, tripping and stumbling with subsequent striking against unspecified object, initial encounter
- Unspecified place in unspecified non-institutional (private) residence as the place of occurrence of the external cause
- Type 2 diabetes mellitus with diabetic chronic kidney disease
- End stage renal disease
- COVID-19
- Elevated blood-pressure reading, without diagnosis of hypertension

*Time gap of **19 days, 17 hours, 45 minutes** since last event*

Emergency Department Visit #18

Admission Time: December 19, 2194 at 11:58 AM

This is the patient's **18th** Emergency Department visit. The patient arrived via **N/A**. The patient departed at **2194-12-19 19:44:00**, staying for a duration of **0 days 7 hours 46 minutes**.

*Time gap of **6 hours, 17 minutes** since last event*

Hospital Admission #16

Admission Time: December 19, 2194 at 06:15 PM

This is the patient's **16th** hospital admission. The patient was admitted from **AMBULATORY SURGERY TRANSFER** as an **EU OBSERVATION** admission. The admitting provider was **P830TJ**. Insurance coverage: **Private**. Chief complaint: **N/A**. The primary service provided was **N/A**.

Race: **WHITE**, Marital Status: **SINGLE**, Language: **English**

Unit Admission - Medicine

Admission Time: December 19, 2194 at 07:44 PM

The patient was admitted to **Medicine** ward. The patient stayed in this unit until **2194-12-21 19:27:52**, for a total duration of **1 days 23 hours 43 minutes**. During this time, various procedures, laboratory tests, and microbiology events were performed.

Prescriptions Ordered: December 19, 2194 at 08:00 PM

2 medication(s) prescribed:

• Heparin 5000 UNIT SC 2.0x/day

• Sodium Chloride 0.9% Flush 3-10 mL IV 3.0x/day

Prescriptions Ordered: December 19, 2194 at 09:00 PM

16 medication(s) prescribed:

• Glucose Gel 15 g PO

- Senna 8.6 mg PO/NG
- Dextrose 50% 12.5 gm IV
- Atorvastatin 20 mg PO/NG 1.0x/day
- Calcium Acetate 1334 mg PO/NG 3.0x/day
- Acetaminophen 650 mg PO/NG
- Bisacodyl 10 mg PR
- Pantoprazole 40 mg PO 1.0x/day
- Ondansetron 4 mg PO/NG
- Metoprolol Succinate XL 100 mg PO 1.0x/day
- Glucagon 1 mg IM
- Nephrocaps 1 CAP PO/NG 1.0x/day
- Fleet Enema (Mineral Oil) 1 Enema PR
- OxyCODONE (Immediate Release) 5 mg PO/NG
- sevelamer CARBONATE 800 mg PO 3.0x/day
- Aspirin 81 mg PO/NG 1.0x/day

Prescriptions Ordered: December 19, 2194 at 10:00 PM

1 medication(s) prescribed:

- Insulin 4 UNIT SC 1.0x/day

Prescriptions Ordered: December 20, 2194 at 09:00 AM

1 medication(s) prescribed:

- Iso-Osmotic Sodium Chloride 1 BAG IV 1.0x/day

Prescriptions Ordered: December 20, 2194 at 10:00 AM

3 medication(s) prescribed:

- Furosemide 80 mg IV 1.0x/day
- Dextrose 50% 50 gm IV 1.0x/day
- Insulin (Regular) for Hyperkalemia 10 UNIT IV 1.0x/day

Prescriptions Ordered: December 20, 2194 at 11:00 AM

2 medication(s) prescribed:

- Heparin (Hemodialysis) 4000 UNIT IV 1.0x/day

• Sodium Chloride 0.9% 1000 mL IV

Prescriptions Ordered: December 21, 2194 at 10:00 AM

3 medication(s) prescribed:

• Lidocaine 1% 1 mL ID

• Sodium Chloride 0.9% 1000 mL IV

• Heparin (Hemodialysis) 4000 UNIT IV 1.0x/day

Prescriptions Ordered: December 21, 2194 at 12:00 PM

1 medication(s) prescribed:

• Lisinopril 5 mg PO/NG 1.0x/day

Discharge Summary

Discharge Time: December 21, 2194 at 07:27 PM

The patient was discharged from **Medicine** with disposition to **N/A**.

Known Allergies: None

Discharge Diagnoses:

Complete Diagnosis List:

• Other mechanical complication of surgically created arteriovenous fistula, initial encounter

• Surgical operation with anastomosis, bypass or graft as the cause of abnormal reaction of the patient, or of later complication, without mention of misadventure at the time of the procedure

• Unspecified place in nursing home as the place of occurrence of the external cause

• Type 2 diabetes mellitus with diabetic chronic kidney disease

• Hypertensive chronic kidney disease with stage 5 chronic kidney disease or end stage renal disease

• End stage renal disease

• Dependence on renal dialysis

• Hyperkalemia

• Unspecified displaced fracture of surgical neck of right humerus, initial encounter for closed fracture

• Unspecified fall, initial encounter

• Unspecified place or not applicable

• Long term (current) use of insulin

• Anemia in chronic kidney disease

- Personal history of nicotine dependence
- Hyperlipidemia, unspecified
- Gastro-esophageal reflux disease without esophagitis

*Time gap of **188 days, 21 hours, 35 minutes** since last event*

Emergency Department Visit #19

Admission Time: June 28, 2195 at 05:03 PM

This is the patient's **19th** Emergency Department visit. The patient arrived via **N/A**. The patient departed at **2195-06-29 18:37:00**, staying for a duration of **1 days 1 hours 34 minutes**.

*Time gap of **2 hours, 37 minutes** since last event*

Hospital Admission #17

Admission Time: June 28, 2195 at 07:40 PM

This is the patient's **17th** hospital admission. The patient was admitted from **WALK-IN/SELF REFERRAL** as an **EU OBSERVATION** admission. The admitting provider was **P3715A**. Insurance coverage: **Private**. Chief complaint: **N/A**. The primary service provided was **N/A**.

Race: **WHITE**, Marital Status: **SINGLE**, Language: **English**

Unit Admission - Emergency Department Observation

Admission Time: June 29, 2195 at 06:37 PM

The patient was admitted to **Emergency Department Observation** ward. The patient stayed in this unit until **2195-06-29 18:41:49**, for a total duration of **0 days 0 hours 4 minutes**. During this time, various procedures, laboratory tests, and microbiology events were performed.

Discharge Summary

Discharge Time: June 29, 2195 at 06:41 PM

The patient was discharged from **Emergency Department Observation** with disposition to **N/A**.

Known Allergies: None

Discharge Diagnoses:

Complete Diagnosis List:

• Thrombosis due to vascular prosthetic devices, implants and grafts, initial encounter

• Surgical operation with anastomosis, bypass or graft as the cause of abnormal reaction of the patient, or of later complication, without mention of misadventure at the time of the procedure

• Unspecified place or not applicable

• Hypertensive chronic kidney disease with stage 5 chronic kidney disease or end stage renal disease

• Type 2 diabetes mellitus with diabetic chronic kidney disease

• End stage renal disease

• Dependence on renal dialysis

• Long term (current) use of insulin

• Type 2 diabetes mellitus with diabetic peripheral angiopathy without gangrene

• Peripheral vascular angioplasty status with implants and grafts

• Anemia in chronic kidney disease

• Contact with and (suspected) exposure to other viral communicable diseases

• Hyperlipidemia, unspecified

• Acquired absence of other right toe(s)

• Personal history of nicotine dependence

*Time gap of **215 days, 18 hours, 50 minutes** since last event*

Emergency Department Visit #20

Admission Time: January 31, 2196 at 01:32 PM

This is the patient's **20th** Emergency Department visit. The patient arrived via **N/A**. The patient departed at **2196-01-31 21:23:00**, staying for a duration of **0 days 7 hours 51 minutes**.

*Time gap of **5 hours, 38 minutes** since last event*

Hospital Admission #18

Admission Time: January 31, 2196 at 07:10 PM

This is the patient's **18th** hospital admission. The patient was admitted from **WALK-IN/SELF REFERRAL** as an **OBSERVATION ADMIT** admission. The admitting provider was **P59ZHU**. Insurance coverage: **Private**. Chief complaint: **N/A**. The primary service provided was **N/A**.

Race: **WHITE**, Marital Status: **SINGLE**, Language: **English**

APR DRG Code:

Code: 207

Description: OTHER CIRCULATORY SYSTEM DIAGNOSES

Severity: 3.0

Mortality Risk: 3.0

HCFA DRG Code:

Code: 314

Description: OTHER CIRCULATORY SYSTEM DIAGNOSES WITH MCC

Unit Admission - Medicine/Cardiology

Admission Time: January 31, 2196 at 09:23 PM

The patient was admitted to **Medicine/Cardiology** ward. The patient stayed in this unit until **2196-02-01 18:08:53**, for a total duration of **0 days 20 hours 45 minutes**. During this time, various procedures, laboratory

tests, and microbiology events were performed.

Prescriptions Ordered: January 31, 2196 at 10:00 PM

2 medication(s) prescribed:

• Sodium Chloride 0.9% Flush 3-10 mL IV 3.0x/day

• Heparin 5000 UNIT SC 2.0x/day

Prescriptions Ordered: February 01, 2196 at 12:00 AM

13 medication(s) prescribed:

• Lisinopril 5 mg PO/NG 1.0x/day

• Pantoprazole 40 mg PO 1.0x/day

• Atorvastatin 20 mg PO/NG 1.0x/day

• FLUoxetine 20 mg PO/NG 1.0x/day

• Insulin 0 UNIT SC

• Vitamin D 2000 UNIT PO/NG 1.0x/day

• Midodrine 5 mg PO/NG

• Glucagon 1 mg IM

• LORazepam 0.5 mg PO/NG

• Nephrocaps 1 CAP PO/NG 1.0x/day

• Calcium Acetate 1334 mg PO/NG 3.0x/day

• Glucose Gel 15 g PO

• Dextrose 50% 12.5 gm IV

Procedure(s) Performed: February 01, 2196 at 12:00 AM

2 procedure(s) performed (Source: ICD):

• Drainage of Pericardial Cavity with Drainage Device, Percutaneous Approach

• Drainage of Left Pleural Cavity with Drainage Device, Percutaneous Approach

Prescriptions Ordered: February 01, 2196 at 05:00 AM

1 medication(s) prescribed:

• Ramelteon 8 mg PO/NG

Prescriptions Ordered: February 01, 2196 at 08:00 AM

1 medication(s) prescribed:

• Aspirin 81 mg PO/NG 1.0x/day

Microbiology Tests Performed: February 01, 2196 at 12:42 PM

Total of **6** microbiology result(s):

GRAM STAIN: PLEURAL FLUID:

• 1+ (<1 per 1000X FIELD): POLYMORPHONUCLEAR LEUKOCYTES. NO MICROORGANISMS SEEN.

FLUID CULTURE: PLEURAL FLUID:

• NO GROWTH.

ANAEROBIC CULTURE: PLEURAL FLUID:

• NO GROWTH.

FUNGAL CULTURE: PLEURAL FLUID:

• NO FUNGUS ISOLATED.

ACID FAST SMEAR: PLEURAL FLUID:

• NO ACID FAST BACILLI SEEN ON DIRECT SMEAR.

ACID FAST CULTURE: PLEURAL FLUID:

• NO MYCOBACTERIA ISOLATED.

Prescriptions Ordered: February 01, 2196 at 01:00 PM

1 medication(s) prescribed:

• Lidocaine 1%/Epinephrine 1:100,000 20 mL SC 1.0x/day

Prescriptions Ordered: February 01, 2196 at 04:00 PM

2 medication(s) prescribed:

• OxyCODONE (Immediate Release) 2.5-5 mg PO/NG

• Acetaminophen 500-1000 mg PO/NG

Microbiology Tests Performed: February 01, 2196 at 05:24 PM

Total of **6** microbiology result(s):

GRAM STAIN: FLUID,OTHER:

• 1+ (<1 per 1000X FIELD): POLYMORPHONUCLEAR LEUKOCYTES. NO MICROORGANISMS SEEN.

FLUID CULTURE: FLUID,OTHER:

• NO GROWTH.

ANAEROBIC CULTURE: FLUID,OTHER:

• NO GROWTH.

FUNGAL CULTURE: FLUID,OTHER:

• NO FUNGUS ISOLATED.

ACID FAST SMEAR: FLUID,OTHER:

• NO ACID FAST BACILLI SEEN ON DIRECT SMEAR.

ACID FAST CULTURE: FLUID,OTHER:

• NO MYCOBACTERIA ISOLATED.

Microbiology Tests Performed: February 01, 2196 at 05:24 PM

Total of **1** microbiology result(s):

Fluid Culture in Bottles: FLUID RECEIVED IN BLOOD CULTURE BOTTLES:

• NO GROWTH.

Unit Admission - Medicine/Cardiology Intermediate

Admission Time: February 01, 2196 at 06:08 PM

The patient was admitted to **Medicine/Cardiology Intermediate** ward. The patient stayed in this unit until **2196-02-06 14:11:58**, for a total duration of **4 days 20 hours 3 minutes**. During this time, various procedures, laboratory tests, and microbiology events were performed.

Prescriptions Ordered: February 01, 2196 at 08:00 PM

1 medication(s) prescribed:

• Sodium Chloride 0.9% Flush 2-4 mL IV 6.0x/day

Procedure(s) Performed: February 02, 2196 at 12:00 AM

1 procedure(s) performed (Source: ICD):

• Performance of Urinary Filtration, Intermittent, Less than 6 Hours Per Day

Prescriptions Ordered: February 02, 2196 at 08:00 AM

3 medication(s) prescribed:

• Sodium Chloride 0.9% 1000 mL IV

• Lidocaine 1% 1 mL ID

• Calcitriol 0.25 mcg PO

Prescriptions Ordered: February 02, 2196 at 11:00 AM

1 medication(s) prescribed:

• Lidocaine 5% Patch 1 PTCH TD 1.0x/day

Prescriptions Ordered: February 03, 2196 at 10:00 AM

1 medication(s) prescribed:

• Colchicine 0.3 mg PO

Prescriptions Ordered: February 04, 2196 at 10:00 AM

1 medication(s) prescribed:

• sevelamer CARBONATE 2400 mg PO/NG 3.0x/day

Prescriptions Ordered: February 05, 2196 at 07:00 AM

3 medication(s) prescribed:

• Epoetin alfa-epbx (Retacrit) 4000 UNIT IV 1.0x/day

• Lidocaine 1% 1 mL ID

• Sodium Chloride 0.9% 1000 mL IV

Discharge Summary

Discharge Time: February 06, 2196 at 02:11 PM

The patient was discharged from **Medicine/Cardiology Intermediate** with disposition to **N/A**.

Known Allergies: None

Discharge Diagnoses:

Complete Diagnosis List:

- Acute pericarditis, unspecified
- End stage renal disease
- Pleural effusion, not elsewhere classified
- Hypertensive chronic kidney disease with stage 5 chronic kidney disease or end stage renal disease
- Cardiac tamponade
- Type 2 diabetes mellitus with diabetic chronic kidney disease
- Dependence on renal dialysis
- Long term (current) use of insulin
- Type 2 diabetes mellitus with diabetic neuropathy, unspecified
- Hyperlipidemia, unspecified
- Alcohol dependence, in remission
- Gastro-esophageal reflux disease without esophagitis
- Major depressive disorder, single episode, unspecified
- Acquired absence of other right toe(s)
- Anemia in chronic kidney disease
- Contact with and (suspected) exposure to COVID-19

*Time gap of **202 days, 1 hour, 50 minutes** since last event*

Emergency Department Visit #21

Admission Time: August 26, 2196 at 04:02 PM

This is the patient's **21st** Emergency Department visit. The patient arrived via **N/A**. The patient departed at **2196-08-27 11:16:00**, staying for a duration of **0 days 19 hours 14 minutes**.

*Time gap of **7 hours, 45 minutes** since last event*

Hospital Admission #19

Admission Time: August 26, 2196 at 11:47 PM

This is the patient's **19th** hospital admission. The patient was admitted from **WALK-IN/SELF REFERRAL** as an **EU OBSERVATION** admission. The admitting provider was **P152TM**. Insurance coverage: **Private**. Chief complaint: **N/A**. The primary service provided was **N/A**.

Race: **WHITE**, Marital Status: **SINGLE**, Language: **English**

Unit Admission - Emergency Department Observation

Admission Time: August 27, 2196 at 11:16 AM

The patient was admitted to **Emergency Department Observation** ward. The patient stayed in this unit until **2196-08-27 11:51:02**, for a total duration of **0 days 0 hours 35 minutes**. During this time, various procedures, laboratory tests, and microbiology events were performed.

Discharge Summary

Discharge Time: August 27, 2196 at 11:51 AM

The patient was discharged from **Emergency Department Observation** with disposition to **N/A**.

Known Allergies: None

Discharge Diagnoses:

Complete Diagnosis List:

- Cellulitis, unspecified

*Time gap of **134 days, 13 hours, 26 minutes** since last event*

Emergency Department Visit #22

Admission Time: January 09, 2197 at 01:18 AM

This is the patient's **22nd** Emergency Department visit. The patient arrived via **N/A**. The patient departed at **2197-01-09 05:01:55**, staying for a duration of **0 days 3 hours 43 minutes**.

*Time gap of **3 hours, 42 minutes** since last event*

Hospital Admission #21

Admission Time: January 09, 2197 at 05:00 AM

This is the patient's **21st** hospital admission. The patient was admitted from **TRANSFER FROM SKILLED NURSING FACILITY** as an **EU OBSERVATION** admission. The admitting provider was **P250TS**. Insurance coverage: **Private**. Chief complaint: **N/A**. The primary service provided was **N/A**.

Race: **WHITE**, Marital Status: **SINGLE**, Language: **English**

Unit Admission - Emergency Department Observation

Admission Time: January 09, 2197 at 05:01 AM

The patient was admitted to **Emergency Department Observation** ward. Service provided: **MED**. The patient stayed in this unit until **2197-01-09 07:25:12**, for a total duration of **0 days 2 hours 23 minutes**. During this time, various procedures, laboratory tests, and microbiology events were performed.

Unit Admission - PACU

Admission Time: January 09, 2197 at 07:25 AM

The patient was admitted to **PACU** ward. The patient stayed in this unit until **2197-01-09 16:23:00**, for a total duration of **0 days 8 hours 57 minutes**. During this time, various procedures, laboratory tests, and microbiology events were performed.

Unit Admission - PACU

Admission Time: January 09, 2197 at 04:23 PM

The patient was admitted to **PACU** ward. The patient stayed in this unit until **2197-01-09 17:45:32**, for a total duration of **0 days 1 hours 22 minutes**. During this time, various procedures, laboratory tests, and microbiology events were performed.

Unit Admission - Med/Surg

Admission Time: January 09, 2197 at 05:45 PM

The patient was admitted to **Med/Surg** ward. The patient stayed in this unit until **2197-01-11 15:32:23**, for a total duration of **1 days 21 hours 46 minutes**. During this time, various procedures, laboratory tests, and microbiology events were performed.

Prescriptions Ordered: January 09, 2197 at 06:00 PM

6 medication(s) prescribed:

- Sodium Chloride 0.9% Flush 3-10 mL IV 3.0x/day
- Dextrose 10% in Water (D10W) 100 mL IV BOLUS
- Glucagon 1 mg IM
- Glucose Gel 1 TUBE PO
- Acetaminophen 650 mg PO/NG
- Pantoprazole 40 mg PO 1.0x/day

Prescriptions Ordered: January 09, 2197 at 08:00 PM

1 medication(s) prescribed:

- Heparin 5000 UNIT SC 2.0x/day

Prescriptions Ordered: January 09, 2197 at 10:00 PM

2 medication(s) prescribed:

- Insulin 8 UNIT SC 1.0x/day
- Gabapentin 100 mg PO/NG 1.0x/day

Prescriptions Ordered: January 10, 2197 at 08:00 AM

3 medication(s) prescribed:

- Metoprolol Succinate XL 50 mg PO 1.0x/day
- Aspirin 81 mg PO/NG 1.0x/day

• FLUoxetine 20 mg PO/NG 1.0x/day

Prescriptions Ordered: January 10, 2197 at 10:00 AM

1 medication(s) prescribed:

• Midodrine 5 mg PO

Prescriptions Ordered: January 11, 2197 at 01:00 AM

1 medication(s) prescribed:

• OxyCODONE (Immediate Release) 5 mg PO/NG

Prescriptions Ordered: January 11, 2197 at 07:00 AM

4 medication(s) prescribed:

• Heparin Dwell (1000 Units/mL) 1000-4000 UNIT DWELL

• Vial 10 mL IV 1.0x/day

• Sodium Chloride 0.9% 1000 mL IV

• Heparin Dwell (1000 Units/mL) 1000-4000 UNIT DWELL

Prescriptions Ordered: January 11, 2197 at 08:00 AM

1 medication(s) prescribed:

• Sodium Zirconium Cyclosilicate 10 g PO/NG 1.0x/day

Discharge Summary

Discharge Time: January 11, 2197 at 03:32 PM

The patient was discharged from **Med/Surg** with disposition to **N/A**.

Known Allergies: None

Discharge Diagnoses:

Complete Diagnosis List:

• Hemorrhage due to vascular prosthetic devices, implants and grafts, initial encounter

• Non-pressure chronic ulcer of skin of other sites with unspecified severity

• Type 2 diabetes mellitus with diabetic chronic kidney disease

• Hypertensive chronic kidney disease with stage 5 chronic kidney disease or end stage renal disease

• Dependence on renal dialysis

- Long term (current) use of insulin
- Hyperlipidemia, unspecified
- Contact with and (suspected) exposure to COVID-19
- Type 2 diabetes mellitus with diabetic peripheral angiopathy without gangrene
- Chronic obstructive pulmonary disease, unspecified
- Gastro-esophageal reflux disease without esophagitis
- Type 2 diabetes mellitus with diabetic polyneuropathy
- Alcohol abuse, in remission
- Personal history of nicotine dependence